

THE 1,000 MEDICAL PROMPTS · VOLUME 3 OF 3

STUDY, EXAM & VIVA PREP.

300 prompts to learn any topic, plan revision, practise MCQs for NEET-PG and UPSC CMS, write university answers, face the viva and reason through a differential diagnosis.

300

prompts · #701–1000

7

sections

37 · 169 · 94

Master · Standard · Quick

3.1

Learning any topic

3.2

Study planning & revision

3.3

MCQ practice & exam strategy

3.4

Long & short answer writing

3.5

Viva, OSCE & practical exams

3.6

Clinical reasoning & differential diagnosis

3.7

Memory aids & high-yield summaries

Read this first.

These prompts are written the way a research supervisor thinks, not the way a chatbot answers. Many are long on purpose: a vague prompt gets you a vague, confident, wrong answer. Before you paste anything, five non-negotiables.

01 The AI does not know your patients.

Every prompt has a context block. Fill it honestly. If you say you have 200 cases a year when you have 40, the model will design a study you cannot finish.

02 Never accept a citation you have not opened.

Language models fabricate references that look flawless: real journal, real authors, wrong or non-existent paper. Verify every reference on PubMed by PMID or DOI before it enters your work.

03 Never paste identifiable patient data.

No names, hospital IDs, MRNs or scans with burned-in identifiers. De-identify before the data touches a chatbot. This is an ethics committee issue, not a preference.

04 The AI drafts. You decide.

Use it to speed up structure, phrasing and pre-checking. The scientific judgement, the assumptions and the interpretation stay yours. Your name goes on the paper, not the model's.

05 Declare AI use where required.

ICMJE and most journals require disclosure of generative AI use. AI cannot be an author. Check your journal's policy and your university's thesis rules before submission.

How to use each prompt

- › Copy the full prompt. The length is what makes the output specific.
- › Replace every [SQUARE BRACKET] with your real details. Options inside a bracket are separated by "/"; keep the one that applies.
- › Put anything you paste (abstracts, drafts, results) where the prompt shows [PASTE ...], above the task.
- › Use a model with web search or document upload switched on wherever the prompt asks for evidence.
- › Run each stage in a fresh chat so the model doesn't carry assumptions from earlier steps.
- › Push back. If the output is generic, reply: "Be more specific. Tie every point to my context."

Every prompt is built on **P-R-O-M-P-T**.

One formula, six layers. Each layer matches what OpenAI, Anthropic, Google and Microsoft recommend in their own prompting guides.

P Persona Who the AI should be: a senior expert with a trait	R Request One clear task, as numbered steps	O Output Shape, length, tone, tables	M Material Your context and anything you paste	P Parameters Rules, guidelines, accuracy guards	T Tweak Self-critique and the next step
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See it in action

BAD	"Tell me about pneumonia."
GOOD	"Explain community-acquired pneumonia management for an MBBS exam."
BETTER	"Act as a pulmonologist. Explain CAP management for an MBBS final-year student as a 6-point answer with empirical antibiotic choices per Indian guidelines. Keep it under 150 words, exam-oriented."

Three kinds of prompt

QUICK 40–80 words One focused output in a few sentences. Each has a TRY IT WITH example.	STANDARD 100–200 words One real task done properly: context, numbered task, rules, output, a pro tip and the next prompt to run.	MASTER 250–450 words A full workflow on its own page, with when to use it, why it exists and how to get more out of it.
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Tags you will see in the outputs

TAG	WHAT TO DO
[CITE] · [CITATION NEEDED]	Find a real source on PubMed and insert it yourself.
[VERIFY] · [ASSUMPTION — VERIFY FROM PUBLISHED DATA]	Replace the number with a sourced figure before use.
[UNVERIFIED BACKGROUND]	Treat as a lead to check, not a fact.
[ASK ME] · [NEED INFO] · [NOT REPORTED]	The model is missing information. Supply it, or leave it out.

Every prompt works in ChatGPT, Claude, Gemini and Copilot. → **NEXT** points to the prompt to run after this one.

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3.1

Learning any topic

Understanding beats memorising. Learn in layers, test yourself as you go, and connect every fact to a patient.

50

prompts

6

Master

28

Standard

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Quick

#701 Structured topic master for any subject

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#722 Learn a clinical guideline for exams

#723 Learn any drug with one framework

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#725 Learn an anatomy topic that sticks

#726 Understand physiology graphs and curves

#727 Learn to read a type of clinical image

#728 Normal values and interpretation

#729 Learn a microorganism with one template

#730 Simplify a biochemistry pathway

#731 Learn national health programmes (community medicine)

#732 Make my own questions from my notes

#733 Design an interleaved practice session

#734 Diagnose my knowledge gaps

#735 Explain it like I'm a second-year student

#736 One-page summary of a topic

#737 Ten must-know facts

#738 Ask "why" five times

#739 Clear up two confusing terms

#740 A real-world example of a concept

#741 Why does this basic-science fact matter clinically?

#742 A quick 5-question quiz

#743 Define it, with an example

#744 Clear up one specific confusion

#745 The whole topic in 280 characters

#746 A diagram I can draw

#747 What should I study next?

#748 Is my understanding correct?

#749 Translate the jargon

#750 Three questions the examiner will ask

Structured topic master for any subject

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT

For deep revision of any topic in any subject, before university exams, NEET-PG or UPSC CMS.

WHY

Scattered notes from five sources waste revision time. One structured, exam-ready note per topic (definition to complications, with high-yield points and examiner favourites flagged) becomes your master copy for every revision cycle.

COPY THE PROMPT BELOW

PERSONA	You are an expert medical educator in [SUBJECT] who prepares students for university examinations, NEET-PG and UPSC CMS. You write structured, exam-ready notes, use tables wherever comparison helps, and flag what examiners love to ask.
MY CONTEXT	Audience and time available: [brief] What I most want to get out of this: [one line] Topic: [e.g. primary open-angle glaucoma / nephrotic syndrome / brachial plexus] My level: [MBBS year / intern / PG resident in SPECIALTY] Exam I'm preparing for: [university theory / NEET-PG / UPSC CMS / MD-MS-DNB final / USMLE / PLAB] Standard textbook I use: [name and edition] Time I can spend on this topic: [e.g. 2 hours]
TASK	1. Definition and classification. 2. Aetiology and risk factors. 3. Pathophysiology, with the key mechanisms as a flow. 4. Clinical features: symptoms, signs, and grading or staging scales where applicable. 5. Investigations, with interpretation pearls. 6. Management: medical, surgical and procedural, in a stepwise format. 7. Complications and their management. 8. High-yield exam points, previous-year question themes, and commonly asked viva questions with model answers. 9. A one-line "if you remember only one thing" summary.
RULES	Keep facts consistent with standard textbooks and current guidelines, and mark any drug dose, cut-off or guideline-dependent statement as [VERIFY FROM TEXTBOOK / GUIDELINE], because exam answers must match the standard source. Flag anything that is a favourite examiner question. Say so where textbooks disagree.
OUTPUT	Headed sections with bullets and tables, then the high-yield box and the viva questions with answers.
TWEAK	End with 5 active-recall questions I should answer tomorrow without looking at the notes.

How to get more out of it

- › Replace [TOPIC] with anything: retinal detachment, uveitis, heart failure, brachial plexus.
- › Cross-check the output against your standard textbook once, then revise only from this note.
- › Answer the 5 recall questions the next day. That's what turns reading into remembering.
- › → NEXT #702 A personal tutor that checks my understanding

A personal tutor that checks my understanding

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT

When a topic feels confusing and you want to understand it, not just read it.

WHY

Reading explanations feels like learning but often isn't. A tutor that explains in small parts, asks you a question after each part, and moves on only when you answer correctly forces real understanding, and shows you exactly where your gaps are.

COPY THE PROMPT BELOW

PERSONA	You are an expert, patient tutor in [SUBJECT] who teaches using the Socratic method. You explain simply, use real-world and clinical analogies, and never move ahead until the learner shows understanding.
MY CONTEXT	Topic: [TOPIC] My level and what I already know: [brief] Exam or purpose: [university exam / NEET-PG / UPSC CMS / residency exam / clinical work] Where I usually get confused: [brief or "not sure"]
TASK	1. Break the topic into 5–7 small, logical parts, and show me the list first. 2. Teach one part at a time in simple language, with an analogy and a clinical example. 3. After each part, ask me one question that tests understanding, not recall. 4. Wait for my answer. If it's correct, confirm it and add one deeper point; if it's wrong, explain the misconception differently and ask another question. 5. Move to the next part only when I answer correctly. 6. At the end, summarise the topic in 5 bullet points. 7. Give me 3 exam-style questions to test myself, with answers hidden until I attempt them.
RULES	Ask only one question at a time and wait for my reply, because the learning happens in my answering. Keep facts accurate to standard textbooks and mark guideline-dependent details [VERIFY]. Don't give me the answers to your check questions before I try.
OUTPUT	Conversational: one part and one question per turn, then the final summary and exam questions.
TWEAK	At the end, tell me which part I struggled with most and suggest one resource type to strengthen it.

How to get more out of it

- › Answer honestly, including "I don't know". The tutor adapts to your answers.
- › Ask "explain it differently" whenever an explanation doesn't click.
- › Revisit the part you struggled with after 2 days. Spacing makes it stick.
- › → **NEXT #707** Learn a topic in four layers

Build a custom tutor that answers only from my notes

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT

When setting up a Custom GPT (ChatGPT), a Claude Project or a Gemini Gem with your own notes and textbooks.

WHY

A general chatbot mixes your standard textbook with random internet content. A custom tutor instructed to answer only from your uploaded material, cite the file and page, and say "not in your notes" when it doesn't know gives exam-consistent answers you can trust and check.

COPY THE PROMPT BELOW

PERSONA	You are an expert at configuring AI study assistants for medical exams. You write system instructions that keep the assistant grounded in the uploaded files and in exam style.
MY CONTEXT	Tool: [ChatGPT Custom GPT / Claude Project / Gemini Gem] Exam: [NEET-PG / UPSC CMS / MD-MS-DNB final / university exam] Files I will upload (textbooks, notes, PYQs): [list] How I want to use it: [Q&A / viva drill / MCQ practice / revision summaries]
TASK	1. Write the complete system instructions for the tutor: answer ONLY from the uploaded files; if the answer isn't in the files, reply "Not in your uploaded notes. Verify from a textbook."; always cite the file name and page or section; use exam-style language; highlight high-yield facts and previous-year traps. 2. Add modes the tutor can switch between on command: "Explain", "Viva", "MCQ", "Summary" and "Compare", each with its rules. 3. Add instructions for handling conflicting information between files. 4. Write 5 starter prompts I can use daily. 5. Explain how to test the tutor on 5 questions whose answers I already know, to check its grounding.
RULES	Keep the instructions within typical length limits for the tool and mark tool-specific settings as [CHECK CURRENT SETTINGS], because platform features change often. Upload only material I'm allowed to use.
OUTPUT	The system instructions in a code block, the mode definitions, the conflict rule, the starter prompts, then the testing procedure.
TWEAK	End with the 3 warning signs that the tutor is answering from outside my files, and what to do.

How to get more out of it

- Test the tutor with a question you know isn't in your notes. It should say so.
- Keep one tutor per subject. Mixed subjects confuse retrieval.
- Upload your previous-year papers too, so it can link topics to past questions.
- → NEXT #704 Study a chapter with NotebookLM

Study a chapter with NotebookLM

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT

When you have a chapter, notes or a set of PDFs and want grounded summaries, questions and audio revision.

WHY

NotebookLM answers only from the sources you upload and shows exactly where each answer came from. Used with the right questions and a customised Audio Overview, it turns a dense chapter into structured notes, self-tests and a podcast for your commute.

COPY THE PROMPT BELOW

PERSONA	You are a study coach who has built efficient NotebookLM workflows for medical exam preparation.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Sources I'll upload: [chapter / notes / guideline PDFs] Topic and exam: [e.g. hepatitis B, NEET-PG] Time available: [e.g. 90 minutes]
TASK	1. Give a step-by-step workflow: upload, first-pass summary, structured questions, self-test, audio overview and review. 2. Write 12 questions to ask NotebookLM that cover definitions, classification, mechanisms, diagnosis, management, complications and exam traps. 3. Write the Audio Overview customisation: "Generate an Audio Overview of this chapter for a [EXAM] aspirant. Cover only high-yield points: definitions, classifications, diagnostic criteria, drug doses, side effects and the 5 most-asked previous-year themes. Skip history and rare exceptions." 4. Write prompts to generate a study guide, a comparison table and 15 self-test questions from the sources. 5. Explain how to check each answer using NotebookLM's citations. 6. Suggest how to organise notebooks (one per subject or system) and name sources so that citations stay easy to follow. 6. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Phrase everything so that answers come only from my sources, and tell me to verify drug doses against the source, because grounding is NotebookLM's strength only if I use it.
OUTPUT	The workflow, the question list, the audio instruction in a code block, the generation prompts, then the verification steps.
TWEAK	End with a 20-minute review routine for the next day using the same notebook.

How to get more out of it

- › Listen to the Audio Overview on your commute, then answer the self-test questions.
- › Click the citation numbers. They take you to the exact passage, and that is your verification.
- › One notebook per subject or system keeps the sources focused.
- › → **NEXT #703** Build a custom tutor that answers only from my notes

Understand a mechanism from first principles

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT

When you have memorised something but can't explain why it happens.

WHY

Mechanisms learned from first principles don't need to be memorised: you can reconstruct them in the exam hall. "Why" chains, from a clinical feature back to physiology, turn isolated facts into understanding that survives pressure and tricky questions.

COPY THE PROMPT BELOW

PERSONA	You are a physiology-minded clinician-teacher who explains every clinical fact by asking "why" until reaching basic principles.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The fact or phenomenon I want to understand: [e.g. why hypokalaemia causes muscle weakness, why the JVP rises in right heart failure] My level: [brief] The basic science I'm comfortable with: [brief]
TASK	1. Start from the clinical fact and build a "why" chain of 4–6 steps down to the basic principle. 2. Explain each step in plain language with a simple analogy. 3. Draw the chain as a flow diagram in text, with arrows between the steps. 4. Show how the same principle explains 2–3 other clinical facts. 5. Give 3 "predict the effect" questions that test whether I can apply the principle. 6. Link the principle to one treatment and explain why the treatment works. 7. Point out one common exam question that this principle answers, and show the answer built from the chain. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep each step physiologically accurate and mark any simplification as "simplified", because oversimplified mechanisms cause errors in harder questions. Mark anything textbook-dependent [VERIFY].
OUTPUT	The why-chain, the explanations, the flow diagram, the connected facts, the prediction questions, then the treatment link.
TWEAK	End with a one-sentence version of the principle I can say in a viva.

How to get more out of it

- › Try to answer each "why" yourself before reading the explanation.
- › One principle often explains ten facts. Find the principle and you save hours.
- › Draw the chain from memory the next day.
- › → NEXT #706 One topic across all subjects: vertical integration

One topic across all subjects: vertical integration

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT

When you want to see how a topic connects across anatomy, physiology, pathology, pharmacology, microbiology and clinical medicine.

WHY

Exams, especially NEET-PG and clinical vivas, increasingly test integration: a question about a disease may ask about its anatomy, mechanism or drug. Studying one topic vertically across subjects builds connected knowledge that is easier to recall.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator who designs integrated, organ-system teaching across preclinical, paraclinical and clinical subjects.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Topic or disease: [e.g. myocardial infarction / tuberculosis / diabetes mellitus] My year and exam: [brief] Subjects to cover: [all / list]
TASK	1. Build an integration map: for each subject (anatomy, physiology, biochemistry, pathology, microbiology, pharmacology, community medicine, forensic medicine if relevant, and clinical medicine or surgery), list the 3–5 high-yield points about this topic. 2. Show the links between subjects (e.g. coronary anatomy → infarct territory → ECG leads → complications). 3. Write 5 integrated questions that cross at least 2 subjects each, with answers. 4. Suggest a one-page visual map of the connections, with the disease in the centre. 5. Mark the facts most often asked in NEET-PG and UPSC CMS style questions, as recurring themes, not claimed statistics. 6. Suggest the order in which to revise the subjects for this topic, and how long each should take. 6. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep facts consistent with standard textbooks and mark doses and cut-offs [VERIFY], because integrated questions punish small factual errors.
OUTPUT	The integration table by subject, the link list, the integrated questions with answers, the map description, then the high-yield marks.
TWEAK	End with the 3 cross-subject links students most often miss.

How to get more out of it

- › Use this for the 20 most important diseases. It covers a surprising share of the syllabus.
- › Draw the map by hand. It sticks better than a printed one.
- › Revise the integrated questions before mock tests.
- › → NEXT #701 Structured topic master for any subject

707 Learn a topic in four layers

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When a topic is new or feels overwhelming.

PERSONA	You are a teacher who builds understanding in layers, from simple to advanced.
MY CONTEXT	Topic: [TOPIC] My level: [brief] Exam: [brief]
TASK	Teach me the topic in 4 layers: 1. A one-sentence explanation. 2. A beginner explanation (about 100 words). 3. The underlying mechanism (about 150 words). 4. An advanced, exam-level explanation with nuances and exceptions. Define new terms as they appear, use the same clinical example throughout, and end each layer with one check question.
RULES	Keep each layer accurate, labelling simplifications as such, and mark guideline-dependent facts [VERIFY], because early simplifications must not become later errors.
OUTPUT	Four labelled layers, each with its check question.

PRO TIP Stop at the layer your exam needs. Most MBBS exams need layer 3; residency exams need layer 4.

TRY IT WITH Topic: the renin-angiotensin-aldosterone system.

→ **NEXT #708** Explain it back and find my gaps (Feynman technique)

708 Explain it back and find my gaps (Feynman technique)

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT After studying a topic, to test whether you really understand it.

PERSONA	You are a demanding but kind examiner who uses the Feynman technique.
MY CONTEXT	Topic: [TOPIC] My explanation in my own words (typed or voice-to-text): [PASTE MY EXPLANATION] My level: [brief]
TASK	1. Identify what I explained correctly. 2. Identify errors, gaps, vague phrases and jargon I used without understanding. 3. Ask me 3 follow-up questions targeting the gaps. 4. After I answer, give a corrected, simple explanation.
RULES	Be specific about each gap, quoting my words, and don't simply rewrite everything for me, because the goal is to find what I don't understand.
OUTPUT	The correct points, a gaps table (my words, problem), the follow-up questions, then (after my answers) the corrected explanation.

PRO TIP If you can't explain it to a 12-year-old, you don't understand it yet. That's the Feynman test.

TRY IT WITH My explanation of why loop diuretics cause hypokalaemia.

→ **NEXT #702** A personal tutor that checks my understanding

709 Learn a hard concept through analogies

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When a concept won't stick through textbook explanations.

PERSONA	You are a gifted medical teacher known for memorable analogies.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Concept: [CONCEPT] My level: [brief]
TASK	1. Give 3 different everyday analogies for the concept. 2. For each, map the parts of the analogy to the parts of the concept. 3. State where each analogy breaks down. 4. Recommend the best analogy and give one exam question it helps answer, with a short model answer.
RULES	Make each analogy's limitations explicit, because an analogy taken too far teaches the wrong physiology.
OUTPUT	Three analogies with mapping tables and limitations, then the recommendation.

PRO TIP Draw your chosen analogy. Visual analogies stick best.

TRY IT WITH Concept: oxygen-haemoglobin dissociation curve shifts.

→ **NEXT #707** Learn a topic in four layers

710 Surface features and deeper mechanisms (iceberg)

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When you know a disease's features but not why they occur.

PERSONA	You are a clinician-teacher who uses the iceberg model to connect signs to mechanisms.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Disease or syndrome: [name] My level: [brief]
TASK	1. Above the surface: list the visible symptoms, signs and test findings. 2. Just below: the patterns and physiological changes that produce them. 3. Deep below: the underlying mechanisms and causes. 4. Link each surface feature to its mechanism with an arrow. 5. Distinguish established mechanisms from hypotheses.
RULES	Don't invent mechanisms to fill the diagram; say where the mechanism is unclear, because false certainty is harder to unlearn.
OUTPUT	A three-level iceberg as a table, then the feature-to-mechanism links.

PRO TIP Examiners love "explain why..." questions. The deep layer answers them.

TRY IT WITH Disease: nephrotic syndrome.

→ **NEXT #705** Understand a mechanism from first principles

711 Learn a process as a flowchart

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT For pathways, algorithms, cascades and step-by-step processes.

PERSONA	You are a teacher who converts processes into clear flowcharts.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Process: [e.g. coagulation cascade, approach to anaemia, steps of an ABG] My level: [brief]
TASK	1. Draw the process as a text flowchart with a start, steps, decision points (yes/no) and outcomes. 2. Label every branch. 3. Add the clinical relevance at the key steps. 4. Give a numbered-outline version as a text fallback. 5. Give 3 questions that test the flowchart.
RULES	Keep the steps accurate to standard sources and mark guideline thresholds [VERIFY], because flowcharts are often reproduced in exam answers.
OUTPUT	The flowchart, the outline, then the questions.

PRO TIP Redraw the flowchart from memory. The step you forget is the one to revise.

TRY IT WITH Approach to a patient with metabolic acidosis.

→ **NEXT #713** Classifications as a tree

712 Compare options in a decision matrix

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When you need to choose between treatments, tests or approaches, as exams often ask.

PERSONA	You are a clinical pharmacologist and teacher who builds comparison matrices.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The options to compare: [e.g. antiepileptic drugs, imaging modalities, antihypertensives in special populations] Criteria: [list or "suggest"] Exam context: [brief]
TASK	1. Build a decision matrix with the options as rows and the criteria as columns. 2. Assess each cell briefly, stating assumptions. 3. State which option fits which clinical situation. 4. Add 3 exam-style "which is best for..." questions with answers.
RULES	Don't invent scores; use qualitative assessments and mark doses and specifics [VERIFY], because a false precision score misleads.
OUTPUT	The matrix, the situation guide, then the questions.

PRO TIP "Drug of choice in pregnancy" and "drug to avoid in renal failure" columns answer many exam questions.

TRY IT WITH First-line antihypertensives in diabetes, CKD, pregnancy and heart failure.

→ **NEXT #723** Learn any drug with one framework

713 Classifications as a tree

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT For topics with complex classifications (drugs, tumours, anaemias, glomerular diseases).

PERSONA	You are a teacher who organises knowledge into hierarchical trees.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Topic: [TOPIC] Classification basis (if a standard one exists): [brief]
TASK	1. Organise the topic as a hierarchy: main concept → categories → subcategories → examples. 2. Use one consistent classification rule at each level. 3. Flag overlaps rather than forcing items into single branches. 4. Add one distinguishing feature per category. 5. Give a text-tree version for quick revision.
RULES	Follow the standard textbook classification and mark any that differs between sources [CHECK TEXTBOOK], because exams expect the standard version.
OUTPUT	The tree, the distinguishing features, then the text-tree.

PRO TIP Classifications are frequent 3-mark questions. Practise writing them from memory.

TRY IT WITH Classification of anaemias by red-cell indices.

→ **NEXT #711** Learn a process as a flowchart

714 A learning roadmap from my level to my goal

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When starting a large subject or skill, such as ECGs, radiology or pharmacology.

PERSONA	You are a curriculum designer who builds realistic learning roadmaps.
MY CONTEXT	Subject or skill: [e.g. ECG interpretation] Current level: [brief] Goal: [e.g. confidently interpret common ECGs for the NEET-PG image-based questions] Time available: [weeks and hours per week]
TASK	1. Break the path into 4–6 stages with prerequisites. 2. For each stage: learning goals, resources by type, practice tasks and a measurable checkpoint. 3. Estimate the time per stage. 4. State whether the scope exceeds the available time, and what to cut if so.
RULES	Keep the plan realistic for my stated time, because over-ambitious roadmaps are abandoned by week 2.
OUTPUT	A stage table (stage, goals, practice, checkpoint, time), then the feasibility note.

PRO TIP A checkpoint you can test yourself on (e.g. "interpret 20 ECGs at 80% accuracy") beats "finish chapter 3".

TRY IT WITH Learning chest X-ray interpretation in 4 weeks, 5 hours a week.

→ **NEXT #751** A personalised day-by-day study plan

715 Connect this topic to what I already know

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When a new topic feels isolated from everything else you've learned.

PERSONA	You are a teacher who builds knowledge by connecting new ideas to old ones.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] New topic: [TOPIC] Topics I already know well: [list]
TASK	1. Identify 5 links between the new topic and what I know (shared mechanisms, similar presentations, related drugs). 2. Explain each link in 2 sentences. 3. Identify one common confusion between the new topic and a related known topic. 4. Give 3 questions that require using both topics together.
RULES	Keep every link accurate, and flag superficial similarities that could mislead, because false connections cause exam errors.
OUTPUT	A link table, the confusion note, then the questions.

PRO TIP Knowledge sticks to knowledge. Always ask "what does this remind me of?"

TRY IT WITH New topic: SIADH. Known topics: diabetes insipidus and hyponatraemia.

→ **NEXT #713** Classifications as a tree

716 Learn through a worked example

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT For problem-solving topics: acid-base, dosing calculations, ECG axes, statistics, fluid prescriptions.

PERSONA	You are a tutor who teaches problem-solving through clear worked examples.
MY CONTEXT	Problem type: [e.g. ABG interpretation, paediatric fluid calculation] My level: [brief]
TASK	1. Show a fully worked example, step by step, explaining the reasoning at each step. 2. Show a second example with a twist. 3. Give me a third problem to solve alone, and wait for my answer. 4. Check my answer step by step and correct any errors.
RULES	Make every example internally consistent (numbers that are physiologically possible) and mark formulas and normal ranges [VERIFY], because inconsistent examples confuse learners.
OUTPUT	Two worked examples, a practice problem, then feedback after my attempt.

PRO TIP Cover the steps and try each one yourself before reading it.

TRY IT WITH Calculating maintenance IV fluids for a 22-kg child using the Holliday-Segar method.

→ **NEXT #708** Explain it back and find my gaps (Feynman technique)

717 Common misconceptions in a topic

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT Before exams, to avoid the traps most students fall into.

PERSONA	You are an experienced examiner who has marked thousands of answers and knows the common errors.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Topic: [TOPIC] Exam: [brief]
TASK	1. List 8 common misconceptions or confusions about this topic. 2. For each, state the correct understanding and why the misconception arises. 3. Give an exam-style question that exploits each misconception, with the answer.
RULES	Base the corrections on standard textbooks, and mark disputed or guideline-dependent points [VERIFY], because a "corrected" misconception must itself be right.
OUTPUT	A misconception table (misconception, truth, why it arises), then the trap questions.

PRO TIP Trap questions in MCQ exams are built on exactly these misconceptions.

TRY IT WITH Topic: diuretics and their effects on electrolytes.

→ **NEXT #701** Structured topic master for any subject

718 Sort a topic into high- and low-yield

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When time is short and you need to prioritise.

PERSONA	You are an exam strategist who knows what is repeatedly tested.
MY CONTEXT	Topic or chapter contents: [PASTE LIST OR UPLOAD] Exam: [NEET-PG / UPSC CMS / university / residency exam] Time available: [hours]
TASK	1. Sort the content into must-know, should-know and nice-to-know for my exam. 2. Justify the must-know items (commonly tested themes, clinical importance, basis for other topics). 3. Suggest a study order within my time. 4. List the recurring question patterns for the must-know items.
RULES	Describe recurring exam themes rather than claiming specific question frequencies, and mark claims about past papers [VERIFY WITH PYQs], because invented statistics about exams mislead.
OUTPUT	The three-tier sort, the justifications, the study order, then the patterns.

PRO TIP Check the sort against your previous-year papers. They are the most reliable guide.

TRY IT WITH The chapter on thyroid disorders, for NEET-PG with 3 hours available.

→ **NEXT #783** Analyse previous-year questions for patterns

719 Turn a textbook chapter into structured notes

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When you have a chapter to study and want concise, exam-ready notes.

PERSONA	You are a note-making expert who condenses chapters without losing exam content.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The chapter (upload or paste the sections): [UPLOAD / PASTE] Exam: [brief] Target length: [e.g. 2 pages]
TASK	1. Extract the key concepts, definitions, classifications, tables and clinical points. 2. Organise them under clear headings with bullets and tables. 3. Highlight exam-relevant facts. 4. Add 10 recall questions at the end.
RULES	Use only the chapter's content, and mark anything I should check as [VERIFY IN CHAPTER], because notes must match the source I'll be examined on.
OUTPUT	Structured notes within the target length, then the recall questions.

PRO TIP Make notes after one full reading, not during it. You'll know what matters.

TRY IT WITH A 30-page chapter on acute kidney injury.

→ **NEXT #704** Study a chapter with NotebookLM

720 Clean up my lecture notes

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT

USE IT After a lecture, when your notes are rough and incomplete.

PERSONA	You are a study assistant who organises lecture notes clearly.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] My rough notes: [PASTE] The lecture topic and the standard textbook: [brief]
TASK	1. Organise the notes under logical headings. 2. Mark gaps where the notes are incomplete, as [FILL FROM TEXTBOOK], without filling them from memory. 3. Convert lists into tables where helpful. 4. Add 5 questions to check understanding.
RULES	Keep my content and mark gaps rather than inventing material, because my notes must reflect what was taught and what the textbook says.
OUTPUT	The organised notes, the gap list, then the questions.

PRO TIP Clean up notes the same day. After a week, you won't remember what your shorthand meant.

TRY IT WITH Rough notes from a lecture on congenital heart disease.

→ **NEXT #719** Turn a textbook chapter into structured notes

721 Notes from a video lecture transcript

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When you learn from online video lectures and want notes from the transcript.

PERSONA	You are a note-taker who extracts structured learning from lectures.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The transcript (paste or upload): [PASTE] Topic and exam: [brief]
TASK	1. Extract the key points in the order taught. 2. Organise them into headings, bullets and tables. 3. Capture the teacher's mnemonics and "exam tips". 4. Flag any statement that sounds doubtful or outdated, as [VERIFY]. 5. Add 8 recall questions.
RULES	Use only what the lecture said and flag doubtful claims, because video lectures sometimes contain errors or outdated guidance.
OUTPUT	Structured notes, the tips and mnemonics, the flags, then the questions.

PRO TIP Watch at normal speed for new topics. Speed up only for revision.

TRY IT WITH A 60-minute video lecture on antiepileptic drugs.

→ **NEXT #719** Turn a textbook chapter into structured notes

722 Learn a clinical guideline for exams

STANDARD

3.1 LEARNING ANY TOPIC · PG RESIDENT / MBBS INTERN

USE IT When exams ask about a current guideline (GOLD, ADA, WHO, national programme guidelines).

PERSONA	You are a clinician-educator who extracts the exam-relevant parts of guidelines.
MY CONTEXT	The guideline (upload or name and year): [UPLOAD / NAME] Exam: [brief]
TASK	1. Summarise the key definitions, classifications and criteria. 2. Summarise the management recommendations as a stepwise table. 3. Highlight what changed from the previous version, if known. 4. List the numbers to remember (thresholds, targets, doses) with [VERIFY]. 5. Write 8 exam-style questions with answers.
RULES	Quote from the uploaded guideline where possible, and mark anything from memory [VERIFY], because exams test the current version.
OUTPUT	The definitions, the management table, the changes, the numbers list, then the questions.

PRO TIP Exams often lag guideline updates. Check which version your exam expects.

TRY IT WITH The current GOLD report for COPD, for MD Respiratory Medicine exams.

→ **NEXT #701** Structured topic master for any subject

723 Learn any drug with one framework

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT For pharmacology revision, drug by drug.

PERSONA	You are a clinical pharmacologist who teaches drugs with a consistent framework.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Drug or drug class: [name] Exam: [brief]
TASK	1. Cover: class, mechanism of action, pharmacokinetics (key points only), indications, dose and route [VERIFY], adverse effects (common and serious), contraindications, interactions, monitoring, and special populations. 2. Highlight the exam favourites (e.g. drug of choice, unique side effect). 3. Add a one-line clinical pearl. 4. Give 5 MCQ-style questions.
RULES	Mark every dose [VERIFY FROM FORMULARY / TEXTBOOK], because pharmacology questions test exact details.
OUTPUT	A framework table, the favourites, the pearl, then the questions.

PRO TIP Learn drugs in classes, then the exceptions. The exceptions are what exams ask.

TRY IT WITH Amiodarone.

→ **NEXT #712** Compare options in a decision matrix

724 A standard template for any disease

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT

USE IT For consistent notes on every disease in clinical subjects.

PERSONA	You are a medical educator who teaches students to organise disease knowledge consistently.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Disease: [name] Subject and exam: [brief]
TASK	Fill the standard template: definition; epidemiology (with [VERIFY] for numbers); aetiology and risk factors; pathogenesis; pathology; clinical features; complications; investigations; diagnosis (criteria); differential diagnosis; management; prognosis; prevention. Then highlight the 5 most important points, and suggest one 15-mark question this template would answer.
RULES	Keep facts consistent with standard textbooks and mark numbers and doses [VERIFY], because template notes are revised many times.
OUTPUT	The filled template, then the top 5 points.

PRO TIP Using the same template for every disease makes long answers almost automatic.

TRY IT WITH Rheumatic fever.

→ **NEXT #701** Structured topic master for any subject

725 Learn an anatomy topic that sticks

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT (SURGICAL)

USE IT For regional anatomy, neuroanatomy and applied anatomy.

PERSONA	You are an anatomy teacher who makes structures three-dimensional and clinically relevant.
MY CONTEXT	Structure or region: [e.g. inguinal canal, facial nerve, brachial plexus] Exam: [brief]
TASK	1. Describe the structure in a logical order: location, boundaries and relations, contents, blood supply, nerve supply and lymphatics. 2. Describe a diagram to draw, with labels. 3. Give the applied anatomy: clinical conditions and surgical relevance. 4. Give a memory aid where useful. 5. Give 5 viva questions with answers.
RULES	Keep descriptions accurate to standard anatomy textbooks and mark uncertain details [VERIFY], because anatomy vivas test precision.
OUTPUT	The structured description, the diagram description, the applied anatomy, the memory aid, then the viva questions.

PRO TIP Draw it. Anatomy you have drawn is anatomy you remember.

TRY IT WITH The facial nerve: course, branches and lesions.

→ **NEXT #711** Learn a process as a flowchart

726 Understand physiology graphs and curves

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT For graphs such as the oxygen dissociation curve, Frank-Starling, pressure-volume loops and action potentials.

PERSONA	You are a physiology teacher who explains graphs axis by axis.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Graph: [name] My difficulty: [brief]
TASK	1. Explain the axes and units. 2. Explain the normal curve's shape, and why it has that shape. 3. Explain the shifts or changes and their causes. 4. Link each change to clinical situations. 5. Describe how to draw it in an exam. 6. Give 3 "predict the shift" questions.
RULES	Keep the physiology accurate and label any simplification, because graph questions test precise understanding.
OUTPUT	The axis explanation, the shape, the shifts table, the clinical links, the drawing guide, then the questions.

PRO TIP Draw the normal curve first, then the shifted one in another colour.

TRY IT WITH The pressure-volume loop of the left ventricle.

→ **NEXT #705** Understand a mechanism from first principles

727 Learn to read a type of clinical image

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT For image-based questions (X-rays, ECGs, instruments, spotters, clinical photos).

PERSONA	You are a teacher who trains students to read clinical images systematically.
MY CONTEXT	Image type: [e.g. chest X-ray, ECG, CT head, clinical photographs] Exam: [NEET-PG image questions / university spotters / residency practicals]
TASK	1. Teach a systematic approach for this image type in 5–7 steps. 2. List the 15 most commonly tested findings or diagnoses for this image type, with the key feature of each. 3. Describe practice examples of graded difficulty (as text descriptions). 4. List the common pitfalls.
RULES	Describe findings conservatively and accurately, and suggest verified image sources for practice (standard atlases, educational sites), because AI descriptions can't replace looking at real images.
OUTPUT	The approach, the findings table, the practice examples, then the pitfalls.

PRO TIP Look at 10 real images a day. Pattern recognition only comes from volume.

TRY IT WITH Chest X-ray patterns for NEET-PG.

→ **NEXT #714** A learning roadmap from my level to my goal

728 Normal values and interpretation

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT For learning lab reference ranges and what abnormal values mean.

PERSONA	You are a clinical pathologist who teaches lab interpretation.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Tests: [e.g. LFT, RFT, CBC, electrolytes, thyroid profile] Exam: [brief]
TASK	1. List the reference ranges with units, marked [VERIFY WITH YOUR LAB AND TEXTBOOK]. 2. For each test, list the causes of high and low values, grouped logically. 3. Give pattern-recognition tips (e.g. hepatocellular versus cholestatic LFT patterns). 4. Give 5 interpretation questions with answers.
RULES	Note that reference ranges vary between labs and textbooks, because exams usually follow one standard source.
OUTPUT	A reference table, a causes table, the pattern tips, then the questions.

PRO TIP Learn the patterns, not only the numbers. Exams ask "what does this pattern suggest?".

TRY IT WITH Liver function tests: hepatocellular, cholestatic and mixed patterns.

→ **NEXT #716** Learn through a worked example

729 Learn a microorganism with one template

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT

USE IT For microbiology revision, organism by organism.

PERSONA	You are a microbiology teacher who organises organisms consistently.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Organism: [name] Exam: [brief]
TASK	Fill the template: morphology and staining, culture characteristics, virulence factors, diseases caused, transmission, laboratory diagnosis (specimen, stain, culture, serology, molecular tests), treatment [VERIFY], and prevention (including vaccines). Then compare it briefly with the organism it is most often confused with. Add the exam favourites, a one-line clinical scenario, and 3 quick recall questions.
RULES	Keep the details consistent with standard microbiology textbooks and mark treatment specifics [VERIFY], because resistance patterns and recommendations change.
OUTPUT	The filled template, then the favourites and the scenario.

PRO TIP Group organisms by the disease they cause as well. Clinical questions start from the disease.

TRY IT WITH Staphylococcus aureus.

→ **NEXT #724** A standard template for any disease

730 Simplify a biochemistry pathway

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT

USE IT For metabolic pathways and their disorders.

PERSONA	You are a biochemistry teacher who makes pathways clinically meaningful.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Pathway: [e.g. urea cycle, glycogen metabolism, haem synthesis] Exam: [brief]
TASK	1. Show the pathway as a simplified flow with only the key steps and enzymes. 2. Mark the rate-limiting step and the regulation. 3. Link the enzyme deficiencies to disorders and their features. 4. Give the clinical and exam pearls. 5. Give 5 questions.
RULES	Keep the pathway accurate while simplifying, and label what has been omitted, because exams can ask about any step.
OUTPUT	The simplified flow, the regulation, the disorders table, the pearls, then the questions.

PRO TIP Learn the disorders from the enzyme blocks. It's how questions are written.

TRY IT WITH Haem synthesis and the porphyrias.

→ **NEXT #711** Learn a process as a flowchart

731 Learn national health programmes (community medicine)

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / UPSC CMS ASPIRANT

USE IT For community medicine and preventive and social medicine, heavily tested in UPSC CMS and NEET-PG.

PERSONA	You are a community medicine teacher who knows Indian national health programmes.
MY CONTEXT	Programme: [e.g. National TB Elimination Programme, National Immunisation Schedule, NVBDCP] Exam: [UPSC CMS / NEET-PG / university]
TASK	1. Summarise the programme: objectives, targets, strategy, key components and case definitions. 2. List the numbers to remember (targets, schedules, doses), each marked [VERIFY FROM CURRENT PROGRAMME GUIDELINES]. 3. Note recent changes, marked [VERIFY]. 4. Give 8 exam-style questions with answers.
RULES	Mark every programme detail for verification against the latest official document, because programmes are renamed and revised frequently and exams test the current version.
OUTPUT	The summary, the numbers table, the changes note, then the questions.

PRO TIP Keep a single notebook of programme numbers and update it from the official sources before exams.

TRY IT WITH The National Immunisation Schedule.

→ **NEXT #718** Sort a topic into high- and low-yield

732 Make my own questions from my notes

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT After making notes, to turn them into active-recall material.

PERSONA	You are an assessment writer who creates questions at different levels.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] My notes: [PASTE] Exam format: [MCQ / short answer / viva]
TASK	1. Write 15 questions from my notes: 5 recall, 5 understanding, 5 application. 2. Give the answers separately. 3. Mark which questions suit flashcards, and which suit a viva.
RULES	Base every question and answer only on my notes, and flag any note content that seems wrong, as [VERIFY], because questions built on errors reinforce them.
OUTPUT	The questions by level, then the answer key and flashcard marks.

PRO TIP Writing your own questions is itself one of the best ways to learn.

TRY IT WITH My notes on acute pancreatitis.

→ **NEXT #971** High-yield flashcards for Anki or Notion

733 Design an interleaved practice session

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT When you have studied related topics separately and need to learn to tell them apart.

PERSONA	You are a learning-science coach who applies interleaving and retrieval practice.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Related topics studied: [e.g. types of shock, causes of jaundice, anaemias] Time: [minutes]
TASK	1. Explain briefly why interleaving helps discrimination. 2. Build a mixed practice set of 12 questions or cases that alternate between the topics. 3. Include "which one is this?" discrimination questions. 4. Give answers with the distinguishing feature highlighted.
RULES	Keep the cases realistic and accurate, and mark specifics [VERIFY], because discrimination practice relies on precise features.
OUTPUT	The explanation, the mixed set, then the answers.

PRO TIP Interleaving feels harder than blocked practice. That difficulty is what makes it work.

TRY IT WITH Mixed practice on the types of shock.

→ **NEXT #755** A spaced-repetition revision schedule

734 Diagnose my knowledge gaps

STANDARD

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

USE IT At the start of revising a subject, to know where to focus.

PERSONA	You are a diagnostic tutor who assesses knowledge efficiently.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Subject or system: [e.g. cardiology] Exam: [brief]
TASK	1. Give me a diagnostic quiz of 15 questions spanning the major topics, one at a time. 2. After each answer, note whether I was right, without a long explanation. 3. At the end, map my strengths and gaps by topic. 4. Recommend a study order that prioritises the gaps.
RULES	Ask one question at a time and wait for my answer, because the diagnosis depends on my unaided responses.
OUTPUT	One question per turn, then the gap map and the study order.

PRO TIP Take the diagnostic quiz before studying. It tells you what you don't need to re-read.

TRY IT WITH A cardiology diagnostic quiz for NEET-PG.

→ **NEXT #751** A personalised day-by-day study plan

735 Explain it like I'm a second-year student

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT

Act as a friendly senior teacher. Explain [CONCEPT] as you would to a second-year MBBS student, in under 150 words, with one analogy and one clinical example, then ask me one question to check that I understood. Flag anything you are unsure of so I can check it.

TRY IT WITH Concept: why ACE inhibitors cause a dry cough.

736 One-page summary of a topic

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as an exam-focused tutor. Summarise [TOPIC] for [EXAM] on one page: key definitions, one table, 5 must-know facts and 3 exam traps, marking doses and cut-offs [VERIFY], so I can revise it in 5 minutes. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: diabetic ketoacidosis for NEET-PG.

737 Ten must-know facts

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as an experienced examiner. List the 10 must-know facts about [TOPIC] for [EXAM], one line each, prioritised by importance, with [VERIFY] beside any number, dose or guideline-dependent statement. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: hepatitis B serology.

738 Ask "why" five times

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a Socratic physiology teacher. Starting from the fact "[CLINICAL FACT]", ask and answer "why?" 5 times, going one level deeper each time, until you reach a basic physiological principle, in simple language. Flag anything you are unsure of so I can check it.

TRY IT WITH Fact: patients with severe anaemia develop high-output heart failure.

739 Clear up two confusing terms

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a medical tutor. Explain the difference between [TERM A] and [TERM B] in a short comparison table (definition, mechanism, example, clinical significance), then give one sentence that helps me never confuse them again. Flag anything you are unsure of so I can check it.

TRY IT WITH Sensitivity versus specificity.

740 A real-world example of a concept

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT

Act as a clinical teacher. Give me 2 realistic clinical examples of [CONCEPT], as short de-identified scenarios of 2–3 lines each, and explain how each one demonstrates the concept. Flag anything you are unsure of so I can check it.

TRY IT WITH Concept: the Frank-Starling mechanism.

741 Why does this basic-science fact matter clinically?

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT

Act as a clinician who teaches basic science. Explain why [BASIC SCIENCE FACT] matters in clinical practice, with 3 clinical applications (a disease, a drug and a test), each in 1–2 lines, so that I remember it for exams and wards.

TRY IT WITH Fact: the recurrent laryngeal nerve loops under the arch of the aorta on the left.

742 A quick 5-question quiz

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a quizmaster for medical students. Give me 5 mixed-difficulty questions on [TOPIC] for [EXAM], one at a time, waiting for my answer each time, then give a score and a one-line correction for each wrong answer. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: antiepileptic drugs.

743 Define it, with an example

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT

Act as a textbook author. Give the standard definition of [TERM] as expected in university exams, explain it in one simpler sentence, and give one clinical example, marking the definition's source type as [CHECK STANDARD TEXTBOOK]. Flag anything you are unsure of so I can check it.

TRY IT WITH Term: pulsus paradoxus.

744 Clear up one specific confusion

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a patient tutor. I'm confused about [SPECIFIC CONFUSION]. Explain it step by step in under 150 words, point out exactly where students usually go wrong, and give me one question to confirm that I've understood. Flag anything you are unsure of so I can check it.

TRY IT WITH Why metabolic alkalosis occurs with vomiting but acidosis with diarrhoea.

745 The whole topic in 280 characters

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a concise medical educator. Summarise [TOPIC] in under 280 characters, capturing the definition, the key feature and the main treatment principle, as a revision one-liner I can remember. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: myasthenia gravis.

746 A diagram I can draw

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a medical illustrator and teacher. Describe step by step how to draw a simple, labelled exam diagram of [STRUCTURE OR PROCESS], in 6–8 drawing steps with a list of the labels, drawable in under 3 minutes in an exam.

TRY IT WITH A cross-section of the spinal cord showing the major tracts.

747 What should I study next?

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a study mentor. I've just finished [TOPIC] for [EXAM]. Suggest the 3 most logical topics to study next (those that build on it, are commonly tested with it, or are easily confused with it), with one line of reasoning each.

TRY IT WITH I've just finished heart failure for NEET-PG.

748 Is my understanding correct?

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as an examiner. Here is my understanding of [TOPIC], in 2–4 sentences: [MY STATEMENT]. Tell me whether it is correct, partly correct or wrong, quote the part that's wrong, and correct it in one sentence. Flag anything you are unsure of so I can check it.

TRY IT WITH "Digoxin is contraindicated in all patients with renal failure."

749 Translate the jargon

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT

Act as a friendly teacher. Rewrite this textbook passage, [PASTE PASSAGE], in plain language for a second-year student, defining each technical term in brackets the first time it appears, and keeping every fact accurate. Flag anything you are unsure of so I can check it.

TRY IT WITH A dense textbook paragraph on glomerular filtration and autoregulation.

750 Three questions the examiner will ask

QUICK

3.1 LEARNING ANY TOPIC · MBBS STUDENT / PG RESIDENT

Act as a university examiner in [SUBJECT]. Give the 3 questions you would most likely ask on [TOPIC] in a [theory / viva / practical] exam, with the key points a full-marks answer must contain for each. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: tetralogy of Fallot, in a paediatrics viva.

3.2

Study planning & revision

A plan you can actually follow beats a perfect plan you abandon in week two. Prioritise weak areas, revise in cycles, and test yourself relentlessly.

30

prompts

4

Master

17

Standard

9

Quick

#751 A personalised day-by-day study plan

#752 NEET-PG preparation strategy

#753 Plan for the MD / MS / DNB final exam

#754 UPSC CMS preparation strategy

#755 A spaced-repetition revision schedule

#756 The last 30 days before the exam

#757 The last 7 days before the exam

#758 I've fallen behind: recover my plan

#759 Weekly study review

#760 Which subject should I study first?

#761 Study around internship or residency duties

#762 Plan revision cycles (R1, R2, R3)

#763 Analyse a mock or grand test properly

#764 Rescue my weakest subject

#765 Am I heading for burnout?

#766 Design focused study sessions

#767 Run an effective study group

#768 Too many resources: choose and commit

#769 The night before and the exam day

#770 Sleep, memory and study performance

#771 Build a simple accountability tracker

#772 Plan my study day

#773 My three priorities today

#774 How long will this subject take?

#775 What should I revise today?

#776 Reframe a demotivating thought

#777 Fix my biggest distraction

#778 Set a goal for this study session

#779 Did I hit my weekly target?

#780 Study breaks that actually refresh

A personalised day-by-day study plan

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT

At the start of exam preparation, or whenever your current plan has fallen apart.

WHY

Generic toppers' timetables fail because your syllabus, weak areas, duty hours and energy are yours alone. A plan built around your real constraints, weighted towards weak areas, with revision and practice built in, is one you can actually follow.

COPY THE PROMPT BELOW

PERSONA	You are an experienced study coach for Indian medical exams who builds realistic, personalised plans and adjusts them honestly when students fall behind.
MY CONTEXT	Exam: [NEET-PG / UPSC CMS / MD-MS-DNB final / university exam / USMLE / PLAB / MRCP] Exam date and days left: [date / N days] Syllabus or subjects: [list] Strongest areas: [list] · Weakest areas: [list] Hours I can realistically study on duty days and on off days: [e.g. 3 / 8] Current stage: [not started / first reading done / revising] Resources I'm using (keep it to what I actually have): [list]
TASK	1. Check the feasibility: the total hours available against a realistic estimate of the hours needed, and say honestly whether it fits. 2. Divide the time into phases: first coverage, first revision, second revision with tests, and final revision. 3. Build a day-by-day plan for the first 2 weeks and a week-by-week plan for the rest, weighted towards my weak areas. 4. Mix theory with practice questions daily, and schedule grand tests or mock exams at intervals. 5. Include revision days, buffer days and at least one lighter day a week. 6. Include a daily 15-minute review of the previous day's topics. 7. Define weekly checkpoints (topics covered, test scores) and what to do if I miss them.
RULES	Base the plan on my stated hours rather than ideal hours, and don't claim official exam weightage or pattern details from memory; mark them [VERIFY WITH THE OFFICIAL NOTIFICATION], because exam patterns change. Ask me clarifying questions first if anything important is missing.
OUTPUT	The feasibility check, a phase table, the 2-week daily plan, the weekly plan, then the checkpoint rules.
TWEAK	End with the 3 most likely reasons this plan will fail for me, and how to prevent each.

How to get more out of it

- › Plan for 80% of your available hours. Life always takes the rest.
- › Weak areas get the most time early, when you have energy and time to fix them.
- › Review the plan every Sunday. Adjust it; don't abandon it.
- › → NEXT #755 A spaced-repetition revision schedule

NEET-PG preparation strategy

3.2 STUDY PLANNING & REVISION · MBBS FINAL YEAR / INTERN / NEET-PG ASPIRANT

USE IT

When planning NEET-PG preparation over several months.

WHY

NEET-PG rewards breadth, repeated revision and huge volumes of MCQ practice under time pressure. A strategy that plans the subjects, revision cycles, question banks and mock tests (and protects your internship and health) gives you the best return on every hour.

COPY THE PROMPT BELOW

PERSONA	You are a NEET-PG mentor who has guided many aspirants, known for realistic plans, careful use of question banks, and honesty about what works.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Months until the exam: [N] My status: [final year / internship with duties / dropper, full-time] Daily hours available (on duty and off duty): [numbers] Previous attempt score or mock score, if any: [brief] Strong and weak subjects: [lists] Resources I have (one video platform, one question bank, notes): [list]
TASK	1. Recommend a subject order and grouping (e.g. high-volume subjects and systems approach), with the reasoning. 2. Plan 3 revision cycles with their duration, shrinking with each cycle. 3. Plan daily MCQ practice targets and the review method (analysing wrong answers). 4. Plan grand tests or mock exams: frequency, and how to analyse each. 5. Plan image-based and recent-updates preparation. 6. Plan the last 30 days separately. 7. Build in a weekly off-half-day and health habits. 8. Explain how to adapt if mock scores stagnate.
RULES	Don't state official exam pattern, marks or subject weightage as fact; mark every such detail [VERIFY WITH THE CURRENT NBEMS NOTIFICATION], because the pattern is revised. Recommend using a small number of resources thoroughly rather than many superficially.
OUTPUT	The subject-order plan, the cycle timeline, the MCQ and mock-test plan, the last-30-days plan, then the adaptation rules.
TWEAK	End with the 5 mistakes NEET-PG aspirants most commonly make, and how this plan avoids them.

How to get more out of it

- › Analysing a grand test for as long as it took to write it is where the marks come from.
- › Fewer resources, revised more times, beats more resources read once.
- › Protect sleep in the final month. Tired recall is poor recall.
- › → NEXT #763 Analyse a mock or grand test properly

Plan for the MD / MS / DNB final exam

3.2 STUDY PLANNING & REVISION · PG RESIDENT (FINAL YEAR)

USE IT

6–9 months before your residency final exam (theory, practical and viva).

WHY

Residency finals test depth, clinical skills and presentation, all while you're working full duties and finishing a thesis. A plan that combines theory writing, clinical case practice and viva preparation with your duty roster is what gets residents through.

COPY THE PROMPT BELOW

PERSONA	You are a senior faculty member and examiner for MD, MS and DNB examinations who mentors final-year residents.
MY CONTEXT	Audience and time available: [brief] What I most want to get out of this: [one line] Specialty and exam type: [e.g. MD General Medicine university exam / DNB final] Months until the theory and practical exams: [N / N] Duty pattern (calls, OPD, ICU postings): [brief] Thesis status: [submitted / pending] Weak areas: [list] Exam format as I understand it (papers, long and short cases, viva, OSCE, spotters): [brief, marked "to confirm"]
TASK	1. Map the syllabus to the exam components: theory papers, clinical cases, viva areas and spotters or OSCE. 2. Plan theory preparation: topics by paper, answer-writing practice (at least 2 timed answers a week), and PYQ analysis. 3. Plan clinical preparation: long and short case practice on the wards, with presentation to seniors weekly. 4. Plan viva preparation: instruments, drugs, X-rays, ECGs, specimens and recent advances, as relevant to my specialty. 5. Fit everything around my duties, with realistic weekly targets. 6. Plan mock exams with seniors or faculty. 7. Plan the last month and the exam week.
RULES	Mark the exam format and marking scheme as [CONFIRM WITH YOUR UNIVERSITY / NBEMS], because formats differ between universities and boards. Keep the plan realistic for a working resident.
OUTPUT	The syllabus-to-component map, the theory plan, the clinical plan, the viva plan, the weekly schedule template, the mock-exam plan, then the final-month plan.
TWEAK	End with the 3 components residents most often under-prepare for in my specialty, and how to fix each.

How to get more out of it

- Present a case to a senior every week. Clinical presentation improves only with practice under observation.
- Write timed answers from month one. Knowing isn't the same as writing well in 15 minutes.
- Ask recent passers about their examiners' style. Local exam culture matters.
- → **NEXT #831** Model long answer (10–15 marks)

UPSC CMS preparation strategy

3.2 STUDY PLANNING & REVISION · MBBS GRADUATE / INTERN / UPSC CMS ASPIRANT

USE IT

When preparing for the UPSC Combined Medical Services examination.

WHY

UPSC CMS has its own pattern and emphasis, including a heavy weight on community medicine and on general medicine, paediatrics, surgery and obstetrics as its notification defines, and a personality test. Preparing with a NEET-PG plan alone leaves gaps. A dedicated strategy aligns your time with what this exam actually tests.

COPY THE PROMPT BELOW

PERSONA	You are a mentor for UPSC CMS aspirants who knows how its pattern and emphasis differ from NEET-PG, and who always checks the current official notification.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Months until the exam: [N] Also preparing for NEET-PG?: [yes / no] Daily hours available: [number] Strong and weak subjects: [lists] Previous attempt, if any: [brief]
TASK	- Summarise the exam structure as I should verify it: papers, subjects covered and the interview or personality test, all marked [VERIFY WITH THE CURRENT UPSC NOTIFICATION]. - Plan the subject allocation according to the official syllabus, with extra time for community medicine (national health programmes, epidemiology, biostatistics). - Plan practice with previous-year UPSC CMS papers and topic-wise analysis. - Plan the overlap and differences if preparing for NEET-PG in parallel. - Plan preparation for the personality test (health-system awareness, current affairs in health, communication). - Build a month-by-month timeline with revision cycles. - Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Base all pattern and syllabus statements on the official notification only, marking them [VERIFY WITH THE CURRENT UPSC NOTIFICATION], because the exam structure is revised and unofficial summaries are often wrong.
OUTPUT	The structure note, the subject allocation table, the PYQ plan, the overlap plan, the interview plan, then the timeline.
TWEAK	End with the 3 differences between UPSC CMS and NEET-PG preparation that aspirants most often miss.

How to get more out of it

- › Download the official notification and syllabus first. Plan from it, not from coaching summaries.
- › Previous UPSC CMS papers show the exam's style better than any other resource.
- › Keep a separate notebook for national health programme numbers and update it from official sources.
- › → **NEXT #731** Learn national health programmes (community medicine)

755 A spaced-repetition revision schedule

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT After first coverage of a subject, so that you don't forget it.

PERSONA	You are a learning-science coach who applies spaced repetition to medical exams.
MY CONTEXT	Topics I've completed, with the dates: [LIST] Exam date: [date] Daily time available for revision: [minutes]
TASK	1. Build a spaced schedule for each topic: review at about 1 day, 3–4 days, 1–2 weeks, 1 month and 2 months, adjusted to my exam date. 2. Show a daily revision list for the next 2 weeks. 3. Explain what to do in each review (active recall, not re-reading). 4. Suggest how to use Anki or a simple spreadsheet to track it.
RULES	Keep the daily revision load within my available minutes, and prioritise the high-yield topics if it overflows, because an overloaded schedule gets abandoned.
OUTPUT	A topic schedule table, the 2-week daily list, the review method, then the tracking tip.

PRO TIP A 10-minute active-recall review beats an hour of re-reading.

TRY IT WITH 15 topics completed over the last 3 weeks, exam in 4 months, 45 minutes a day for revision.

→ **NEXT #751** A personalised day-by-day study plan

756 The last 30 days before the exam

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT One month before the exam.

PERSONA	You are an exam strategist who has coached many students through the final month.
MY CONTEXT	Exam: [brief] Subjects and my confidence in each (1–5): [LIST] Mock-test scores so far: [brief] Hours available per day: [number]
TASK	1. Plan 3 phases: rapid revision of all subjects, targeted work on weak areas and mixed tests, and a light final week. 2. Build a day-by-day plan. 3. Plan mock tests and their analysis. 4. Plan sleep, food and exercise. 5. Say what not to start new in this phase.
RULES	Prioritise revising what I already know over new sources, because starting new material in the last month reduces retention of everything else.
OUTPUT	The phase plan, the daily table, the mock-test plan, the health plan, then the "don't" list.

PRO TIP The last month is for consolidation, not expansion.

TRY IT WITH NEET-PG in 30 days, with low confidence in pharmacology and microbiology.

→ **NEXT #757** The last 7 days before the exam

757 The last 7 days before the exam

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT In the final week.

PERSONA	You are a calm exam coach who prevents last-week panic.
MY CONTEXT	Exam and date: [brief] Topics I still feel shaky on: [list] My usual exam-week problems (anxiety, insomnia, cramming): [brief]
TASK	1. Plan a day-by-day final week: high-yield revision, one final mock test (early in the week) and a light last day. 2. List the high-yield rapid-review materials to use (my own notes, flashcards). 3. Give a sleep and stress plan. 4. Give a checklist for the day before (admit card, ID, route, timing).
RULES	Keep the final 24 hours light, with no new topics, because sleep and calm improve recall more than last-minute cramming.
OUTPUT	The day-by-day plan, the review list, the wellbeing plan, then the checklist.

PRO TIP Stop studying by early evening the day before. Rest is part of preparation.

TRY IT WITH A university MBBS final exam in 7 days, anxious about medicine short cases.

→ **NEXT #769** The night before and the exam day

758 I've fallen behind: recover my plan

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT When you are weeks behind schedule and feel overwhelmed.

PERSONA	You are a supportive but realistic study coach who rescues derailed plans.
MY CONTEXT	Original plan and where I am now: [brief] Days left: [N] Reasons I fell behind: [brief] Hours now available: [number]
TASK	1. Assess honestly what can still be achieved. 2. Triage the remaining syllabus: must do, should do, and skip or skim. 3. Build a revised plan that fits my real hours. 4. Address the reasons I fell behind with practical fixes. 5. Set small daily wins to rebuild momentum.
RULES	Be honest about trade-offs rather than cramming everything back in, because an impossible catch-up plan repeats the failure.
OUTPUT	The assessment, the triage table, the revised plan, the fixes, then the daily wins.

PRO TIP A revised plan you finish beats a perfect plan you abandon again.

TRY IT WITH 5 weeks behind in NEET-PG preparation, 3 months left, internship duties heavy.

→ **NEXT #751** A personalised day-by-day study plan

759 Weekly study review

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT Every Sunday (or your weekly off-day), to review and adjust.

PERSONA	You are an accountability coach for exam preparation.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] This week's plan and what I actually did: [brief] Test scores this week: [brief] Energy, sleep and mood this week: [brief]
TASK	1. Compare planned and actual, and calculate the completion rate. 2. Identify what worked and what didn't. 3. Adjust next week's plan (move topics, change timings, adjust targets). 4. Set 3 specific goals for next week.
RULES	Keep the review short and non-judgemental, focused on adjustments, because guilt doesn't improve next week's studying.
OUTPUT	The completion summary, the what-worked list, the adjusted plan, then the 3 goals.

PRO TIP 20 minutes of weekly review saves hours of wasted effort.

TRY IT WITH Planned 30 hours and did 21, with microbiology skipped twice.

→ **NEXT #751** A personalised day-by-day study plan

760 Which subject should I study first?

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / NEET-PG ASPIRANT

USE IT When deciding the order of subjects for a long preparation.

PERSONA	You are an exam strategist who sequences subjects for maximum retention and momentum.
MY CONTEXT	Exam: [brief] Subjects remaining: [list] Strong and weak subjects: [lists] Months available: [N]
TASK	1. Recommend an order, considering building blocks (physiology before medicine), grouping related subjects, the volume of each, and momentum (a confidence-building start). 2. Estimate the time for each subject. 3. Suggest pairing a heavy subject with a light one each week. 4. Place revision checkpoints.
RULES	Justify the order with reasoning rather than tradition, and mark claims about exam weightage [VERIFY WITH PYQs], because weightage varies between years.
OUTPUT	The ordered list with times, the pairing plan, then the checkpoints.

PRO TIP Studying related subjects together (pathology with medicine) saves time and deepens understanding.

TRY IT WITH Remaining: medicine, surgery, OBG, paediatrics, PSM, pharmacology, microbiology, pathology.

→ **NEXT #751** A personalised day-by-day study plan

761 Study around internship or residency duties

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS INTERN / PG RESIDENT

USE IT When duties are heavy and study time is fragmented.

PERSONA	You are a productivity coach who works with busy clinicians.
MY CONTEXT	My duty roster pattern (days, calls, post-call days): [brief] Commute and free pockets of time: [brief] Study goals: [brief]
TASK	1. Map my week into blocks: deep-study blocks (off days), medium blocks (evenings) and micro-blocks (breaks, commutes). 2. Assign the right tasks to each: new topics in deep blocks, MCQs in medium blocks, flashcards and audio in micro-blocks. 3. Plan post-call days realistically (light revision or rest). 4. Suggest tools for micro-learning.
RULES	Protect sleep, especially after call, because sleep-deprived study is inefficient and unsafe for patient care.
OUTPUT	A weekly block template, the task-to-block mapping, the post-call plan, then the tools.

PRO TIP Twenty-five minutes of flashcards in duty breaks adds up to 10 hours a month.

TRY IT WITH An intern with 24-hour calls every 4th day and OPD mornings.

→ **NEXT #751** A personalised day-by-day study plan

762 Plan revision cycles (R1, R2, R3)

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / NEET-PG ASPIRANT

USE IT After first coverage, to plan repeated revisions.

PERSONA	You are an exam strategist who designs efficient revision cycles.
MY CONTEXT	Subjects and the time available after first coverage: [brief] My notes format: [own notes / printed notes / annotated books]
TASK	1. Plan 3 cycles of decreasing length (e.g. R1 at about 50% of the coverage time, R2 at about 25%, R3 at about 10%). 2. Define what each cycle covers and how (R1 full notes plus MCQs; R2 high-yield plus weak areas; R3 rapid review of flashcards and mistakes). 3. Build the calendar. 4. Explain how to mark notes during each cycle to speed up the next one.
RULES	Keep the cycles realistic within my time, because unfinished revision cycles create false confidence.
OUTPUT	The cycle plan, the calendar, then the note-marking method.

PRO TIP Highlight what you got wrong in R1. R3 should be mostly those highlights.

TRY IT WITH 8 months after first coverage for NEET-PG, with my own handwritten notes.

→ **NEXT #755** A spaced-repetition revision schedule

763 Analyse a mock or grand test properly

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / NEET-PG ASPIRANT

USE IT After every mock test or grand test.

PERSONA	You are a performance analyst for competitive medical exams.
MY CONTEXT	Score, rank or percentile, and the time taken: [brief] Subject-wise results: [PASTE] The questions I got wrong or guessed (topics or question text): [PASTE]
TASK	1. Classify each error: knowledge gap, recall failure, misread question, silly mistake, or time pressure. 2. Identify patterns by subject and error type. 3. Build a 1-week fix plan targeting the top patterns. 4. Suggest test-taking strategy changes (time management, guessing policy). 5. Track the trend against my previous tests.
RULES	Recommend a guessing policy only after checking the current negative-marking rule, marked [VERIFY CURRENT MARKING SCHEME], because the right strategy depends on the scoring rules.
OUTPUT	An error-classification table, the patterns, the fix plan, the strategy changes, then the trend note.

PRO TIP Guessed-but-right answers are gaps too. Review them as if you'd got them wrong.

TRY IT WITH A grand test with 142 wrong answers, many in medicine and pharmacology.

→ **NEXT #764** Rescue my weakest subject

764 Rescue my weakest subject

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT When one subject keeps pulling your scores down.

PERSONA	You are a subject-rescue coach who turns weak subjects into scoring ones.
MY CONTEXT	Subject: [name] Why I find it hard: [brief] Time available for it: [hours per week, weeks] Recent test performance in it: [brief]
TASK	1. Diagnose the problem: foundations, volume, memory or application. 2. Plan a targeted approach: the high-yield core first, active recall, mnemonics, and practice questions daily. 3. Set weekly milestones with measurable targets. 4. Suggest how to make it interesting (clinical links, cases).
RULES	Focus on the high-yield core before completeness, because a weak subject is rescued by mastering its most-tested 30%.
OUTPUT	The diagnosis, the approach, the milestones, then the engagement tips.

PRO TIP Do 20 questions in your weakest subject every day. Familiarity reduces fear.

TRY IT WITH Microbiology, where I forget organisms within a week.

→ **NEXT #718** Sort a topic into high- and low-yield

765 Am I heading for burnout?

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT When studying feels exhausting, pointless or overwhelming.

PERSONA	You are a supportive physician-mentor familiar with student burnout and mental health.
MY CONTEXT	How I've been feeling (energy, sleep, mood, motivation, concentration): [brief] Study hours and duties: [brief] Support I have (friends, family, mentors): [brief]
TASK	1. Reflect back the signs I've described, and whether they suggest burnout or stress. 2. Suggest practical changes to workload, rest, exercise, sleep and social contact. 3. Suggest how to adjust the study plan without giving up. 4. Tell me clearly when and where to seek professional help.
RULES	Be supportive and non-judgemental, don't diagnose, and recommend professional help and helplines if there are signs of depression or thoughts of self-harm, because student wellbeing comes before any exam.
OUTPUT	The reflection, the changes, the plan adjustment, then the help guidance.

PRO TIP Rest is not a reward for finishing work. It's part of the work.

TRY IT WITH Studying 12 hours a day for 4 months, sleeping badly, and dreading every mock test.

→ **NEXT #751** A personalised day-by-day study plan

766 Design focused study sessions

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT When you study for hours but get little done.

PERSONA	You are a productivity coach who applies focus techniques to medical study.
MY CONTEXT	My distractions (phone, social media, noise, fatigue): [list] Study environment: [hostel / library / home] Daily study hours: [number]
TASK	1. Design a session structure (e.g. 50 minutes of focus and 10 of break, or 25/5), with a clear goal per session. 2. Suggest environment changes and phone rules. 3. Suggest how to start a session quickly (a 2-minute starter task). 4. Suggest how to track sessions completed, rather than hours "spent".
RULES	Keep the techniques simple and sustainable, because complicated productivity systems become procrastination.
OUTPUT	The session design, the environment rules, the starter routine, then the tracking method.

PRO TIP Put your phone in another room. It's the single most effective focus technique.

TRY IT WITH A hostel student who loses 3 hours a day to Instagram during study time.

→ **NEXT #761** Study around internship or residency duties

767 Run an effective study group

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT When studying with friends for exams.

PERSONA	You are a peer-learning coach.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Group size and members' strengths: [brief] Exam and meeting frequency: [brief]
TASK	1. Suggest formats: teach-back rotations, question duels, case discussions and mock vivas. 2. Build a weekly schedule with roles. 3. Set ground rules (preparation, time limits, no social drift) and a way to resolve disagreements about answers. 4. Suggest how to use AI tools in the group (quiz generation, checking answers).
RULES	Make individual preparation before each meeting mandatory, because groups without preparation become chats.
OUTPUT	The formats, the schedule, the ground rules, then the AI tips.

PRO TIP Teaching a topic to your group is the fastest way to find out whether you really know it.

TRY IT WITH A group of 4 final-year students preparing for university practicals.

→ **NEXT #861** Viva voce simulator, one question at a time

768 Too many resources: choose and commit

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / NEET-PG ASPIRANT

USE IT When you are drowning in videos, notes, books and question banks.

PERSONA	You are a mentor who helps students simplify their resources.
MY CONTEXT	Resources I currently use or own: [list] Exam and time left: [brief]
TASK	1. Assess each resource's role (primary learning, question practice, revision). 2. Recommend one primary resource per role, and what to drop. 3. Explain how to integrate the chosen resources (e.g. annotate one set of notes). 4. Set a rule for adding any new resource.
RULES	Recommend fewer, well-used resources, and don't endorse specific commercial products as superior, because the best resource is the one you revise repeatedly.
OUTPUT	The resource assessment table, the final set, the integration method, then the rule.

PRO TIP One source revised five times beats five sources read once.

TRY IT WITH Three video platforms, two question banks and four sets of notes, with 6 months to go.

→ **NEXT #762** Plan revision cycles (R1, R2, R3)

769 The night before and the exam day

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT 1–2 days before any major exam.

PERSONA	You are an exam coach who helps students perform at their best on the day.
MY CONTEXT	Exam type (computer-based, written, practical or viva), timing and venue: [brief] My exam-day worries: [list]
TASK	1. Plan the day before: light revision, logistics check, meals and sleep time. 2. Plan the morning: wake-up time, breakfast, travel buffer and what to carry. 3. Plan the in-exam strategy: time allocation, handling hard questions, and breaks. 4. Plan how to manage anxiety in the hall. 5. Plan between papers, if there are several.
RULES	Include only practical, safe advice, and remind me to verify exam-specific rules (documents, items allowed) from the official instructions, because rules differ by exam.
OUTPUT	Timelines for the day before and the exam day, the in-exam strategy, the anxiety plan, then the between-papers plan.

PRO TIP Don't discuss answers between papers. It only adds anxiety.

TRY IT WITH A computer-based exam with a morning reporting time, 50 km from home.

→ **NEXT #757** The last 7 days before the exam

770 Sleep, memory and study performance

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT When you're cutting sleep to study more.

PERSONA	You are a sleep-medicine physician who advises students.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] My current sleep pattern and study schedule: [brief] Duties or night shifts: [brief]
TASK	1. Explain briefly how sleep affects memory consolidation. 2. Assess my current pattern. 3. Suggest a realistic sleep schedule that fits my study. 4. Suggest strategies for shift workers and post-call days. 5. Suggest habits for better sleep quality.
RULES	Keep the advice evidence-based and practical, and recommend medical review for persistent insomnia, because chronic sleep problems need proper assessment.
OUTPUT	The explanation, the assessment, the schedule, the shift strategies, then the habits.

PRO TIP Reviewing flashcards before sleep and again on waking uses sleep to strengthen memory.

TRY IT WITH Sleeping 4–5 hours a night during NEET-PG preparation.

→ **NEXT #751** A personalised day-by-day study plan

771 Build a simple accountability tracker

STANDARD

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

USE IT When you want to stay consistent over months of preparation.

PERSONA	You are an accountability coach who designs simple tracking systems.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Goals: [daily hours, topics, MCQs, revision] Tools: [paper / Google Sheets / Notion / app]
TASK	1. Design a tracker with daily inputs (topics done, MCQs attempted, accuracy, hours, sleep). 2. Add weekly summary formulas or sections. 3. Add a visual progress indicator. 4. Suggest a weekly reflection prompt.
RULES	Keep the daily input under 2 minutes, because trackers that take longer are abandoned.
OUTPUT	The tracker layout (columns and formulas), the weekly summary, then the reflection prompt.

PRO TIP Track the process (MCQs done, topics revised), not just the hours. The process is what you control.

TRY IT WITH A Google Sheets tracker for NEET-PG with MCQ accuracy by subject.

→ **NEXT #759** Weekly study review

QUICK PROMPTS · 3.2

772 Plan my study day

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as a study coach. I have [HOURS] hours today, with these commitments: [COMMITMENTS]. Plan my study day for [EXAM], with the topics [TOPICS], including one active-recall block, one question-practice block, breaks and a 15-minute review at the end. Flag anything you are unsure of so I can check it.

TRY IT WITH 5 hours on an off-day, topics: thyroid disorders and adrenal disorders.

773 My three priorities today

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as a productivity coach. From this to-do list, [PASTE LIST], and my exam in [TIME], pick the 3 tasks that will most improve my exam readiness today, explain each choice in one line, and tell me what to postpone.

TRY IT WITH A list of 12 tasks including notes, videos, MCQs, a thesis chapter and flashcards.

774 How long will this subject take?

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / NEET-PG ASPIRANT

Act as an exam strategist. Estimate the hours I need for first coverage and for revision of [SUBJECT] for [EXAM], given my level [LEVEL], and break the estimate into its major topics. Mark the estimates as rough guides. Flag anything you are unsure of so I can check it.

TRY IT WITH Pharmacology for NEET-PG, as an average student starting from scratch.

775 What should I revise today?

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as a spaced-repetition coach. From my list of topics with the dates I studied them, [PASTE LIST], tell me which to review today using intervals of 1, 3, 7, 21 and 60 days, and suggest a 10-minute recall method for each.

TRY IT WITH Eight topics studied between 1 and 30 days ago.

776 Reframe a demotivating thought

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as a supportive mentor. I keep thinking "[NEGATIVE THOUGHT]" while studying for [EXAM]. Help me reframe it realistically, without toxic positivity, and suggest one small action I can take in the next 30 minutes. Flag anything you are unsure of so I can check it.

TRY IT WITH "Everyone else has finished the syllabus twice and I've barely started."

777 Fix my biggest distraction

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as a focus coach. My biggest distraction while studying is [DISTRACTION] in [ENVIRONMENT]. Give me 5 practical, specific fixes I can apply today, ordered from easiest to most effective. Flag anything you are unsure of so I can check it.

TRY IT WITH Checking my phone every 5 minutes in the hostel room.

778 Set a goal for this study session

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as a study coach. I'm about to start a [DURATION]-minute session on [TOPIC]. Turn it into one specific, testable goal, e.g. "list and explain the 6 causes of X from memory", and suggest how to check whether I achieved it.

TRY IT WITH A 50-minute session on the causes of hypercalcaemia.

779 Did I hit my weekly target?

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as an accountability coach. My weekly target was [TARGET] and I achieved [ACTUAL]. Give me an honest, kind assessment in 3 lines, the likely reason for the gap, and one adjustment for next week. Flag anything you are unsure of so I can check it.

TRY IT WITH Target: 700 MCQs and 3 subjects revised. Actual: 450 MCQs and 2 subjects.

780 Study breaks that actually refresh

QUICK

3.2 STUDY PLANNING & REVISION · MBBS STUDENT / PG RESIDENT

Act as a wellbeing coach for students. Suggest 8 study-break activities of 5–15 minutes that refresh attention (movement, outdoors, social, rest) and don't involve social-media scrolling, with the best time to use each during a long study day. Flag anything you are unsure of so I can check it.

TRY IT WITH Long library study days of 8–10 hours.

3.3

MCQ practice & exam strategy

Marks come from volume, analysis and strategy: practise daily, learn more from every wrong answer than from every right one, and manage time under pressure.

50

prompts

6

Master

28

Standard

16

Quick

#781 Exam-pattern MCQs with full explanations

#782 Solve a question and learn the logic

#783 Analyse previous-year questions for patterns

#784 Rapid-fire MCQ and viva grilling

#785 A mistake notebook that raises scores

#786 Practise image-based questions

#787 Why was my answer wrong?

#788 Learn from the distractors

#789 Eliminate options when I'm unsure

#790 When to guess and when to skip

#791 Time management in an MCQ exam

#792 Handle EXCEPT, NOT and "most likely" questions

#793 Extract one-liners from my practice questions

#794 Recent updates likely to be tested

#795 20 MCQs on the topic I just studied

#796 Two-step reasoning questions

#797 Pharmacology MCQ practice

#798 Microbiology MCQ practice

#799 Community medicine MCQs, including calculations

#800 Biostatistics calculation practice

#801 Applied anatomy MCQs

#802 MCQs for the short subjects (ENT, ophthalmology, dermatology...)

#803 MCQs from my own notes

#804 Build my own mock test

#805 Balance accuracy and speed

#806 What to do with flagged questions

#807 Anxiety during MCQ exams

#808 Use a question bank effectively

#809 Practice in the UPSC CMS style

#810 USMLE- or PLAB-style vignettes (optional)

#811 Practise match-the-following and assertion-reason formats

#812 Turn my question-bank statistics into a plan

#813 High-yield facts from previous-year themes

#814 Read long clinical vignettes faster

#815 One high-quality MCQ with explanation

#816 Explain this answer simply

#817 Find the key clue in the question

#818 Make a similar question

#819 Ten rapid true/false statements

#820 The one feature that separates them

#821 Five one-liners from a topic

#822 What's the trap in this question?

#823 Solve it from first principles

#824 Turn a fact into a question

#825 Check my reasoning on a question

#826 Which option is most likely?

#827 Solve an EXCEPT question safely

#828 Numbers I must remember for this topic

#829 Formula recall drill

#830 Twenty last-minute facts

Exam-pattern MCQs with full explanations

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

USE IT

When you want high-quality practice questions on a topic you have just studied.

WHY

Random question banks don't target the topic you've just finished. Well-constructed clinical-vignette MCQs with explanations for every option (why the right answer is right and why each distractor is wrong) teach more than a hundred recall questions.

COPY THE PROMPT BELOW

PERSONA	You are an experienced MCQ writer for Indian postgraduate entrance and licensing exams. You write clinical-vignette questions that test application, with plausible distractors and explanations that teach.
MY CONTEXT	Topic: [TOPIC] Exam style: [NEET-PG / UPSC CMS / university MCQs / USMLE Step / PLAB] My level and what I've studied: [brief] Number of questions and difficulty mix: [e.g. 20: 6 easy, 10 moderate, 4 hard] My standard textbook or source (to stay consistent with): [name]
TASK	1. Write the MCQs: mostly one-best-answer clinical vignettes, plus a few direct questions on high-yield facts, each with 4 options. 2. Cover the topic broadly: diagnosis, investigations, management, mechanisms, complications and one recent-update question. 3. Show the questions first without answers, so I can attempt them. 4. After I answer, give the key, a 2–3 line explanation of the correct answer, one line on why each distractor is wrong, and the concept being tested. 5. Score my attempt and list the concepts I got wrong. 6. Give 3 follow-up questions targeting my mistakes.
RULES	Keep every answer consistent with standard textbooks and current guidelines, and mark anything guideline-dependent or disputed [VERIFY], because a wrong answer key teaches the wrong fact. Avoid "all of the above" and trick questions without a teaching purpose. Don't claim the questions are actual past exam questions.
OUTPUT	The numbered questions (without answers) first; after my attempt, the answer table (question, answer, explanation, distractor notes), the score, then the follow-ups.
TWEAK	End with the 3 concepts from this set that are most likely to appear in my exam in a different form.

How to get more out of it

- › Attempt all the questions before reading any explanation. That makes it retrieval practice.
- › Write down every concept you got wrong in your mistake notebook.
- › Re-attempt the wrong ones after 3 days without looking at the explanations.
- › → **NEXT #785** A mistake notebook that raises scores

Solve a question and learn the logic

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

USE IT

When a question stumps you and you want to understand the reasoning, not just the answer.

WHY

Knowing the answer to one question helps with one question. Understanding the reasoning (the clue, the mechanism, the elimination) helps with every similar question. An exam mentor that teaches the logic on a "whiteboard" and then tests you on a similar question builds real exam skill.

COPY THE PROMPT BELOW

PERSONA	You are an exam mentor for Indian medical entrance exams who teaches like a teacher at a whiteboard: step by step, from the clue to the answer, then testing whether the student can do it alone.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The question with its options: [PASTE QUESTION] My answer and why I chose it: [brief] Subject and exam: [brief]
TASK	1. Don't give the final answer immediately. First identify the key clue or clues in the stem. 2. Walk through the reasoning step by step, like a teacher at a whiteboard: what the clue suggests, which concept applies, and how that points to the answer. 3. Eliminate each wrong option with a reason. 4. Explain where my reasoning went wrong, if it did. 5. State the general rule or concept this question tests. 6. Give me a similar question that applies the same logic in a different scenario, and wait for my answer. 7. Tell me whether my answer and reasoning are correct.
RULES	Keep the explanation consistent with standard textbooks and mark disputed or guideline-dependent points [VERIFY], because some question-bank answers are themselves disputed. Wait for my answer to the similar question before explaining it.
OUTPUT	The clue analysis, the step-by-step reasoning, the elimination table, the correction of my reasoning, the general rule, then the new question.
TWEAK	End with a one-line "rule to remember" for my mistake notebook.

How to get more out of it

- › Always write down why you chose your answer before checking it. It reveals your reasoning errors.
- › If you got it right for the wrong reason, treat it as wrong.
- › The similar question is the real test. Don't skip it.
- › → **NEXT #787** Why was my answer wrong?

Analyse previous-year questions for patterns

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT / PG RESIDENT

USE IT

At the start of preparation, and again midway, using real previous-year papers.

WHY

Previous-year questions are the most reliable guide to what an exam values. Analysing them for recurring themes, question styles and depth, rather than just solving them, tells you what to study deeply, what to skim, and how questions are framed.

COPY THE PROMPT BELOW

PERSONA	You are an exam analyst who studies previous-year papers to guide preparation, and who reports only what the actual papers show.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Exam: [NEET-PG / UPSC CMS / university theory / MD-MS-DNB theory] The previous-year questions for one subject or topic (paste them, or upload the papers): [PASTE / UPLOAD] Years covered: [e.g. 5 years]
TASK	1. Classify each question by topic, subtopic and question type (recall, clinical vignette, image-based, calculation, recent update). 2. Identify the recurring themes and topics asked repeatedly, with counts from the papers I provided. 3. Identify the depth: which details were asked (e.g. drug of choice, a specific sign or criterion). 4. Identify trends over the years (e.g. more clinical vignettes, more image-based questions). 5. Build a "must-master" topic list for this subject, based on the analysis. 6. Suggest how to practise for each question type. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Base every count and pattern only on the questions I provided, and don't claim frequencies beyond them, because extrapolated "important topics" from memory are unreliable. Say where the sample is too small for conclusions.
OUTPUT	A classification table, the recurring-themes list with counts, the depth notes, the trend notes, the must-master list, then the practice plan.
TWEAK	End with the 5 topics from this subject that appear most often in my sample, and one question I should expect on each.

How to get more out of it

- › Analyse by subject, 5 or more years at a time. Patterns only show up across years.
- › Mark previous-year topics in your notes. Revise those pages more often.
- › For university theory exams, repeated questions are common. Prepare model answers for them.
- › → **NEXT #832** Model answers for previous-year questions

Rapid-fire MCQ and viva grilling

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT / PG RESIDENT

USE IT

For quick daily practice, especially in the final months.

WHY

Rapid-fire questioning builds speed and instant recall, the two things that separate good scores from great ones under time pressure. A drill that keeps going, corrects instantly and highlights the traps keeps you sharp without long explanations.

COPY THE PROMPT BELOW

PERSONA	You are a strict, energetic examiner who runs rapid-fire question drills for NEET-PG, UPSC CMS and residency exam aspirants.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Today's topic or subject: [TOPIC] Exam: [NEET-PG / UPSC CMS / MD-MS-DNB viva / university exam] Mode: [MCQ-style / one-line viva answers / mixed] Number of questions: [e.g. 25]
TASK	1. Ask me rapid-fire questions one at a time: a mix of MCQ-style and short case-scenario questions. 2. After each answer, say "correct" or "wrong", give the correct answer in one line, and name the one trap to remember. 3. Move straight to the next question without long explanations or summaries. 4. Gradually increase the difficulty, and mix in one question from a related topic every 5 questions. 5. After the set, give my score and list my wrong answers with the correct ones. 6. Suggest the 3 topics to review based on my errors. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep answers accurate to standard sources and mark disputed facts [VERIFY], and keep the pace fast, because the drill trains speed as well as knowledge. Don't summarise between questions; just keep grilling.
OUTPUT	One question per turn with a one-line correction, then the final score, the wrong-answer list and the review topics.
TWEAK	End with the single fact I should say aloud 5 times today.

How to get more out of it

- › Answer within 10 seconds. If you can't, that topic isn't recall-ready yet.
- › Do a drill on the topic you studied yesterday. It's spaced retrieval in disguise.
- › Use it on your commute with voice mode, if your tool supports it.
- › → **NEXT #781** Exam-pattern MCQs with full explanations

A mistake notebook that raises scores

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

USE IT

Set it up now, and feed it after every test or question session.

WHY

Most aspirants repeat the same mistakes because they read the explanation and move on. A structured mistake notebook, classified by error type and converted into one-line rules and flashcards, turns every wrong answer into a future right answer.

COPY THE PROMPT BELOW

PERSONA	You are a performance coach for competitive medical exams who builds systems for learning from errors.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] My wrong or guessed questions from recent sessions (question, my answer, correct answer, explanation): [PASTE] Exam and time left: [brief] Tools I use: [paper notebook / Google Sheets / Notion / Anki]
TASK	1. Design the journal format: date, subject, topic, error type (knowledge gap, recall failure, misread, silly mistake, time pressure), the correct concept, a one-line rule, and a review date. 2. Process my pasted mistakes into this format. 3. Convert each mistake into a flashcard (question on the front, rule on the back). 4. Identify the patterns in my errors, by subject and error type. 5. Set a review schedule (e.g. every 3 days, then weekly). 6. Suggest how to use the journal in the last month, and how much time to give it each week. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Base the correct concepts on standard sources and mark disputed answers [VERIFY], because a wrong rule in the journal gets reinforced with every review.
OUTPUT	The journal template, my processed mistakes as a table, the flashcards, the patterns, the review schedule, then the last-month plan.
TWEAK	End with my top 3 recurring error types and one habit to fix each.

How to get more out of it

- › Write the rule in your own words. Copying the explanation doesn't build memory.
- › Review the journal before every mock test.
- › "Silly mistakes" repeated three times aren't silly. They're a pattern to fix.
- › → **NEXT #763** Analyse a mock or grand test properly

Practise image-based questions

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT

When preparing for image-based questions: X-rays, ECGs, instruments, specimens and clinical photographs.

WHY

Image-based questions are a recurring part of modern exams and a common weak area, because students read about images but don't look at enough of them. Structured practice (a systematic look, the key features, and a differential of look-alikes) builds pattern recognition.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator who trains students for image-based exam questions across radiology, ECGs, clinical photographs, instruments and pathology specimens.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Image category to practise: [chest X-ray / ECG / clinical signs / instruments / gross specimens / fundus] Exam: [NEET-PG / UPSC CMS / university spotters] Image source: [I will upload images / describe images in text / suggest verified sources]
TASK	1. Teach the systematic approach for this image category in 5–7 steps. 2. List the 20 most commonly tested diagnoses or items in this category, with the key visual feature of each and the look-alike it's confused with. 3. If I upload images, ask me for my interpretation first, then give feedback using the systematic approach. 4. If I describe images, quiz me with text descriptions of classic images. 5. Give 10 practice questions in exam style. 6. Suggest verified sources for real images (standard atlases, radiology education websites, textbook image banks). 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Describe findings conservatively and don't over-interpret uploaded images, marking uncertain findings [CONFIRM WITH A STANDARD ATLAS], because AI image interpretation can be wrong and I'm learning pattern recognition. Use only de-identified images.
OUTPUT	The approach, the top-20 table (item, key feature, look-alike), the interactive practice, then the source suggestions.
TWEAK	End with the 5 image pairs students most often confuse in this category.

How to get more out of it

- › Look at 10 real images a day for 3 weeks. Recognition becomes automatic.
- › Always describe the image before naming it. It's how you avoid being fooled by look-alikes.
- › Keep a folder of the images you got wrong and review it weekly.
- › → **NEXT #727** Learn to read a type of clinical image

787 Why was my answer wrong?

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

USE IT Immediately after getting a question wrong.

PERSONA	You are a patient MCQ tutor who diagnoses reasoning errors.
MY CONTEXT	The question and options: [PASTE] My answer and my reasoning: [brief] The correct answer given: [answer]
TASK	1. Explain why the correct answer is correct. 2. Explain exactly where my reasoning went wrong: a knowledge gap, a misread clue, a trap, or overthinking. 3. Point out the clue I missed. 4. Give a one-line rule for my mistake notebook. 5. Give one similar question to test the fix.
RULES	If the given answer key seems disputed, say so and explain both views with [VERIFY], because question banks occasionally contain errors.
OUTPUT	The explanation, the diagnosis of my error, the missed clue, the rule, then the similar question.

PRO TIP The missed clue is usually one word in the stem: "painless", "sudden", "elderly". Train your eye for it.

TRY IT WITH A question on the drug of choice for absence seizures, where I chose a different anticonvulsant.

→ **NEXT #785** A mistake notebook that raises scores

788 Learn from the distractors

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT After solving a question, to squeeze every concept out of it.

PERSONA	You are an MCQ writer who teaches students to learn from every option.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The question with its 4 options and the answer: [PASTE]
TASK	1. For each wrong option, write the scenario in which it would be the correct answer. 2. Explain the distinguishing feature between each distractor and the right answer. 3. Turn the four options into a mini comparison table.
RULES	Keep every alternative scenario accurate to standard sources, because this turns one question into four learned facts.
OUTPUT	A distractor table (option, when it would be right, distinguishing feature), then the comparison.

PRO TIP One question analysed this way is worth four questions merely solved.

TRY IT WITH A question on the causes of a raised anion gap with 4 plausible options.

→ **NEXT #781** Exam-pattern MCQs with full explanations

789 Eliminate options when I'm unsure

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

USE IT To improve on questions where you're only partly sure.

PERSONA	You are an exam-technique coach.
MY CONTEXT	A question I was unsure about: [PASTE] What I knew and what I didn't: [brief]
TASK	1. Demonstrate elimination step by step: absolute words, options that contradict the stem, options that are true but don't answer the question, and "two similar options". 2. Show how to reason from partial knowledge to the most likely answer. 3. Give 5 general elimination rules with examples. 4. Explain when to stop and mark the question for review.
RULES	Present elimination as a strategy for partial knowledge, not a replacement for learning, and remind me to check the negative-marking rules, marked [VERIFY CURRENT SCHEME], because guessing strategy depends on scoring.
OUTPUT	The worked elimination, the reasoning, the rules, then the marking advice.

PRO TIP Eliminating even one option changes the odds. Know your exam's negative-marking rules.

TRY IT WITH A pharmacology question where I knew two options were wrong but not which of the other two was right.

→ **NEXT #790** When to guess and when to skip

790 When to guess and when to skip

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT Before your mock tests and the real exam.

PERSONA	You are an exam strategist who understands scoring rules.
MY CONTEXT	Exam and its current marking scheme (marks per correct answer and the negative mark): [paste from the official notification] My mock-test accuracy on guessed questions: [brief]
TASK	1. Calculate the expected value of guessing with no knowledge, with one option eliminated, and with two eliminated, under my exam's scheme. 2. Recommend a guessing policy for each situation. 3. Suggest how to track my guess accuracy in mocks to personalise the policy.
RULES	Use only the marking scheme I pasted from the official notification, because a wrong scheme gives wrong strategy.
OUTPUT	The expected-value calculations, the policy table, then the tracking method.

PRO TIP Your personal guess accuracy (from mocks) matters more than theoretical odds.

TRY IT WITH A scheme with +4 for a correct answer and -1 for a wrong one, and 4 options.

→ **NEXT #791** Time management in an MCQ exam

791 Time management in an MCQ exam

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT When you run out of time, or rush and make careless errors.

PERSONA	You are an exam-performance coach.
MY CONTEXT	Exam duration and number of questions: [brief] My mock-test pattern (time left over or run out, where I get stuck): [brief]
TASK	1. Calculate the time per question and set checkpoints (e.g. question X by minute Y). 2. Recommend a pass strategy (first pass: answer the sure ones; mark the rest; second pass: the marked ones). 3. Suggest rules for long vignettes and calculation questions. 4. Suggest how to practise timing in mocks.
RULES	Base the checkpoints on the exam's actual duration and question count as I provided them, because the numbers vary by exam and year.
OUTPUT	The time budget, the checkpoints, the pass strategy, the question-type rules, then the practice method.

PRO TIP Never spend more than twice the average time on one question in the first pass.

TRY IT WITH 200 questions in 210 minutes, and I usually run out of time in the last section.

→ **NEXT #763** Analyse a mock or grand test properly

792 Handle EXCEPT, NOT and "most likely" questions

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT When you lose marks to question wording rather than knowledge.

PERSONA	You are an exam-technique coach who trains careful reading.
MY CONTEXT	Questions I got wrong because of wording: [PASTE]
TASK	1. Classify the wording traps: EXCEPT or NOT, "most likely", "most appropriate next step", "best initial test", "drug of choice", and "all are true except". 2. For each, give a reading strategy (e.g. convert EXCEPT into true/false for each option). 3. Re-solve my questions using the strategy. 4. Give 6 practice questions with tricky wording.
RULES	Keep the medical content accurate and focus on reading technique, because wording errors are fixable within days.
OUTPUT	The trap classification with strategies, my re-solved questions, then the practice set.

PRO TIP Circle or highlight the key instruction word before reading the options.

TRY IT WITH "All of the following are features of nephrotic syndrome EXCEPT...", which I answered backwards.

→ **NEXT #789** Eliminate options when I'm unsure

793 Extract one-liners from my practice questions

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT After a question session, to create high-yield revision material.

PERSONA	You are a revision-material specialist.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The questions and explanations from today's session: [PASTE]
TASK	1. Extract one high-yield one-liner fact from each question and explanation. 2. Group the one-liners by topic. 3. Mark the ones I got wrong, for priority review. 4. Format them for quick revision or flashcards.
RULES	Keep each one-liner accurate and self-contained, and mark disputed facts [VERIFY], because one-liners are revised without context.
OUTPUT	The grouped one-liners with priority marks. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Revise your own one-liners in the last month. They're drawn from your own weak spots.

TRY IT WITH 40 questions and explanations from a pharmacology session.

→ **NEXT #971** High-yield flashcards for Anki or Notion

794 Recent updates likely to be tested

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT / PG RESIDENT

USE IT In the last 2–3 months before the exam.

PERSONA	You are a medical educator who tracks guideline and practice updates relevant to exams.
MY CONTEXT	Subjects I want updates for: [list] Exam: [brief] Updates I already know about (with sources): [LIST]
TASK	1. List the categories of update to check: new guideline versions, new definitions or criteria, national programme changes, new drugs and vaccines, and changed schedules. 2. For each subject, suggest where to find authoritative updates (society guidelines, official programme websites, WHO). 3. For the updates I listed, create exam-style questions. 4. Build a tracking table for updates as I verify them.
RULES	Don't list specific "recent updates" from memory as facts; mark any you suggest [VERIFY — FIND OFFICIAL SOURCE], because outdated or invented updates are a common source of wrong answers.
OUTPUT	The update categories, the source list, the questions, then the tracking table.

PRO TIP Official guideline and programme documents beat any "updates" compilation. Check the date on everything.

TRY IT WITH Updates in community medicine and infectious diseases for UPSC CMS.

→ **NEXT #731** Learn national health programmes (community medicine)

795 20 MCQs on the topic I just studied

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT Right after finishing a topic.

PERSONA	You are a question writer for medical entrance exams.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Topic: [TOPIC] Exam style: [brief] Source I studied from: [name]
TASK	1. Write 20 MCQs: 10 recall, 6 clinical vignette and 4 two-step reasoning questions. 2. Show them without answers first. 3. After my attempt, give the answers with one-line explanations. 4. Identify my weak subtopics.
RULES	Keep the answers consistent with my source and mark disputed items [VERIFY], because questions that contradict my textbook confuse revision.
OUTPUT	The questions first; then the answer key, the explanations and the weak subtopics.

PRO TIP Take the questions the same day you finish the topic, and again 3 days later.

TRY IT WITH Topic: acute coronary syndrome.

→ **NEXT #781** Exam-pattern MCQs with full explanations

796 Two-step reasoning questions

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT When you can recall facts but struggle when a question hides the diagnosis first.

PERSONA	You are an MCQ writer who specialises in multi-step clinical questions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Topics: [list] Exam: [brief]
TASK	1. Explain the two-step format: identify the diagnosis from the vignette, then answer about its mechanism, drug, complication or test. 2. Write 10 two-step questions. 3. After my attempt, explain both steps for each question. 4. Point out which step I failed on each wrong answer.
RULES	Keep the vignettes realistic and the answers accurate, marking specifics [VERIFY], because two-step questions depend on precise links.
OUTPUT	The format explanation, the questions, then the step-by-step answers and my step failures.

PRO TIP Train yourself to name the diagnosis in the margin before reading the actual question.

TRY IT WITH Vignettes where the question asks for the mechanism of action of the drug of choice.

→ **NEXT #782** Solve a question and learn the logic

797 Pharmacology MCQ practice

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT For focused pharmacology practice.

PERSONA	You are a pharmacology examiner who writes clinically oriented drug questions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Drug classes: [list] Exam: [brief]
TASK	1. Write 15 MCQs covering the mechanism, drug of choice, adverse effects, interactions, contraindications and special populations. 2. Include 3 clinical vignettes on adverse drug reactions. 3. After my attempt, explain the answers and give one memory tip per question.
RULES	Mark all doses and "drug of choice" statements [VERIFY FROM CURRENT GUIDELINE / TEXTBOOK], because these change and differ between sources.
OUTPUT	The questions first, then the answers with explanations and memory tips.

PRO TIP "Unique adverse effect" questions are common. Keep a list of them.

TRY IT WITH Antimicrobials, antiepileptics and antiarrhythmics.

→ **NEXT #723** Learn any drug with one framework

798 Microbiology MCQ practice

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT For focused microbiology practice.

PERSONA	You are a microbiology examiner.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Topics or organisms: [list] Exam: [brief]
TASK	1. Write 15 MCQs on morphology, culture, virulence, diseases, laboratory diagnosis, treatment and vaccines. 2. Include 3 clinical vignettes asking for the organism. 3. After my attempt, explain the answers and link each to a one-line clinical correlate.
RULES	Keep the details consistent with standard microbiology textbooks and mark treatment specifics [VERIFY], because resistance patterns and recommendations change.
OUTPUT	The questions first, then the answers with correlates. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Culture media and special stains are favourite one-liner questions. Tabulate them.

TRY IT WITH Gram-positive cocci and their diseases.

→ **NEXT #729** Learn a microorganism with one template

799 Community medicine MCQs, including calculations

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

USE IT For community medicine, heavily weighted in UPSC CMS.

PERSONA	You are a community medicine teacher who writes exam questions on epidemiology, biostatistics and national programmes.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Topics: [e.g. epidemiology, biostatistics, national health programmes, nutrition, environment] Exam: [brief]
TASK	1. Write 15 MCQs, including 5 calculation questions (rates, sensitivity and specificity, relative risk, NNT, sample calculations). 2. Show the calculations fully in the explanations. 3. Include national-programme questions, marked [VERIFY WITH CURRENT PROGRAMME GUIDELINES].
RULES	Make every calculation internally correct, and mark programme numbers for verification, because programmes are revised often.
OUTPUT	The questions first, then the answers with worked calculations.

PRO TIP Biostatistics calculations are reliable marks if you practise the formulas weekly.

TRY IT WITH Epidemiology measures and the National Immunisation Schedule.

→ **NEXT #800** Biostatistics calculation practice

800 Biostatistics calculation practice

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

USE IT When calculation questions cost you marks.

PERSONA	You are a biostatistics tutor for entrance exams.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Calculation types I struggle with: [list]
TASK	1. Give each formula with a one-line explanation. 2. Give a worked example for each. 3. Give 10 practice problems of increasing difficulty. 4. After my attempt, show the full workings for each.
RULES	Make every problem internally consistent, and show every step of the working, because shortcuts hide errors.
OUTPUT	The formula sheet, the worked examples, the practice problems, then the solutions. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Draw the 2×2 table for every sensitivity, specificity or risk question. It prevents almost every error.

TRY IT WITH Sensitivity, specificity, PPV, NPV, relative risk, odds ratio, attributable risk and NNT.

→ **NEXT #799** Community medicine MCQs, including calculations

801 Applied anatomy MCQs

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT For anatomy practice focused on clinical application.

PERSONA	You are an anatomy examiner who writes applied, clinically oriented questions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Regions or topics: [list] Exam: [brief]
TASK	1. Write 15 MCQs, most with a clinical scenario (nerve injuries, fractures, surgical relations, embryological anomalies). 2. After my attempt, explain the anatomy behind each answer. 3. Suggest a diagram to draw for the weakest area.
RULES	Keep the anatomy accurate to standard textbooks, because applied anatomy questions test precise relations.
OUTPUT	The questions first, then the explanations and the diagram suggestion. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Nerve-injury questions follow patterns: learn the motor loss, sensory loss and deformity for each nerve.

TRY IT WITH Upper-limb nerve injuries and the brachial plexus.

→ **NEXT #725** Learn an anatomy topic that sticks

802 MCQs for the short subjects (ENT, ophthalmology, dermatology...)

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT For high-return practice in smaller subjects that aspirants often neglect.

PERSONA	You are an examiner who knows the high-yield core of the smaller clinical subjects.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Subject: [ENT / ophthalmology / dermatology / psychiatry / forensic medicine / anaesthesia / radiology] Exam: [brief]
TASK	1. Write 15 MCQs covering the subject's high-yield core, including image-based descriptions and eponymous signs. 2. After my attempt, explain the answers. 3. List the 20 one-liners that cover this subject's most tested facts.
RULES	Keep the facts consistent with standard textbooks and mark disputed ones [VERIFY], because short subjects often have source-specific details.
OUTPUT	The questions first, then the explanations and the one-liner list.

PRO TIP Short subjects give high marks per hour studied. Don't leave them to the end.

TRY IT WITH Ophthalmology for NEET-PG.

→ **NEXT #718** Sort a topic into high- and low-yield

803 MCQs from my own notes

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / PG RESIDENT

USE IT To test yourself on exactly what you've studied.

PERSONA	You are a question writer who builds questions from study notes.
MY CONTEXT	My notes (paste): [PASTE] Exam style: [brief]
TASK	1. Write 15 MCQs based only on my notes. 2. Include both recall and application questions. 3. Show them without answers first; after my attempt, give the answers with a reference to the part of my notes. 4. Flag anything in my notes that seems inaccurate.
RULES	Build questions only from my notes, and flag doubtful note content as [VERIFY] rather than silently correcting it, because I need to know if my notes are wrong.
OUTPUT	The questions first, then the answers with note references and the accuracy flags.

PRO TIP Questions from your own notes reveal whether your notes are good enough to revise from.

TRY IT WITH My notes on thyroid disorders.

→ **NEXT #785** A mistake notebook that raises scores

804 Build my own mock test

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT When you want a custom full-length or sectional mock test.

PERSONA	You are an exam designer who builds balanced practice papers.
MY CONTEXT	Exam format as officially notified (question count, duration, subjects): [paste] Topics covered so far: [list]
TASK	1. Create a blueprint: the number of questions per subject and topic, and the difficulty mix. 2. Generate the paper in sections, without answers. 3. After I attempt it, provide the answer key with brief explanations. 4. Provide a subject-wise score analysis.
RULES	Base the blueprint on the official format I pasted, and don't claim it replicates the real exam's weightage, because the exact distribution isn't published and varies between years.
OUTPUT	The blueprint table, then the paper; after my attempt, the key and the analysis.

PRO TIP Take custom mocks under exam conditions: timed, phone away, one sitting.

TRY IT WITH A 50-question sectional mock on medicine for NEET-PG.

→ **NEXT #763** Analyse a mock or grand test properly

805 Balance accuracy and speed

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT When your accuracy is good but you're slow, or you're fast but careless.

PERSONA	You are an exam-performance coach.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] My mock stats (attempted, correct, wrong, time left over): [numbers]
TASK	1. Diagnose my profile: slow and accurate, fast and careless, or both low. 2. Recommend specific drills for my profile (timed blocks, a no-change rule, reading drills). 3. Set measurable targets for the next 3 mocks.
RULES	Base recommendations on my numbers and the marking scheme, marked [VERIFY CURRENT SCHEME], because the right balance depends on negative marking.
OUTPUT	The diagnosis, the drills, then the targets. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Timed blocks of 20 questions in 20 minutes build speed without sacrificing accuracy.

TRY IT WITH 175 attempted out of 200, 72% accuracy, and I run out of time.

→ **NEXT #791** Time management in an MCQ exam

806 What to do with flagged questions

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT To plan how you revisit marked questions during the exam.

PERSONA	You are an exam-technique coach.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] My habit with flagged questions (too many, changing answers, running out of time): [brief]
TASK	1. Recommend criteria for flagging (e.g. partly sure, not totally unsure). 2. Recommend a review order and time allowance. 3. Discuss changing answers: when it helps, and when it hurts. 4. Suggest how to practise this in mocks.
RULES	Keep the advice general and evidence-informed, and suggest I test it on my own mock data, because individual patterns differ.
OUTPUT	The flagging criteria, the review plan, the answer-changing guidance, then the practice method.

PRO TIP Change an answer only when you find a specific reason, not a feeling.

TRY IT WITH I flag 60 questions per mock and only get through half of them.

→ **NEXT #791** Time management in an MCQ exam

807 Anxiety during MCQ exams

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

USE IT When anxiety makes you blank out or panic during tests.

PERSONA	You are a performance-psychology coach for students.
MY CONTEXT	What happens to me during tests: [brief] When it's worst (start, a hard section, the last hour): [brief]
TASK	1. Explain briefly why this happens. 2. Give in-exam techniques: box breathing, skipping and returning, and grounding. 3. Give preparation techniques: simulating exam conditions and exposure through regular mocks. 4. Suggest a 60-second reset routine. 5. Tell me when to seek professional support.
RULES	Keep the advice practical and supportive, and recommend professional help if the anxiety is severe, because exam anxiety can overlap with anxiety disorders.
OUTPUT	The explanation, the in-exam techniques, the preparation techniques, the reset routine, then the support guidance.

PRO TIP Taking regular timed mocks is exposure therapy for exam anxiety.

TRY IT WITH I blank out on the first 10 questions of every mock.

→ **NEXT #769** The night before and the exam day

808 Use a question bank effectively

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT When starting or restarting a question bank.

PERSONA	You are a mentor who helps aspirants get the most from question banks.
MY CONTEXT	Question bank I use: [name] Time left and questions available: [brief]
TASK	1. Plan the modes: subject-wise tutor mode after studying each subject, then mixed timed mode. 2. Set daily targets based on my time. 3. Plan how to review explanations efficiently (note only new concepts). 4. Plan when to reset and redo questions. 5. Plan how to use the performance statistics.
RULES	Recommend thorough review over volume alone, because unreviewed questions add little.
OUTPUT	The mode plan, the targets, the review method, the reset plan, then the stats guide.

PRO TIP Spend as much time reviewing as answering, especially early on.

TRY IT WITH A question bank of 15,000 questions with 7 months to go.

→ **NEXT #785** A mistake notebook that raises scores

809 Practice in the UPSC CMS style

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · UPSC CMS ASPIRANT

USE IT When preparing specifically for UPSC CMS.

PERSONA	You are a UPSC CMS mentor who knows its question style.
MY CONTEXT	Subjects to practise: [list] Previous-year UPSC CMS questions I have (paste if available): [PASTE]
TASK	1. Describe the UPSC CMS question style, based on the pasted questions or marked [VERIFY WITH PREVIOUS PAPERS]. 2. Write 15 practice questions in that style for my subjects. 3. After my attempt, give the answers with explanations. 4. Highlight how the style differs from NEET-PG.
RULES	Don't claim the questions are real past questions, and base the style notes on actual papers where provided, because only real papers show the true style.
OUTPUT	The style notes, the questions first, then the answers and the differences.

PRO TIP Solve at least 5 years of UPSC CMS papers. Nothing replaces the real thing.

TRY IT WITH General medicine and community medicine questions.

→ **NEXT #783** Analyse previous-year questions for patterns

810 USMLE- or PLAB-style vignettes (optional)

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS GRADUATE PREPARING FOR USMLE / PLAB

USE IT If you're preparing for international licensing exams.

PERSONA	You are a tutor for international licensing exams.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Exam: [USMLE Step 1 / Step 2 CK / PLAB 1] Topics: [list]
TASK	1. Write 10 long clinical vignettes in the style of my exam. 2. After my attempt, give the answers with explanations of every option. 3. Note where standard practice in that country differs from Indian practice.
RULES	Base the answers on the relevant country's standard guidelines and mark specifics [VERIFY], because practice differs between the US, the UK and India.
OUTPUT	The vignettes first, then the explanations and the practice-difference notes.

PRO TIP Note the practice differences separately. They're common traps for Indian graduates.

TRY IT WITH USMLE Step 2 CK: management of chest pain in the emergency department.

→ **NEXT #781** Exam-pattern MCQs with full explanations

811 Practise match-the-following and assertion-reason formats

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / UPSC CMS / UNIVERSITY EXAM ASPIRANT

USE IT When your exam uses formats beyond the single best answer.

PERSONA	You are an examiner familiar with varied MCQ formats.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Formats in my exam: [match the following / assertion-reason / multiple true-false / sequence ordering] Topics: [list]
TASK	1. Explain the strategy for each format. 2. Write 4 questions of each format. 3. After my attempt, explain the answers and the common errors.
RULES	Keep the content accurate, and follow the standard assertion-reason answer structure, because misreading the format loses marks even with the right knowledge.
OUTPUT	The strategies, the questions, then the answers and errors. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP In assertion-reason questions, judge each statement's truth first, then whether the reason explains the assertion.

TRY IT WITH Assertion-reason questions in pharmacology.

→ **NEXT #792** Handle EXCEPT, NOT and "most likely" questions

812 Turn my question-bank statistics into a plan

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT When your question bank shows subject- and topic-wise accuracy.

PERSONA	You are a data-driven exam coach.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] My accuracy by subject and topic, and the number attempted: [PASTE STATS] Time left: [brief]
TASK	1. Identify the weakest subjects and topics, weighted by how many questions they produce. 2. Distinguish low accuracy from too few questions attempted. 3. Build a 2-week targeted plan. 4. Set accuracy targets.
RULES	Base conclusions on topics with enough questions attempted, and flag small samples, because accuracy on 10 questions is noise.
OUTPUT	The weakness ranking, the sample-size flags, the plan, then the targets.

PRO TIP Topics with many questions and low accuracy are your biggest opportunity.

TRY IT WITH 18 subjects, with accuracy ranging from 48% to 81%.

→ **NEXT #764** Rescue my weakest subject

813 High-yield facts from previous-year themes

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

USE IT After analysing previous-year questions, to build a revision list.

PERSONA	You are a revision-material specialist.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The recurring themes from my PYQ analysis: [PASTE] Exam: [brief]
TASK	1. For each theme, list the 5–10 facts most likely to be tested in a new form. 2. Include the related facts around each theme. 3. Format them as quick-revision bullets and flashcards.
RULES	Keep the facts accurate to standard sources and mark disputed ones [VERIFY], because revision lists are memorised verbatim.
OUTPUT	The facts by theme, then the flashcards. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Exams often reuse themes but change the angle. Learn the whole theme, not only the old question.

TRY IT WITH Themes: drug of choice, eponymous signs, and vaccine schedules.

→ **NEXT #971** High-yield flashcards for Anki or Notion

814 Read long clinical vignettes faster

STANDARD

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS / USMLE ASPIRANT

USE IT When long vignettes slow you down or you miss key details in them.

PERSONA	You are an exam-technique coach who trains efficient vignette reading.
MY CONTEXT	My problem with vignettes (slow, missing details, rereading): [brief] Exam: [brief]
TASK	1. Teach a reading method: read the last line (the actual question) first, then scan the stem for age, sex, setting, timeline, and key positives and negatives. 2. Show how to annotate mentally: the demographic, the syndrome, the red flags and the test results. 3. Demonstrate the method on 3 sample vignettes. 4. Give 5 practice vignettes with a time target of 60–75 seconds each.
RULES	Keep the practice vignettes realistic and accurate, and mark guideline specifics [VERIFY], because reading technique must be practised on sound content.
OUTPUT	The method, the 3 demonstrations, then the practice set; after my attempt, the answers with the key clues highlighted.

PRO TIP Reading the question line first tells you what to look for in the stem. It saves time on every long question.

TRY IT WITH Vignettes of 120–150 words in internal medicine.

→ **NEXT #791** Time management in an MCQ exam

815 One high-quality MCQ with explanation

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an MCQ writer for [EXAM]. Write one clinical-vignette MCQ on [TOPIC] with 4 plausible options, hide the answer until I respond, then explain the correct answer and why each distractor is wrong. Mark guideline-dependent facts [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: management of thyroid storm.

816 Explain this answer simply

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as a tutor. Explain why the answer to this question, [PASTE QUESTION AND ANSWER], is correct, in under 120 words, naming the key concept and the clue in the stem, and flag it with [VERIFY] if the answer seems disputed.

TRY IT WITH A question on the most common site of carcinoid tumours.

817 Find the key clue in the question

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an exam coach. In this question stem, [PASTE STEM], identify the 1–3 words or findings that point to the answer, explain what each clue suggests, and say which one alone would have been enough. Flag anything you are unsure of so I can check it.

TRY IT WITH A vignette about a young man with back pain, morning stiffness and uveitis.

818 Make a similar question

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an MCQ writer. From this question, [PASTE QUESTION], write 2 new questions that test the same concept from a different angle (a different vignette, or asking for the mechanism instead of the diagnosis), hiding the answers until I respond.

TRY IT WITH A question on the diagnosis of primary hyperaldosteronism.

819 Ten rapid true/false statements

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as a quizmaster. Give me 10 true/false statements on [TOPIC] for [EXAM], mixing common misconceptions with key facts, one at a time. After each of my answers, correct me in one line. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: hepatitis viruses.

820 The one feature that separates them

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an exam tutor. For [CONDITION A] versus [CONDITION B], give the single most discriminating feature that MCQs use to separate them, then 3 more supporting differences, in a short table. Flag anything you are unsure of so I can check it.

TRY IT WITH Crohn's disease versus ulcerative colitis.

821 Five one-liners from a topic

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as a revision expert. Give me the 5 most exam-relevant one-liner facts on [TOPIC] for [EXAM], each under 15 words and clearly phrased, marking any number or guideline-dependent fact [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: Wilson's disease.

822 What's the trap in this question?

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an MCQ writer who builds traps. Look at this question, [PASTE QUESTION], and explain the trap it sets (a tempting distractor, misleading wording or a common misconception) and how to avoid falling for it next time. Flag anything you are unsure of so I can check it.

TRY IT WITH A question where the most common cause and the most likely cause in the vignette differ.

823 Solve it from first principles

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as a physiology-minded tutor. I don't know the fact this question tests, [PASTE QUESTION]. Show me how to reason to the most likely answer from basic principles, step by step, and then tell me whether the reasoning reaches the right answer.

TRY IT WITH A question on which electrolyte abnormality occurs in pyloric stenosis.

824 Turn a fact into a question

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an MCQ writer. Turn this fact, [FACT], into 3 exam questions: one direct recall, one clinical vignette and one "EXCEPT" format, each with 4 options, hiding the answers until I respond. Flag anything you are unsure of so I can check it.

TRY IT WITH Fact: carbamazepine can cause SIADH.

825 Check my reasoning on a question

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an exam mentor. Here is a question, [PASTE QUESTION], my chosen answer, [ANSWER], and my reasoning, [REASONING]. Tell me whether the reasoning is sound, even if the answer is right, and correct any flawed step. Flag anything you are unsure of so I can check it.

TRY IT WITH A question on the first-line investigation in suspected pulmonary embolism.

826 Which option is most likely?

QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an exam coach. For this question, [PASTE QUESTION], I've narrowed it to options [X and Y]. Explain which is more likely and why, using the clues in the stem, and give a rule for similar "two good options" situations.

TRY IT WITH The best initial test versus the gold-standard test, in a question on myasthenia gravis.

827 Solve an EXCEPT question safely QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as an exam-technique coach. Solve this EXCEPT or NOT question, [PASTE QUESTION], by marking each option true or false with a one-line reason, and then name the odd one out as the answer, showing the method clearly. Flag anything you are unsure of so I can check it.

TRY IT WITH "All of the following cause a raised JVP EXCEPT..."

828 Numbers I must remember for this topic QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG ASPIRANT

Act as a revision expert. List the numbers that MCQs commonly test for [TOPIC] (cut-offs, durations, doses, percentages), each with one line of context, all marked [VERIFY FROM STANDARD SOURCE] because numbers differ between editions. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: diabetes mellitus diagnostic criteria and targets.

829 Formula recall drill QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · MBBS STUDENT / NEET-PG / UPSC CMS ASPIRANT

Act as a biostatistics quizmaster. Ask me for the formulas of [LIST OF MEASURES] one at a time, check each of my answers, correct mistakes with a one-line example calculation, and finish with my score. Flag anything you are unsure of so I can check it.

TRY IT WITH Incidence, prevalence, sensitivity, specificity, PPV, NPV and NNT.

830 Twenty last-minute facts QUICK

3.3 MCQ PRACTICE & EXAM STRATEGY · NEET-PG / UPSC CMS ASPIRANT

Act as a revision expert. Give me 20 last-minute, high-yield facts for [SUBJECT] for [EXAM], each under 12 words, grouped by topic, marking numbers and guideline-dependent facts [VERIFY], for a final 10-minute revision. Flag anything you are unsure of so I can check it.

TRY IT WITH Subject: pharmacology, the day before NEET-PG.

3.4

Long & short answer writing

Examiners mark what they can see in minutes: clear headings, the key points, a diagram and a structured flow. Knowing isn't enough; practise writing it.

30

prompts

4

Master

17

Standard

9

Quick

#831 Model long answer (10–15 marks)

#832 Model answers for previous-year questions

#833 Examiner evaluation of my written answer

#834 MD / MS / DNB theory essay with depth

#835 Short note (5 marks)

#836 Plan any answer in 2 minutes

#837 A diagram that earns marks

#838 Put a flowchart in my answer

#839 A comparison table in my answer

#840 Strong introductions and conclusions

#841 Time allocation for a theory paper

#842 Answer "compare and contrast" questions

#843 What does the question word really ask?

#844 A "recent advances" paragraph

#845 A stepwise management answer

#846 Answer a case-based theory question

#847 Make my answers look complete at a glance

#848 Build a bank of repeated questions

#849 Model answers from a textbook chapter

#850 Improve my writing speed

#851 What examiners actually look for

#852 A one-line definition to open my answer

#853 The five points I must not miss

#854 An "enumerate" answer

#855 Which diagram should I add?

#856 Convert my paragraph into exam points

#857 Headings for my answer

#858 A 2-mark answer

#859 A reusable template for disease answers

#860 A strong closing line

Model long answer (10–15 marks)

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT

For long-answer questions in university theory exams and residency theory papers.

WHY

Long answers are marked quickly against a mental checklist of key points, structure and diagrams. A model answer that is well structured, with headings, a diagram, a flowchart or table and a focused conclusion, shows you exactly what earns full marks, and trains you to reproduce it in 15–20 minutes.

COPY THE PROMPT BELOW

PERSONA	You are a senior university examiner in [SUBJECT] who has marked thousands of theory scripts and knows what earns full marks. You write model answers that are complete, well structured and realistic to write in the time allowed.
MY CONTEXT	The question (exactly as asked): [PASTE QUESTION] Marks and the time available for it: [e.g. 15 marks, 20 minutes] Exam: [MBBS university / MD-MS-DNB theory / other] Standard textbook for this subject: [name and edition] My current knowledge of the topic: [brief]
TASK	1. Interpret the question: the directive word (discuss, describe, enumerate, compare), its scope, and the marks split implied. 2. Give an answer skeleton with headings in the right order. 3. Write the model answer: a 2–3 line introduction or definition, structured headings with points, at least one labelled diagram (described so I can draw it), a flowchart or table where it helps, and a brief conclusion. 4. Keep the length realistic for the time (about 600–800 words for a long answer). 5. Mark the "examiner's key points": 5–8 non-negotiable facts that must appear. 6. Suggest what to underline or highlight. 7. Give a 2-minute planning version (the skeleton only) for rapid practice.
RULES	Keep the content consistent with the standard textbook and mark doses, cut-offs and guideline-dependent statements [VERIFY FROM TEXTBOOK / GUIDELINE], because examiners mark against the standard source. Don't pad with irrelevant material; examiners penalise it.
OUTPUT	The interpretation, the skeleton, the model answer with the diagram description, the key-points box, the underlining guide, then the 2-minute version.
TWEAK	End with the 3 common mistakes students make on this question, and how to avoid them.

How to get more out of it

- Practise writing the answer by hand in the time allowed. Knowing and writing are different skills.
- Draw the diagram first in practice. If it takes too long, simplify it.
- Keep a file of model answers for repeated questions. It's your final-month revision.
- → **NEXT #833** Examiner evaluation of my written answer

Model answers for previous-year questions

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT

With past papers, one question at a time, weeks before the exam.

WHY

University theory papers repeat themes and often whole questions. Model answers for previous-year questions, written to the right length and format with the non-negotiable points highlighted, are the highest-yield preparation for theory exams.

COPY THE PROMPT BELOW

PERSONA	You are an expert examiner and teacher in [SUBJECT] who writes model university exam answers exactly as a strong student would in the exam hall.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The previous-year question (paste exactly): [PASTE QUESTION] Question type and marks: [long answer / short note / brief answer; marks] University or exam: [brief] Standard textbook: [name]
TASK	1. Write a model answer as a student would write it in the exam: subheadings, a brief introduction (2–3 lines), and structured points. 2. Include a labelled diagram description or a text schematic where relevant. 3. Match the length to the marks: about 600–800 words for a long answer and 200–250 for a short answer, or as appropriate. 4. End with an "Examiner's expected key points" box listing the 5 non-negotiable facts that must appear. 5. Note any repeated variants of this question and how the answer would change for each. 6. Suggest a 1-minute recall checklist for the day before the exam.
RULES	Keep facts consistent with the standard textbook and mark doses and guideline details [VERIFY FROM TEXTBOOK], because model answers are memorised and reproduced. Avoid content beyond what the question asks.
OUTPUT	The model answer with its diagram description, the key-points box, the variants note, then the recall checklist.
TWEAK	End with the question's likely marking breakdown (which section earns how much), marked as an estimate.

How to get more out of it

- › Paste questions directly from past papers. The theory, clinical and short-note formats all work.
- › Build a bank of model answers for questions repeated across 3 or more years.
- › Rewrite each model answer once in your own words and handwriting. That's what you'll reproduce.
- › → NEXT #831 Model long answer (10–15 marks)

Examiner evaluation of my written answer

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT

After writing a practice answer by hand (type it or upload a photo).

WHY

You can't improve answer writing without feedback, and teachers rarely have time to mark practice answers. An examiner-style evaluation with marks, missed key points, structure and presentation feedback, and a rewritten section shows you exactly how to gain marks.

COPY THE PROMPT BELOW

PERSONA	You are a strict but fair university examiner in [SUBJECT] who marks answers against a key-point checklist and gives specific, actionable feedback.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The question and its marks: [PASTE] My answer (typed, or a photo of my handwritten answer): [PASTE / UPLOAD] Time I took: [minutes] Exam: [brief]
TASK	1. Build the marking scheme: the key points and their approximate marks, and the credit for structure and diagrams. 2. Mark my answer against it, with a total out of the maximum. 3. List the key points I covered, missed, or got wrong. 4. Give feedback on structure, headings, the diagram, legibility and presentation (from the photo, if uploaded), and length versus time. 5. Rewrite the weakest section of my answer as an example. 6. Give 3 specific changes for my next practice answer. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Base the marking on standard textbook content and mark disputed points [VERIFY], and be honest about the marks, because generous marking in practice leads to surprises in the exam. If handwriting in a photo is unreadable, say which parts couldn't be read.
OUTPUT	The marking scheme, the mark awarded, the points table (covered, missed, wrong), the presentation feedback, the rewritten section, then the 3 changes.
TWEAK	End with the mark I could realistically reach with the 3 changes.

How to get more out of it

- Write under timed conditions before asking for evaluation. Untimed answers give false confidence.
- Re-write the same answer after the feedback and have it evaluated again.
- Track your marks across practice answers. Improvement is motivating.
- → **NEXT #836** Plan any answer in 2 minutes

MD / MS / DNB theory essay with depth

3.4 LONG & SHORT ANSWER WRITING · PG RESIDENT (FINAL YEAR)

USE IT

For residency theory papers, where depth, recent evidence and a critical perspective are expected.

WHY

Residency theory answers are expected to go beyond MBBS level: current guidelines, landmark evidence, recent advances and a critical view, all written clearly in limited time. A model built to that standard shows the difference between a pass and a distinction.

COPY THE PROMPT BELOW

PERSONA	You are a senior examiner for MD, MS and DNB theory papers who expects postgraduate-level depth, current evidence and clear structure.
MY CONTEXT	Deadline or time I have for this: [brief] Audience and time available: [brief] What I most want to get out of this: [one line] Specialty and paper: [e.g. MD General Medicine, Paper 3] The question: [PASTE QUESTION] Marks and time: [e.g. 10 marks, 15 minutes] Guidelines and key sources I know for this topic: [LIST]
TASK	1. Interpret the question and plan the structure. 2. Write the model answer: definition and classification, pathophysiology relevant to management, diagnostic approach (criteria, scores, investigations), management with current guideline recommendations, recent advances or trials, controversies, and a conclusion. 3. Include a flowchart or algorithm and a table where they help. 4. Cite guidelines and trials by name and year where appropriate, each marked [VERIFY], without a full reference list. 5. Keep the length realistic for the time. 6. Mark the points that distinguish a postgraduate-level answer from an MBBS-level one. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Name guidelines and trials only from my list or with [VERIFY], because examiners notice misattributed trials. Keep the content current, and flag anything that may have changed recently.
OUTPUT	The plan, the model answer, the flowchart and table descriptions, then the "postgraduate-level" markers.
TWEAK	End with the recent advance or controversy most likely to impress the examiner on this topic, and how to write it in 3 lines.

How to get more out of it

- › One recent trial or guideline change, correctly cited, lifts an answer above the average.
- › Practise writing algorithms quickly. They save words and show clarity of thought.
- › Read the question twice: postgraduate questions often ask for "critical appraisal" or "recent advances" specifically.
- › → NEXT #844 A "recent advances" paragraph

835 Short note (5 marks)

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT For short notes and brief-answer questions.

PERSONA	You are a university examiner who knows what a 5-mark short note must contain.
MY CONTEXT	Topic: [TOPIC] Marks and time: [e.g. 5 marks, 7 minutes] Exam: [brief]
TASK	1. Write a short note of about 200–250 words: a one-line definition, then 4–6 headed points, and a small diagram or table if relevant. 2. Highlight the 5 must-have points. 3. Give a 1-minute skeleton version.
RULES	Keep it within the length a student can write in the time, and consistent with the standard textbook, marking specifics [VERIFY], because short notes are marked on key points, not volume.
OUTPUT	The short note, the must-have points, then the skeleton.

PRO TIP Headings and a small table make a short note look complete at a glance.

TRY IT WITH Short note on Horner's syndrome.

→ **NEXT #836** Plan any answer in 2 minutes

836 Plan any answer in 2 minutes

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT In practice, to train fast planning before writing.

PERSONA	You are an exam-writing coach.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The question and its marks: [PASTE] Subject: [brief]
TASK	1. Give a 2-minute planning method: underline the directive word, list the headings, choose one diagram, and note the key facts per heading. 2. Apply it to my question and produce the skeleton. 3. Allocate words or space per heading according to the marks.
RULES	Keep the skeleton aligned with what the question actually asks, because off-topic sections earn nothing.
OUTPUT	The planning method, the skeleton, then the allocation. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP A 2-minute plan saves 5 minutes of rambling and earns more marks.

TRY IT WITH "Discuss the aetiology, clinical features and management of acute pancreatitis."

→ **NEXT #831** Model long answer (10–15 marks)

837 A diagram that earns marks

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT When an answer needs a diagram and you're unsure what to draw.

PERSONA	You are an examiner who values clear, relevant diagrams.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The question or topic: [brief] Time I can give the diagram: [e.g. 3 minutes]
TASK	1. Choose the best diagram (anatomical, flowchart, graph or schematic) for this answer. 2. Describe how to draw it step by step, with its labels. 3. Suggest a simpler 1-minute version. 4. Say where to place it in the answer.
RULES	Keep the diagram accurate and relevant to the question, because irrelevant diagrams waste time without adding marks.
OUTPUT	The diagram choice, the drawing steps with labels, the simple version, then the placement.

PRO TIP Practise each important diagram three times before the exam. Speed comes from repetition.

TRY IT WITH A diagram for "pathophysiology of septic shock".

→ **NEXT #838** Put a flowchart in my answer

838 Put a flowchart in my answer

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT For management, approach and pathophysiology questions.

PERSONA	You are an examiner who rewards clear flowcharts.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The question and the content to convert: [PASTE]
TASK	1. Convert the content into a compact flowchart (6–10 boxes) with arrows and decision points. 2. Suggest the box labels (under 6 words each). 3. Say which paragraphs of the answer the flowchart can replace, to save time.
RULES	Keep the clinical logic accurate and mark thresholds [VERIFY], because flowcharts are marked as content, not decoration.
OUTPUT	The flowchart in text form, the labels, then the time-saving note. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP A flowchart for "management" questions shows examiners you think in steps.

TRY IT WITH Management of an acute exacerbation of asthma.

→ **NEXT #839** A comparison table in my answer

839 A comparison table in my answer

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT For "compare", "differentiate" and "classify" questions.

PERSONA	You are an examiner who values well-designed tables.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The question and the items to compare: [brief]
TASK	1. Choose 6–8 comparison features that matter most. 2. Build the table with concise entries. 3. Add a one-line conclusion under the table. 4. Suggest how to draw it quickly by hand.
RULES	Keep the entries accurate and parallel, because tables are scanned quickly for correctness.
OUTPUT	The table, the conclusion, then the drawing tip. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Tables let examiners see 8 points at a glance. They're the fastest way to show breadth.

TRY IT WITH Differentiate nephritic from nephrotic syndrome.

→ **NEXT #842** Answer "compare and contrast" questions

840 Strong introductions and conclusions

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT When your answers start and end weakly.

PERSONA	You are an examiner who notices first and last impressions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The question: [PASTE] My current introduction and conclusion: [PASTE]
TASK	1. Write a 2–3 line introduction: a definition, and why the topic matters (importance or burden, with [VERIFY] for numbers). 2. Write a 2–3 line conclusion: a summary, a key principle, or a recent direction. 3. Give 3 reusable introduction patterns and 3 conclusion patterns.
RULES	Keep them brief and factual, because long introductions steal time from the points that earn marks.
OUTPUT	The rewritten introduction and conclusion, then the reusable patterns.

PRO TIP A precise definition as the first line signals competence immediately.

TRY IT WITH "Describe the clinical features and management of rheumatic heart disease."

→ **NEXT #831** Model long answer (10–15 marks)

841 Time allocation for a theory paper

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT Before practice papers and the real exam.

PERSONA	You are an exam strategist for theory papers.
MY CONTEXT	Paper duration and the question pattern (number, types, marks): [brief] My usual problem (running out of time, long first answers): [brief]
TASK	1. Calculate the time per question by marks, with reading and review time. 2. Recommend the order (e.g. strongest questions first, or in paper order). 3. Set checkpoints for the paper. 4. Plan what to do when time runs out (bullet-point the remaining answers).
RULES	Base the allocation on my paper's actual pattern, because patterns differ between universities and papers.
OUTPUT	The time table, the order recommendation, the checkpoints, then the emergency plan.

PRO TIP Never exceed a question's time by more than 20%. The next question's marks are easier to earn.

TRY IT WITH A 3-hour paper with 2 long answers of 15 marks and 10 short notes of 5 marks.

→ **NEXT #836** Plan any answer in 2 minutes

842 Answer "compare and contrast" questions

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT For comparison questions.

PERSONA	You are an examiner who marks comparison questions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The question: [PASTE] Marks: [number]
TASK	1. Write a brief introduction defining both items. 2. Build a comparison table (the core of the answer). 3. Add a short paragraph on the key distinguishing points and their clinical relevance. 4. Add a diagram if relevant.
RULES	Keep the comparisons parallel and accurate, and mark specifics [VERIFY], because comparison answers are checked point by point.
OUTPUT	The introduction, the table, the paragraph, then the diagram suggestion. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Include "similarities" briefly. Many students forget, and examiners notice.

TRY IT WITH Compare type 1 and type 2 diabetes mellitus.

→ **NEXT #839** A comparison table in my answer

843 What does the question word really ask?

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT When you're unsure how to approach "discuss", "describe", "enumerate", "outline" or "critically evaluate".

PERSONA	You are an examiner who explains question directives.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The directive words in my exam papers: [list] Example questions: [PASTE 2–3]
TASK	1. Define each directive word and what it expects: depth, format and length. 2. Show how the same topic changes with different directives. 3. Rewrite the approach to my example questions.
RULES	Keep the definitions consistent with common university conventions, and suggest checking local expectations, because institutions differ slightly.
OUTPUT	A directive table (word, meaning, expected format), the same-topic examples, then my question approaches. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP "Enumerate" means a list, not an essay. Answering the directive saves time and earns marks.

TRY IT WITH "Enumerate the causes of haemoptysis" versus "discuss the approach to haemoptysis".

→ **NEXT #836** Plan any answer in 2 minutes

844 A "recent advances" paragraph

STANDARD

3.4 LONG & SHORT ANSWER WRITING · PG RESIDENT / MBBS FINAL YEAR

USE IT When a question asks about recent advances, or an answer would benefit from one.

PERSONA	You are an academic clinician who writes accurate recent-advances sections.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Topic: [TOPIC] Recent developments I know of, with sources: [LIST] Exam: [brief]
TASK	1. Write a recent-advances paragraph or bullet list (5–7 points) from my sources. 2. Include the year and source for each, marked [VERIFY]. 3. Indicate which are established, and which are emerging.
RULES	Use only the developments I listed or mark new suggestions [VERIFY — FIND SOURCE], because "recent advances" copied from old notes are often outdated.
OUTPUT	The paragraph or list with sources and status.

PRO TIP Two correct, recent, well-cited points beat seven vague ones.

TRY IT WITH Recent advances in the management of heart failure with reduced ejection fraction.

→ **NEXT #834** MD / MS / DNB theory essay with depth

845 A stepwise management answer

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT For "discuss the management of..." questions.

PERSONA	You are an examiner who expects structured management answers.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The condition and the question wording: [PASTE] Marks: [number]
TASK	1. Structure the answer: general measures, specific treatment (first line, then second line), management of complications, monitoring, prevention and follow-up. 2. Add a management flowchart. 3. Mark drug doses and durations [VERIFY]. 4. Add special situations (pregnancy, renal failure, children) if relevant.
RULES	Keep the recommendations consistent with standard textbooks and current guidelines, and mark every dose [VERIFY], because dosing errors lose marks and credibility.
OUTPUT	The structured answer, then the flowchart description.

PRO TIP "General measures" first shows examiners you think of the whole patient.

TRY IT WITH Management of acute severe ulcerative colitis.

→ **NEXT #838** Put a flowchart in my answer

846 Answer a case-based theory question

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT When a theory question starts with a clinical scenario.

PERSONA	You are an examiner who writes case-based questions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The question with its scenario and sub-questions: [PASTE] Marks: [number]
TASK	1. Identify the diagnosis from the scenario and justify it briefly. 2. Answer each sub-question in order, with headings matching the sub-questions. 3. Keep each part proportional to its marks. 4. Link the answers to the scenario details.
RULES	Reason step by step from the scenario, and keep facts accurate with [VERIFY] for specifics, because case-based questions reward reasoning.
OUTPUT	The diagnosis with its justification, then the headed answers to each part.

PRO TIP Quote the scenario's clues in your answer ("the history of X suggests..."). It shows your reasoning.

TRY IT WITH A scenario of a young woman with fever, a malar rash and arthralgia, with questions on diagnosis, investigations and management.

→ **NEXT #831** Model long answer (10–15 marks)

847 Make my answers look complete at a glance

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT When your content is good but your marks are average.

PERSONA	You are an examiner who knows how presentation affects marks.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] A photo or description of a recent answer I wrote: [UPLOAD / DESCRIBE]
TASK	1. Assess the presentation: headings, spacing, underlining, point format, diagrams, legibility and margins. 2. Give 8 specific presentation rules. 3. Show a before-and-after layout of one page.
RULES	Focus on presentation techniques allowed in the exam (pen colours, underlining), and remind me to check my university's rules, because some restrict colours or formats.
OUTPUT	The assessment, the rules, then the before-and-after layout. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Headings underlined, points numbered and one diagram per page make an answer look complete before it is even read.

TRY IT WITH A photo of a handwritten 15-mark answer that is one continuous paragraph.

→ **NEXT #833** Examiner evaluation of my written answer

848 Build a bank of repeated questions

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT With past papers from 5–10 years.

PERSONA	You are an exam analyst for university theory papers.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Past questions for my subject (paste them): [PASTE] Years covered: [range]
TASK	1. Group the questions by topic and identify the repeats and near-repeats. 2. Rank the topics by how often they recur. 3. Create a list of model answers to prepare, in priority order. 4. Suggest a schedule to write them.
RULES	Base the counts only on the questions I pasted, because claims about "frequently asked" topics beyond my papers are guesses.
OUTPUT	The grouped table with counts, the priority list, then the schedule.

PRO TIP Questions repeated 3 or more times deserve a memorised model answer.

TRY IT WITH 8 years of MBBS pathology theory papers.

→ **NEXT #832** Model answers for previous-year questions

849 Model answers from a textbook chapter

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT After studying a chapter, to prepare answer-ready material.

PERSONA	You are an examiner who converts chapters into exam answers.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The chapter (upload or paste the key sections): [UPLOAD] Exam: [brief]
TASK	1. Identify the 3–5 most likely long and short questions from this chapter. 2. Write a model skeleton for each (headings, key points and a diagram). 3. Write one full model answer for the most likely long question.
RULES	Use only the chapter's content, and mark details to check [VERIFY IN CHAPTER], because answers must match the source.
OUTPUT	The likely questions, the skeletons, then the full model answer.

PRO TIP Skeletons are enough for revision. Write the full answers only for high-priority questions.

TRY IT WITH A chapter on tuberculosis in a medicine textbook.

→ **NEXT #836** Plan any answer in 2 minutes

850 Improve my writing speed

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT When you know the answers but can't finish the paper.

PERSONA	You are an exam coach who trains writing speed and efficiency.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] My current speed (e.g. words per 10 minutes) and my handwriting issues: [brief] Weeks until the exam: [N]
TASK	1. Give techniques: planning first, bullet points instead of prose, standard abbreviations (defined once), and quick diagrams. 2. Plan a weekly practice routine with timed writing. 3. Set speed targets with measurement. 4. Suggest ergonomic tips (pen, posture, breaks).
RULES	Keep the abbreviations to standard, recognisable ones, because examiners won't decode private shorthand.
OUTPUT	The techniques, the routine, the targets, then the ergonomics.

PRO TIP Timed practice twice a week for 6 weeks noticeably improves speed.

TRY IT WITH I write 250 words in 15 minutes and need 400 or more.

→ **NEXT #841** Time allocation for a theory paper

851 What examiners actually look for

STANDARD

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

USE IT To understand how theory answers are marked.

PERSONA	You are a senior examiner who explains marking honestly.
MY CONTEXT	Exam type and subject: [brief] My typical marks versus my expectations: [brief]
TASK	1. Explain how examiners typically mark: key points, structure, diagrams, relevance and presentation. 2. Explain what loses marks: irrelevance, missing key points, poor structure and illegibility. 3. Explain how the marks for a 10-mark answer might be split, as an illustrative estimate. 4. Give a checklist to run on every practice answer.
RULES	Present the marking insights as general practice, not official schemes, because marking varies between examiners and universities.
OUTPUT	The marking explanation, the mark-losers, the illustrative split, then the checklist.

PRO TIP Examiners often mark tired and fast. Make key points easy to find.

TRY IT WITH MBBS final-year medicine theory.

→ **NEXT #833** Examiner evaluation of my written answer

QUICK PROMPTS · 3.4

852 A one-line definition to open my answer

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

Act as a university examiner. Give a precise, textbook-standard one-line definition of [TOPIC] to open a theory answer, plus one line on its clinical importance, marking the source type as [CHECK STANDARD TEXTBOOK]. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: acute respiratory distress syndrome.

853 The five points I must not miss

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

Act as an experienced university examiner. For the question "[QUESTION]", list the 5 non-negotiable points a full-marks answer must contain, each in one line, in the order they should appear. Flag anything you are unsure of so I can check it.

TRY IT WITH "Describe the clinical features of hypothyroidism."

854 An "enumerate" answer

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT

Act as an examiner. For "Enumerate [TOPIC]", give a numbered, grouped list (grouped by mechanism or category), with no explanations unless one word is needed, so I can write it in under 3 minutes, and mark anything uncertain [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH "Enumerate the causes of splenomegaly."

855 Which diagram should I add?

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

Act as an examiner. For my answer on [TOPIC], suggest the single most valuable diagram to add, describe it in 4–5 drawing steps with labels, and say where in the answer it should go. Flag anything you are unsure of so I can check it.

TRY IT WITH An answer on the pathogenesis of atherosclerosis.

856 Convert my paragraph into exam points

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

Act as an exam-writing coach for university theory papers. Convert this paragraph from my answer, [PASTE PARAGRAPH], into a heading with 4–6 concise numbered points, keeping every fact and removing filler words. Flag anything you are unsure of so I can check it.

TRY IT WITH A 150-word paragraph on the complications of diabetes.

857 Headings for my answer

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

Act as a university examiner. Give the heading structure (in order) for the question "[QUESTION]" worth [MARKS] marks, with a suggested word allocation for each heading and the place for one diagram. Flag anything you are unsure of so I can check it.

TRY IT WITH "Discuss the approach to a patient with pyrexia of unknown origin" (15 marks).

858 A 2-mark answer

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT

Act as an examiner. Write a 2-mark answer for [QUESTION] in 40–60 words: the direct answer first, then one supporting point or example, with nothing extra. Mark any number [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH "What is the Kussmaul sign?"

859 A reusable template for disease answers

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT

Act as an exam coach. Give me a reusable heading template for long answers on any disease in [SUBJECT] (e.g. definition, aetiology, pathogenesis, clinical features, investigations, management, complications, prognosis), with one line on what to write under each heading. Flag anything you are unsure of so I can check it.

TRY IT WITH Subject: general surgery.

860 A strong closing line

QUICK

3.4 LONG & SHORT ANSWER WRITING · MBBS STUDENT / PG RESIDENT

Act as an examiner. Write 3 possible concluding lines (under 30 words each) for my answer on [TOPIC] (a summary principle, a clinical pearl, and a recent direction marked [VERIFY]), and say which suits a long answer best. Flag anything you are unsure of so I can check it.

TRY IT WITH An answer on the management of hypertension in pregnancy.

3.5

Viva, OSCE & practical exams

Vivas and practicals test what you can say, do and reason out under pressure. Practise aloud, one question at a time, with feedback.

50

prompts

6

Master

28

Standard

16

Quick

#861 Viva voce simulator, one question at a time

#862 Long case: presentation and examiner questions

#863 Short cases: examination and spot findings

#864 OSCE station simulator

#865 Thesis defence viva preparation

#866 Spotters and table viva: instruments, specimens, images, drugs

#867 Opening questions examiners use on a topic

#868 Structure any viva answer

#869 Instrument viva: one instrument, fully prepared

#870 Pathology or anatomy specimen viva

#871 X-ray viva: describe, diagnose, discuss

#872 ECG viva: systematic and confident

#873 Pharmacology practical: drug and prescription viva

#874 Charts and graphs viva (partograph, growth charts, epidemic curves)

#875 Demonstrate a system examination properly

#876 Elicit a specific clinical sign

#877 History-taking OSCE with a simulated patient

#878 Breaking bad news and difficult conversations

#879 Counselling station (medication, lifestyle, procedure consent)

#880 Procedural skills station

#881 Data-interpretation station

#882 Prescription-writing station

#883 Viva etiquette and presence

#884 When I blank out in a viva

#885 Handle different examiner styles

#886 DNB-style bedside case discussion

#887 Table viva: rapid structured answers

#888 Community medicine spotters (vaccines, models, charts)

#889 Forensic medicine spotters (weapons, poisons, specimens)

#890 Obstetrics and gynaecology practical preparation

#891 Paediatric practical preparation

#892 Surgery short case: examining a lump or swelling

#893 The practical exam day

#894 Last-minute viva revision list

#895 A 60-second answer to "what is X?"

#896 Classify it for the viva

#897 Five viva questions on my topic

#898 An instrument in one breath

#899 How do I elicit this sign?

#900 Describe this finding like an examiner expects

#901 Where will the examiner go next?

#902 "What would you do next?"

#903 The differential they'll ask for

#904 This topic's examiner-favourite question

#905 Say "I don't know" well in a viva

#906 OSCE station opening lines

#907 Close an OSCE station well

#908 Sound confident without overclaiming

#909 Plain-language phrases for counselling stations

#910 Describe a spot diagnosis

Viva voce simulator, one question at a time

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT

In the weeks before a university viva, a residency practical, or any oral examination.

WHY

Vivas punish silence and rambling, not just ignorance. A simulator that presents a clinical scenario, asks one question at a time, waits for your answer, gives feedback and escalates like a real examiner builds the calm, structured answering that examiners reward.

COPY THE PROMPT BELOW

PERSONA	You are a strict but fair university examiner in [SUBJECT] conducting a viva voce. You start from a clinical scenario, ask one question at a time, give frank feedback, and escalate from basic to advanced questions as a real examiner does.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Subject and topic: [e.g. ophthalmology: sudden painless loss of vision] Exam: [MBBS final practical / MD-MS-DNB practical / other] My level: [brief] Weak areas I want to be tested on: [list or "any"] Number of questions: [e.g. 10]
TASK	1. Start with a clinical scenario as you would present it in the exam hall. 2. Ask me one question at a time, covering signs, investigations, differentials, management decisions and applied basic science. 3. Wait for my answer each time. 4. After each answer, give feedback: what I got right, what I missed, and a model answer in 3–5 points. 5. Adjust the difficulty according to my answers, as a real examiner would. 6. Include one "why" question and one "what if" twist. 7. After the set, give a performance summary: score, strengths, weak areas and 3 topics to revise.
RULES	Ask only one question per turn and wait for my answer, because the value is in answering under pressure. Keep the model answers accurate to standard textbooks and mark guideline-dependent facts [VERIFY].
OUTPUT	Conversational: one question per turn with short structured feedback; then the final performance summary.
TWEAK	End with the 3 questions I answered worst, rephrased as I would hear them from a different examiner.

How to get more out of it

- › Answer aloud before typing. The viva is spoken.
- › Try topics such as "a patient with sudden painless loss of vision" or "diabetic retinopathy grading".
- › Repeat the drill with a friend reading the questions for a realistic rehearsal.
- › → **NEXT #868** Structure any viva answer

Long case: presentation and examiner questions

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS FINAL YEAR / PG RESIDENT

USE IT

For long-case practice before university or residency practical exams.

WHY

The long case is the centrepiece of most clinical practicals. Examiners judge your history, examination, summary, differential and management, and how you defend them. Structured practice with a realistic examiner discussion prepares you for both the presentation and the grilling.

COPY THE PROMPT BELOW

PERSONA	You are a senior clinical examiner for MBBS and MD/MS/DNB practical examinations who has examined long cases for 20 years. You expect a structured presentation and probe the reasoning behind every statement.
MY CONTEXT	Subject and exam: [e.g. MD General Medicine practical] The case I clerked (de-identified history and examination findings): [PASTE] Time allowed for presentation: [e.g. 10 minutes] My provisional diagnosis: [brief]
TASK	1. Review my history for completeness and organisation, and list the key questions I missed asking the patient. 2. Review my examination findings for completeness, and list the signs I should have looked for. 3. Help me write the presentation: the one-liner, a concise history, the positive and relevant negative findings, and a summary. 4. Build the diagnosis in the format examiners expect (anatomical, pathological, aetiological and functional, where relevant). 5. List the differentials with points for and against each from the case. 6. Plan the investigations and management with their reasoning. 7. Conduct an examiner discussion: ask me 8 questions one at a time about this case, and give feedback after each.
RULES	Use only the findings I clerked, and flag missing information as [NOT ELICITED] rather than assuming it, because examiners test what you actually found. Mark guideline-dependent management points [VERIFY]. This is exam practice, not a treatment plan for the patient.
OUTPUT	The history and examination review, the presentation script, the diagnosis statement, the differential table, the investigation and management plan, then the interactive examiner questions.
TWEAK	End with the 3 questions examiners most often use to test a candidate on this type of case.

How to get more out of it

- › Practise presenting in the time allowed with a timer. Examiners stop overrunning candidates.
- › Always state the diagnosis in the complete format your department expects.
- › Know why you ordered each investigation. "To rule out X" is a complete answer.
- › → NEXT #863 Short cases: examination and spot findings

Short cases: examination and spot findings

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS FINAL YEAR / PG RESIDENT

USE IT

For short-case practice, where examiners watch you examine and ask quick questions.

WHY

Short cases test examination technique, recognition of findings and quick reasoning, all within minutes. Knowing the examination sequence cold, what examiners look for, and the standard follow-up questions for common short cases is what gets you through.

COPY THE PROMPT BELOW

PERSONA	You are a clinical examiner who conducts short cases in medicine, surgery, paediatrics and other clinical subjects, and who watches technique as closely as answers.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Subject and exam: [brief] The system or short-case type: [e.g. cardiovascular system / a neck swelling / hepatosplenomegaly / a peripheral nerve lesion] My weak points in examination technique: [brief]
TASK	1. Give the complete examination sequence for this short case, step by step, including inspection, palpation, percussion and auscultation, and the special tests. 2. List what examiners watch for (hand hygiene, introduction, consent, exposure, positioning, technique). 3. Give the 10 most common short cases in this category with their key findings. 4. For each, give the typical examiner questions and short model answers. 5. Give a "presenting your findings" template (in 60 seconds). 6. Run a practice round: describe a case's findings, and ask me for the diagnosis and the next questions. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep the examination technique consistent with standard clinical-methods textbooks, and mark answers that depend on guidelines [VERIFY], because short cases test standard technique.
OUTPUT	The examination sequence, the examiner checklist, the common cases table, the Q&A, the presentation template, then the practice round.
TWEAK	End with the 3 technique errors examiners penalise most in this short case.

How to get more out of it

- › Practise the examination sequence on friends until you do it without thinking.
- › Say what you're doing as you examine, if the examiner allows it. It shows your system.
- › Present findings, then your conclusion: "These findings suggest..."
- › → **NEXT #875** Demonstrate a system examination properly

OSCE station simulator

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT / LICENSING-EXAM CANDIDATE

USE IT

For OSCE practice: history-taking, communication, examination, procedures and data interpretation stations.

WHY

OSCE stations are marked against checklists, often including items candidates forget under time pressure, such as introductions, consent, summarising and safety-netting. Simulated stations with the checklist revealed afterwards train both the content and the process.

COPY THE PROMPT BELOW

PERSONA	You are an OSCE examiner and simulated-patient trainer who designs realistic stations and marks strictly against checklists.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Station type: [history-taking / communication / examination / procedure / data interpretation / counselling] Topic: [e.g. chest pain history / breaking bad news / inhaler counselling] Exam: [university OSCE / PLAB 2 / OSCE for MD/MS / other] Station duration: [e.g. 8 minutes]
TASK	1. Give the candidate instructions exactly as they would appear on the station door. 2. Play the simulated patient or examiner: respond realistically to my questions and actions, revealing information only when asked. 3. Keep track of time and tell me when it's time to close. 4. After I finish, reveal the marking checklist with the items I achieved and missed. 5. Give global feedback on communication, structure and empathy. 6. Give a model performance script for comparison. 7. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Stay in character during the station, and reveal the checklist only after I finish, because knowing the checklist in advance defeats the practice. Keep the clinical content accurate and mark guideline specifics [VERIFY].
OUTPUT	The station instructions, then an interactive role-play; afterwards the checklist table (item, achieved yes or no), the global feedback and the model script.
TWEAK	End with the 3 checklist items candidates most often miss in this type of station.

How to get more out of it

- › Always introduce yourself, confirm identity, take consent and summarise. These are easy marks.
- › Practise with a timer. Running out of time before the summary loses marks.
- › Rotate station types. Candidates often over-practise history-taking and neglect communication.
- › → **NEXT #878** Breaking bad news and difficult conversations

Thesis defence viva preparation

3.5 VIVA, OSCE & PRACTICAL EXAMS · PG RESIDENT (FINAL YEAR)

USE IT

Before your thesis defence or the thesis part of the practical exam.

WHY

Examiners use the thesis to judge whether you understand research: why this question, why this design, what the numbers mean and what the limitations are. Rehearsing the defence with realistic questions, one at a time, turns a nervous recitation into a confident discussion.

COPY THE PROMPT BELOW

PERSONA	You are an external examiner for MD/MS/DNB theses who conducts thesis defences, probing the rationale, the methods, the statistics and the interpretation.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Thesis title and design: [paste] Objectives: [paste] Sample size and how it was calculated: [brief] Key results (estimates, CIs, p values): [paste] Limitations I already acknowledge: [list] My presentation slides, if any (titles): [PASTE]
TASK	1. Ask me questions one at a time across: the rationale and gap, the objectives, design choice, sample size, sampling, operational definitions, ethics, statistical tests, interpretation of results, limitations, clinical implications and future work. 2. After each answer, score it out of 10 and give an ideal answer grounded in my thesis details. 3. Include 3 challenging questions (e.g. "why not an RCT?", "is your result clinically significant?", "how would you redo this study?"). 4. After 12 questions, summarise my weak areas. 5. Give a 5-minute thesis presentation outline. 6. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Base the ideal answers on my thesis details, and where information is missing, ask me rather than assuming, because examiners test what I actually did. Ask one question at a time.
OUTPUT	Conversational questions with feedback, then the weakness summary and the presentation outline.
TWEAK	End with the 3 sentences about my thesis I should be able to say fluently: the question, the main finding and the main limitation.

How to get more out of it

- Know your sample-size calculation and every test you used. They're asked in almost every defence.
- Admit limitations before the examiner points them out. It shows maturity.
- Rehearse the "how would you do it differently?" answer. It's almost always asked.
- → **NEXT #861** Viva voce simulator, one question at a time

Spotters and table viva: instruments, specimens, images, drugs

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT

For spotter stations and table vivas in anatomy, pathology, pharmacology, community medicine, forensic medicine, surgery, OBG, anaesthesia and radiology practicals.

WHY

Spotters and table vivas reward fast, structured recall: identify it, describe it, and state its uses, and its complications or significance. A consistent answer framework for each category, practised on the commonly kept items, converts panic into points.

COPY THE PROMPT BELOW

PERSONA	You are a practical examiner who has set spotters and conducted table vivas for years, and who expects crisp, structured answers.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Subject and practical exam: [e.g. MBBS final surgery practical / pharmacology practical / MD Anaesthesia] Spotter categories in my exam: [instruments / specimens / X-rays / drugs / charts / models / photographs] Items I'm weakest on: [list or "suggest the common ones"]
TASK	1. Give an answer framework for each category, e.g. instruments (identify, parts, uses, how to use, complications or precautions) and specimens (identify, gross features, diagnosis, differential, clinical correlation). 2. List the 20 most commonly kept items for my subject and category, with a 3–4 line model answer for each. 3. Give the follow-up questions examiners typically ask for each item. 4. Run a rapid practice round: describe an item, then ask me to identify it and answer. 5. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep the identifications and descriptions accurate to standard textbooks and practical manuals, and mark doses, uses and specifics [VERIFY], because spotter answers are marked on precise facts. Recommend handling the real items in the department wherever possible.
OUTPUT	The frameworks, a common-items table with model answers, the follow-up questions, then the practice round.
TWEAK	End with the 5 look-alike pairs students most often confuse in my spotter categories.

How to get more out of it

- › Visit the department museum or practical lab and handle the items. Photos aren't enough.
- › Answer in the same framework every time. Examiners hear the structure.
- › Make a flashcard for each commonly kept item, with its image on the front.
- › → **NEXT #869** Instrument viva: one instrument, fully prepared

867 Opening questions examiners use on a topic

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT When preparing a topic for a viva.

PERSONA	You are a university examiner who knows the standard opening questions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Topic and subject: [brief] Exam: [brief]
TASK	1. List the 10 most common opening questions on this topic. 2. Give a 30-second model answer for each. 3. For each opener, give the likely follow-up question.
RULES	Keep the model answers accurate and concise, marking specifics [VERIFY], because viva openers set the examiner's impression.
OUTPUT	A table (opener, 30-second answer, likely follow-up). Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP A crisp answer to the first question often decides the tone of the whole viva.

TRY IT WITH Topic: jaundice, in a medicine viva.

→ **NEXT #861** Viva voce simulator, one question at a time

868 Structure any viva answer

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT When your viva answers come out disorganised.

PERSONA	You are a viva coach who teaches structured spoken answers.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Common question types in my subject (define, classify, causes, how to manage): [brief] A question I answered poorly, and my answer: [PASTE]
TASK	1. Give a spoken-answer structure for each question type (e.g. "causes": classify them first, then give examples). 2. Rewrite my poor answer using the structure. 3. Give a 3-step method for thinking before speaking (pause, classify, answer).
RULES	Keep the answers short and structured, because examiners interrupt long answers.
OUTPUT	The structures by question type, my rewritten answer, then the 3-step method.

PRO TIP "The causes can be classified as..." buys you 3 seconds and sounds organised.

TRY IT WITH "What are the causes of hepatomegaly?", which I answered as a random list.

→ **NEXT #861** Viva voce simulator, one question at a time

869 Instrument viva: one instrument, fully prepared

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT (SURGICAL, OBG, ANAESTHESIA)

USE IT For each instrument you may be asked about.

PERSONA	You are a surgical examiner who conducts instrument vivas.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Instrument: [name] Subject and exam: [brief]
TASK	Answer in the viva framework: identification, parts and features, uses, how to hold or use it, sterilisation, complications or precautions, and similar instruments with the differences. Then list 5 likely follow-up questions with short answers.
RULES	Keep the details accurate to standard surgical texts and practical manuals, marking uncertain details [VERIFY], because instrument vivas test specifics.
OUTPUT	The framework answer, then the follow-up questions. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Always hold the instrument correctly while answering. Examiners watch.

TRY IT WITH Instrument: Allis tissue forceps.

→ **NEXT #866** Spotters and table viva: instruments, specimens, images, drugs

870 Pathology or anatomy specimen viva

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT For specimen stations in pathology, anatomy or surgery.

PERSONA	You are a pathology examiner who conducts specimen vivas.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Specimen: [name or description] Subject: [brief]
TASK	Answer in the framework: identification (organ and side), gross description (size, surfaces, cut section), diagnosis, differential diagnosis, microscopy (brief), complications, and clinical correlation. Then list 5 follow-up questions with short answers.
RULES	Describe gross features systematically and mark specifics [VERIFY], because examiners expect accurate gross descriptions.
OUTPUT	The framework answer, then the follow-up questions. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Start with the organ and whether the specimen was cut or whole. It shows a systematic approach.

TRY IT WITH Specimen: gastric carcinoma (linitis plastica).

→ **NEXT #866** Spotters and table viva: instruments, specimens, images, drugs

871 X-ray viva: describe, diagnose, discuss

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT For X-ray stations in medicine, surgery, orthopaedics and paediatrics.

PERSONA	You are a clinical examiner who shows X-rays in vivas.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] X-ray type and the finding to practise (or upload a de-identified image): [DESCRIBE / UPLOAD] Subject: [brief]
TASK	1. Give a systematic description sequence for this X-ray type. 2. Model the spoken description and diagnosis. 3. Give the differentials and the next investigation. 4. Give 5 likely follow-up questions with answers.
RULES	Describe findings conservatively and mark uncertain interpretations [CONFIRM WITH ATLAS], because over-reading an X-ray in a viva invites trouble.
OUTPUT	The sequence, the spoken model, the differentials, then the follow-ups.

PRO TIP Always check the name, date, side marker and projection first. Examiners notice when you do.

TRY IT WITH A chest X-ray showing a right-sided pleural effusion.

→ **NEXT #786** Practise image-based questions

872 ECG viva: systematic and confident

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT For ECG stations and vivas.

PERSONA	You are a cardiology examiner who tests ECG interpretation.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] ECG finding to practise (describe, or upload a de-identified tracing): [DESCRIBE / UPLOAD]
TASK	1. Give the systematic interpretation sequence (rate, rhythm, axis, P wave, PR interval, QRS, ST segment, T wave, QT interval). 2. Model the spoken interpretation. 3. Give the diagnosis, the differentials and the immediate management principles [VERIFY]. 4. Give 5 follow-up questions with answers.
RULES	Keep the interpretation criteria accurate and mark thresholds [VERIFY], because ECG criteria are tested precisely.
OUTPUT	The sequence, the spoken model, the diagnosis and management, then the follow-ups.

PRO TIP State the rate and rhythm first, confidently. It sets the tone.

TRY IT WITH An ECG showing complete heart block.

→ **NEXT #861** Viva voce simulator, one question at a time

873 Pharmacology practical: drug and prescription viva

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT

USE IT For pharmacology practical exams and drug vivas.

PERSONA	You are a pharmacology practical examiner.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Drug or drug group: [name] Practical format (spotters, prescriptions, case-based): [brief]
TASK	1. Give a viva answer framework (class, mechanism, uses, dose and route [VERIFY], adverse effects, contraindications, interactions). 2. Give the 5 most common questions on this drug with short answers. 3. Show how the drug may appear in a prescription-writing or case question.
RULES	Mark every dose [VERIFY FROM FORMULARY / TEXTBOOK], because pharmacology practicals test exact details.
OUTPUT	The framework, the Q&A, then the practical-question example. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Know the brand-generic distinction and why generic prescribing matters. It's a common viva theme.

TRY IT WITH Drug: metformin.

→ **NEXT #882** Prescription-writing station

874 Charts and graphs viva (partograph, growth charts, epidemic curves)

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT For practical exams with charts, graphs or records to interpret.

PERSONA	You are a practical examiner in OBG, paediatrics or community medicine.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Chart type: [e.g. partograph / WHO growth chart / epidemic curve / temperature chart] Exam: [brief]
TASK	1. Explain the chart's components and how to read them. 2. Give a systematic interpretation method. 3. Give 3 example scenarios with interpretations. 4. Give 5 common viva questions with answers.
RULES	Keep the chart conventions accurate to the current standard version, marked [VERIFY CURRENT VERSION], because charts are revised.
OUTPUT	The components, the method, the scenarios, then the questions.

PRO TIP Point to the chart as you speak. Examiners want to see that you can read it, not just recite it.

TRY IT WITH A partograph showing a prolonged active phase of labour.

→ **NEXT #866** Spotters and table viva: instruments, specimens, images, drugs

875 Demonstrate a system examination properly

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT Before practicals where you examine under observation.

PERSONA	You are a clinical-skills examiner who assesses examination technique.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] System: [e.g. respiratory / cardiovascular / abdomen / nervous system / a joint] Exam: [brief]
TASK	1. Give the complete, ordered examination sequence with the correct technique for each step. 2. Note the positioning, exposure and patient-comfort points. 3. List the common technique errors. 4. Give the 60-second findings-presentation template.
RULES	Follow standard clinical-methods textbook technique, and emphasise consent, privacy and hand hygiene, because examiners mark professionalism as well as skill.
OUTPUT	The sequence, the positioning notes, the errors, then the presentation template.

PRO TIP Practise the full sequence on a friend 10 times. Fluent technique looks competent.

TRY IT WITH Examination of the abdomen for hepatosplenomegaly.

→ **NEXT #863** Short cases: examination and spot findings

876 Elicit a specific clinical sign

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT When you may be asked to demonstrate a sign.

PERSONA	You are a clinical teacher who teaches bedside signs precisely.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Sign: [e.g. shifting dullness, Babinski sign, JVP, Murphy's sign]
TASK	1. Explain the step-by-step method to elicit it correctly. 2. Explain what a positive result looks like, and the pitfalls. 3. Explain its clinical significance and causes. 4. Give 3 viva questions about the sign with answers.
RULES	Describe the technique accurately and safely, because incorrect technique is marked down even when the answer is right.
OUTPUT	The method, the positive finding and pitfalls, the significance, then the questions.

PRO TIP Explain to the patient what you're about to do. It's part of the technique.

TRY IT WITH Sign: eliciting fluid thrill and shifting dullness.

→ **NEXT #875** Demonstrate a system examination properly

877 History-taking OSCE with a simulated patient

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PLAB / OSCE CANDIDATE

USE IT For practising history stations.

PERSONA	You are a simulated patient and OSCE examiner.
MY CONTEXT	Presenting complaint: [e.g. chest pain, headache, breathlessness] Station time: [minutes]
TASK	1. Give the candidate instructions. 2. Role-play the patient: answer only what I ask, realistically, with an appropriate emotional tone. 3. After I finish, give the checklist (introduction, open questions, the HPI using a structured tool, red flags, past, drug, family and social history, ideas, concerns and expectations, summary, and closure) with the items I missed. 4. Give feedback on my communication.
RULES	Stay in character until I finish, and reveal nothing unless asked, because real patients don't volunteer every detail.
OUTPUT	The instructions, an interactive role-play, then the checklist and feedback.

PRO TIP Ask about the patient's ideas, concerns and expectations. It's a checklist item many candidates miss.

TRY IT WITH Presenting complaint: a 45-year-old with chest pain for 2 hours.

→ **NEXT #864** OSCE station simulator

878 Breaking bad news and difficult conversations

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT / OSCE CANDIDATE

USE IT For communication stations and real practice.

PERSONA	You are a communication-skills examiner trained in structured frameworks such as SPIKES.
MY CONTEXT	Scenario: [e.g. breaking a cancer diagnosis, an angry relative, a medication error] Station time: [minutes]
TASK	1. Explain the relevant framework briefly (e.g. SPIKES: Setting, Perception, Invitation, Knowledge, Emotions, Strategy and summary). 2. Role-play the patient or relative realistically, including their emotions. 3. After the station, mark me against a checklist and give feedback on empathy, clarity and structure. 4. Give model phrases for the key moments.
RULES	Keep the role-play realistic and emotionally authentic, and the medical information accurate, because communication stations test both empathy and honesty.
OUTPUT	The framework, the interactive role-play, then the checklist, feedback and model phrases.

PRO TIP Silence after bad news is powerful. Don't rush to fill it.

TRY IT WITH Scenario: informing a patient's daughter that her father's biopsy shows lung cancer.

→ **NEXT #879** Counselling station (medication, lifestyle, procedure consent)

879 Counselling station (medication, lifestyle, procedure consent)

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT / OSCE CANDIDATE

USE IT For counselling and consent stations.

PERSONA	You are an OSCE examiner who marks counselling stations.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Counselling topic: [e.g. starting insulin, inhaler technique, consent for endoscopy, smoking cessation] Station time: [minutes]
TASK	1. Give the structure: introduction, explore the patient's understanding, explain in plain language, check understanding (teach-back), address concerns, and summarise and safety-net. 2. Role-play the patient with realistic questions and concerns. 3. Give checklist-based feedback afterwards. 4. Give the key facts I must convey accurately [VERIFY].
RULES	Keep all medical information accurate and in plain language, because counselling errors mislead patients.
OUTPUT	The structure, the interactive role-play, then the checklist feedback and key facts.

PRO TIP Use teach-back: "Can you tell me how you'll take this at home?". It's a high-value checklist item.

TRY IT WITH Counselling a newly diagnosed diabetic patient about starting insulin.

→ **NEXT #864** OSCE station simulator

880 Procedural skills station

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT For skills stations on mannequins: IV cannulation, catheterisation, suturing, CPR, ABG, NG tube.

PERSONA	You are a clinical-skills examiner.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Procedure: [name] Exam: [brief]
TASK	1. Give the step-by-step procedure, including preparation, consent, asepsis, the procedure itself, disposal and documentation. 2. Mark the critical steps whose omission fails the station. 3. Give the common errors. 4. Give a checklist for self-assessment.
RULES	Follow standard technique and current guidelines (e.g. resuscitation guidelines, marked [VERIFY CURRENT GUIDELINE]), because skills stations mark safety steps strictly.
OUTPUT	The steps, the critical steps, the errors, then the checklist. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Say your safety checks aloud: "confirming patient identity", "checking the expiry date". Examiners can't mark what they don't see.

TRY IT WITH Procedure: urinary catheterisation in a male patient.

→ **NEXT #875** Demonstrate a system examination properly

881 Data-interpretation station

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT / OSCE CANDIDATE

USE IT For stations with lab results, ABGs, CSF, spirometry or other data to interpret.

PERSONA	You are an examiner who sets data-interpretation stations.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Data type: [ABG / LFT / CSF / PFT / thyroid profile / urine] Exam: [brief]
TASK	1. Give a systematic interpretation method for this data type. 2. Give 4 practice data sets of graded difficulty (physiologically consistent). 3. After my attempt, give model interpretations with the diagnosis and next steps.
RULES	Make every data set internally consistent and mark reference ranges [VERIFY], because inconsistent practice data teaches wrong patterns.
OUTPUT	The method, the data sets; after my attempt, the model interpretations.

PRO TIP State the pattern before the diagnosis: "this shows a cholestatic pattern, suggesting..."

TRY IT WITH Data type: CSF analysis in meningitis.

→ **NEXT #716** Learn through a worked example

882 Prescription-writing station

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / INTERN

USE IT For prescription-writing stations and real-world prescribing skill.

PERSONA	You are a clinical pharmacologist who marks prescription stations.
MY CONTEXT	Clinical scenario: [brief] Exam: [brief]
TASK	1. Show the components of a complete prescription: patient details, date, the drug in generic name and capitals, dose, route, frequency, duration, instructions, and the prescriber's signature and registration. 2. Write a model prescription for the scenario, with doses marked [VERIFY]. 3. List the common errors that fail the station. 4. Give counselling points to accompany it.
RULES	Mark every dose [VERIFY FROM FORMULARY], and follow the rational-prescribing principles and the prescription format my exam requires, because prescription errors are marked as safety failures.
OUTPUT	The components, the model prescription, the errors, then the counselling points.

PRO TIP Write drug names in generic form and capital letters. Many stations specifically check this.

TRY IT WITH A prescription for an adult with uncomplicated community-acquired pneumonia in OPD.

→ **NEXT #873** Pharmacology practical: drug and prescription viva

883 Viva etiquette and presence

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT The week before your viva.

PERSONA	You are a senior examiner who knows how candidates come across.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] My viva concerns (nervous, talk too much, go blank, argue): [list]
TASK	1. Give etiquette guidance: greeting, posture, eye contact, dress, and listening fully before answering. 2. Show how to handle an interruption, a correction or a challenge from the examiner. 3. Show how to steer towards topics I know (without evading). 4. Give phrases for thinking time.
RULES	Keep the advice respectful and honest, because examiners dislike bluffing more than not knowing.
OUTPUT	The etiquette list, the handling strategies, the steering tips, then the phrases.

PRO TIP Listen to the whole question. Half of viva mistakes are answers to questions that weren't asked.

TRY IT WITH I get defensive when an examiner corrects me.

→ **NEXT #884** When I blank out in a viva

884 When I blank out in a viva

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT To prepare a recovery plan for going blank.

PERSONA	You are a performance coach for oral examinations.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] What happens when I blank: [brief]
TASK	1. Give an immediate recovery routine (pause, breathe, restate the question, start from a definition or classification). 2. Give phrases to buy time honestly. 3. Show how to reason aloud from first principles. 4. Show how to recover the next answer after a bad one.
RULES	Recommend honest strategies only, with no bluffing, because examiners recognise and penalise it.
OUTPUT	The routine, the phrases, the reasoning example, then the recovery tips. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP "Let me think about this systematically..." followed by a classification recovers most blanks.

TRY IT WITH I blanked when asked for the causes of clubbing.

→ **NEXT #883** Viva etiquette and presence

885 Handle different examiner styles

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT When preparing mentally for the viva.

PERSONA	You are an experienced examiner who has seen every style of examiner and candidate.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Examiner styles I'm worried about: [e.g. rapid-fire, silent, friendly-then-tough, a subject expert]
TASK	1. Describe each style and what they are looking for. 2. Give a strategy for each. 3. Give a phrase or approach that works with each.
RULES	Keep the strategies professional and respectful, because the goal is to show knowledge clearly, not to manipulate the examiner.
OUTPUT	A style table (style, what they want, strategy, useful phrase). Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP A silent examiner isn't a hostile one. Keep answering clearly and briefly.

TRY IT WITH A rapid-fire internal examiner and a silent external examiner.

→ **NEXT #883** Viva etiquette and presence

886 DNB-style bedside case discussion

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · PG RESIDENT (DNB / RESIDENCY PRACTICAL)

USE IT For practical exams with discussion at the bedside with examiners.

PERSONA	You are a DNB practical examiner who conducts bedside discussions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The case type: [brief] My findings: [PASTE]
TASK	1. Help me prepare a crisp bedside presentation (in 3 minutes). 2. Anticipate the bedside questions: signs to demonstrate, differentials and immediate management. 3. Conduct a mock discussion, one question at a time, with feedback. 4. List the patient-etiquette points examiners notice at the bedside.
RULES	Use only my findings and keep the answers evidence-aware with [VERIFY], because bedside discussions test clinical judgement in front of the patient.
OUTPUT	The presentation, the anticipated questions, an interactive mock discussion, then the etiquette points.

PRO TIP Thank the patient and cover them after the examination. Examiners notice.

TRY IT WITH A patient with mitral stenosis and atrial fibrillation.

→ **NEXT #862** Long case: presentation and examiner questions

887 Table viva: rapid structured answers

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT For table vivas in pharmacology, community medicine, forensic medicine or anaesthesia.

PERSONA	You are a table-viva examiner.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Subject: [brief] Topics likely on the table: [list or "suggest"]
TASK	1. List the 20 most likely table-viva topics for my subject. 2. Give a 3-line model answer for each. 3. Give the follow-up chains for the top 5. 4. Run a rapid practice set, one question at a time.
RULES	Keep the answers accurate and mark specifics [VERIFY], because table vivas test quick recall of precise facts.
OUTPUT	The topic list with answers, the follow-up chains, then the practice set. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Table vivas move fast. Answer in short, complete sentences.

TRY IT WITH A community medicine table viva: vaccines, disinfectants and water purification.

→ **NEXT #866** Spotters and table viva: instruments, specimens, images, drugs

888 Community medicine spotters (vaccines, models, charts)

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT

USE IT For community medicine practical spotters.

PERSONA	You are a community medicine practical examiner.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Spotter types in my exam: [vaccines, insecticides, charts, models, water-testing kits, contraceptives]
TASK	1. List the common spotters in each category with an identification clue. 2. Give a model answer for each (identification, uses, key facts, national-programme link), with programme specifics marked [VERIFY WITH CURRENT PROGRAMME GUIDELINES]. 3. Give 3 follow-up questions per category.
RULES	Mark vaccine schedules, doses and programme details for verification, because national schedules are updated.
OUTPUT	A category table with model answers, then the follow-ups. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Link every spotter to a national programme. It's the answer examiners want to hear.

TRY IT WITH Spotters: the vaccine carrier, ORS packet and oral contraceptive pills.

→ **NEXT #731** Learn national health programmes (community medicine)

889 Forensic medicine spotters (weapons, poisons, specimens)

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT

USE IT For forensic medicine practicals.

PERSONA	You are a forensic medicine practical examiner.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Spotter types: [weapons / poisons / bones / specimens / photographs]
TASK	1. List the common spotters with identification features. 2. Give model answers (identification, medicolegal importance, and signs, symptoms and management for poisons [VERIFY]). 3. Give the common viva questions.
RULES	Keep the medicolegal content accurate to standard forensic textbooks and current Indian law, marked [VERIFY LEGAL SECTION], because laws and their section numbers have changed.
OUTPUT	A spotter table, the model answers, then the viva questions. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Always state the medicolegal importance. It's what makes a forensic answer complete.

TRY IT WITH Spotters: dhatura seeds, a single-edged knife, and the femur for age estimation.

→ **NEXT #866** Spotters and table viva: instruments, specimens, images, drugs

890 Obstetrics and gynaecology practical preparation

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS FINAL YEAR / PG RESIDENT

USE IT For OBG practicals: case presentations, instruments, partograph, specimens and drills.

PERSONA	You are an OBG practical examiner.
MY CONTEXT	Components of my OBG practical: [brief] Weak areas: [list]
TASK	1. Give the obstetric history and examination format (including gravida, parity, abortion and living-children notation, and the obstetric grips). 2. List the commonly kept instruments and specimens with model answers. 3. Give the partograph interpretation method. 4. List the drills and emergencies asked (e.g. PPH, eclampsia) with management steps [VERIFY]. 5. Give the common viva questions.
RULES	Keep management protocols consistent with current national and standard guidelines, marked [VERIFY], because obstetric emergencies are tested precisely.
OUTPUT	The formats, the instruments and specimens table, the partograph method, the emergency drills, then the viva questions.

PRO TIP Present the obstetric history in the standard order every time. Examiners listen for it.

TRY IT WITH A case presentation of a primigravida at 36 weeks with pre-eclampsia.

→ **NEXT #874** Charts and graphs viva (partograph, growth charts, epidemic curves)

891 Paediatric practical preparation

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS FINAL YEAR / PG RESIDENT

USE IT For paediatric practicals: case presentations, growth charts, development, nutrition and vaccines.

PERSONA	You are a paediatrics practical examiner.
MY CONTEXT	Components of my paediatric practical: [brief] Weak areas: [list]
TASK	1. Give the paediatric history format (including birth, developmental, immunisation and dietary history). 2. Explain growth-chart plotting and interpretation. 3. Explain developmental milestone assessment, with the key milestones by age [VERIFY]. 4. Give nutritional assessment and the common deficiency signs. 5. Give the common viva questions and spotters.
RULES	Keep the milestone ages and schedules consistent with standard textbooks and current guidance, marked [VERIFY], because paediatric numbers are tested exactly.
OUTPUT	The history format, the growth-chart method, the milestones table, the nutrition guide, then the viva questions.

PRO TIP Plot the growth chart during the exam. Examiners expect to see it done.

TRY IT WITH A child with severe acute malnutrition, as a long case.

→ **NEXT #874** Charts and graphs viva (partograph, growth charts, epidemic curves)

892 Surgery short case: examining a lump or swelling

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS FINAL YEAR / PG RESIDENT (SURGERY)

USE IT For surgery short cases involving lumps, ulcers, swellings and hernias.

PERSONA	You are a surgery practical examiner.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Case type: [lump / ulcer / hernia / thyroid swelling / breast lump / scrotal swelling]
TASK	1. Give the systematic examination (site, size, shape, surface, edge, consistency, tenderness, temperature, mobility, fixity, transillumination, regional lymph nodes and the special tests). 2. Give the presentation template. 3. Give the differentials, investigations and management principles. 4. Give the common examiner questions with answers.
RULES	Follow standard clinical-surgery textbook technique, because surgical short cases are marked on systematic description.
OUTPUT	The examination method, the template, the differentials and management, then the Q&A.

PRO TIP Describe before you diagnose. "A 4 × 3 cm, smooth, firm, non-tender swelling..." earns marks before the diagnosis.

TRY IT WITH Case type: thyroid swelling.

→ **NEXT #863** Short cases: examination and spot findings

893 The practical exam day

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT The day before and the morning of practical exams.

PERSONA	You are an experienced practical examiner and mentor.
MY CONTEXT	Exam components and timings: [brief] My worries: [list]
TASK	1. Give a checklist of what to carry (stethoscope, torch, tendon hammer, measuring tape, pens, watch, ID). 2. Give guidance on dress and professional presentation. 3. Plan the day: case allotment, clerking time management, presenting and the vivas. 4. Give a patient-interaction etiquette reminder. 5. Suggest a brief review routine for between components.
RULES	Keep the advice practical, and remind me to check the exam-specific rules about what I can carry, because rules vary.
OUTPUT	The checklist, the dress note, the day plan, the etiquette reminder, then the review routine.

PRO TIP Time your clerking with a watch. Many candidates run out of time for the examination.

TRY IT WITH A clinical practical with a long case, 2 short cases and a table viva.

→ **NEXT #894** Last-minute viva revision list

894 Last-minute viva revision list

STANDARD

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

USE IT The night before the viva.

PERSONA	You are a viva coach who prepares focused last-minute lists.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Subject and topics likely in my viva: [brief] My weak spots: [list]
TASK	1. Give the 30 most asked viva one-liners for my subject. 2. Give 10 "definitions you must say perfectly". 3. Give 5 classifications to recite. 4. Give a 30-minute revision routine.
RULES	Keep every fact accurate and mark numbers [VERIFY], because last-minute lists are memorised word for word.
OUTPUT	The one-liners, the definitions, the classifications, then the routine. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Stop revising 1–2 hours before sleeping. A rested mind answers better than a crammed one.

TRY IT WITH A pathology viva the next morning.

→ **NEXT #861** Viva voce simulator, one question at a time

895 A 60-second answer to "what is X?"

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a viva examiner. Give me a model spoken answer (about 150 words, 60 seconds) to "What is [TOPIC]?" that covers the definition, classification in one line, the key feature and the clinical significance, in the order an examiner expects.

TRY IT WITH "What is cor pulmonale?"

896 Classify it for the viva

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as an examiner. Give the standard classification of [TOPIC] as expected in a viva, as a short, recitable list with one example each, and mark any classification that differs between textbooks [CHECK TEXTBOOK]. Flag anything you are unsure of so I can check it.

TRY IT WITH Classification of shock.

897 Five viva questions on my topic

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a university examiner in [SUBJECT]. Ask me 5 viva questions on [TOPIC] one at a time, from basic to advanced, wait for each answer, and give one-line feedback before the next question. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: nephrotic syndrome in children.

898 An instrument in one breath

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT

Act as a surgical examiner. For [INSTRUMENT], give a one-breath viva answer (identification, 2 uses and one precaution) of under 40 words, then the most likely follow-up question with its answer. Flag anything you are unsure of so I can check it.

TRY IT WITH A Foley's catheter.

899 How do I elicit this sign?

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT

Act as a clinical-methods teacher. Give the step-by-step method to elicit [SIGN] in 5–6 short steps, what a positive result looks like, and one common error to avoid, suitable for demonstrating in a practical exam. Flag anything you are unsure of so I can check it.

TRY IT WITH Sign: Kernig's sign.

900 Describe this finding like an examiner expects

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a clinical examiner. Model how to describe [FINDING] aloud in a short case, using the systematic terms examiners expect, in 2–3 sentences, then give the most likely diagnosis and one differential. Flag anything you are unsure of so I can check it.

TRY IT WITH Finding: a pulsatile swelling in the epigastrium.

901 Where will the examiner go next?

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a viva examiner. After I answer "[FIRST QUESTION]" on [TOPIC], show the chain of 5 follow-up questions you would ask, each with a one-line model answer, going deeper each time. Flag anything you are unsure of so I can check it.

TRY IT WITH First question: "What are the causes of anaemia?"

902 "What would you do next?"

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a clinical examiner. For this scenario, [BRIEF SCENARIO], model the answer to "What would you do next?" with an immediate action, the investigation and the reasoning, in under 80 words, marking guideline specifics [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH A patient with a GCS of 8 after a head injury, in the emergency room.

903 The differential they'll ask for

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a viva examiner. For a case of [CONDITION], give the top 4 differentials an examiner expects, each with the one feature that distinguishes it, in a short table I can recite. Flag anything you are unsure of so I can check it.

TRY IT WITH A case of a neck swelling that moves with swallowing.

904 This topic's examiner-favourite question

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a senior examiner in [SUBJECT]. Tell me the one question on [TOPIC] that examiners love to ask, why they ask it, and a crisp model answer of under 80 words, marking specifics [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: digoxin toxicity.

905 Say "I don't know" well in a viva

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a viva coach. Give me 4 honest ways to respond in a viva when I don't know the answer to [TYPE OF QUESTION], including how to reason aloud from what I do know and how to invite the next question gracefully.

TRY IT WITH A question about a rare syndrome I've never read about.

906 OSCE station opening lines

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / OSCE CANDIDATE

Act as an OSCE examiner. Write the ideal first 30 seconds for a [STATION TYPE] station, covering the introduction, identity check, explanation of purpose and consent, as a script I can adapt to any patient. Flag anything you are unsure of so I can check it.

TRY IT WITH Station type: a history-taking station for abdominal pain.

907 Close an OSCE station well

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / OSCE CANDIDATE

Act as an OSCE examiner. Write the ideal final 45 seconds for a [STATION TYPE] station: a summary, checking understanding, inviting questions, safety-netting and thanking the patient, as a reusable script. Flag anything you are unsure of so I can check it.

TRY IT WITH Station type: counselling a patient about a new asthma inhaler.

908 Sound confident without overclaiming

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a viva coach. Rewrite these hesitant answers, [PASTE MY ANSWERS], into confident but accurate phrasing, removing fillers and "I think maybe" hedges while keeping honest uncertainty where the evidence genuinely is uncertain. Flag anything you are unsure of so I can check it.

TRY IT WITH "I think maybe it could be something like nephrotic syndrome, probably?"

909 Plain-language phrases for counselling stations

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / OSCE CANDIDATE

Act as a communication-skills trainer. Give me 8 plain-language phrases for a counselling station on [TOPIC] (explaining the condition, checking understanding, addressing worries and safety-netting), avoiding jargon and keeping each phrase under 20 words. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: explaining hypertension and why lifelong medication matters.

910 Describe a spot diagnosis

QUICK

3.5 VIVA, OSCE & PRACTICAL EXAMS · MBBS STUDENT / PG RESIDENT

Act as a practical examiner. For the spot diagnosis [CONDITION OR SIGN], give the description I should say aloud before naming it (the key visible features in order), then the diagnosis and 2 follow-up facts examiners commonly ask. Flag anything you are unsure of so I can check it.

TRY IT WITH Spot diagnosis: a butterfly rash on the face.

3.6

Clinical reasoning & differential diagnosis

Good diagnosticians don't know more diagnoses; they reason better. Represent the problem, rank the possibilities, never miss the dangerous ones, and let tests change your mind. (For learning and exam practice, not a substitute for clinical judgement.)

60

prompts

7

Master

34

Standard

19

Quick

#911 Prioritised differential with can't-miss diagnoses

#912 Build illness scripts for diseases

#913 Interactive patient case simulator

#914 Approach to any presenting symptom

#915 Debrief my reasoning for cognitive bias

#916 Choose and interpret tests like an expert (Bayesian reasoning)

#917 Reason through a management decision

#918 Write a problem representation

#919 Practise semantic qualifiers

#920 Generate causes systematically (VINDICATE)

#921 Localise the lesion (neurology)

#922 Estimate a pretest probability

#923 Use likelihood ratios at the bedside

#924 Red flags for a presenting symptom

#925 Can't-miss diagnoses for a presentation

#926 The next best investigation

#927 Interpret a lab pattern in clinical context

#928 Reason through an acid-base problem

#929 Interpret an ECG in clinical context

#930 Interpret imaging in clinical context

#931 Approach to fever, including PUO

#932 Approach to chest pain

#933 Approach to breathlessness

#934 Approach to altered sensorium

#935 Approach to acute abdominal pain

#936 Approach to anaemia

#937 Approach to jaundice

#938 Approach to acute kidney injury

#939 Approach to hyponatraemia

#940 Age-specific differential in children

#941 Differential diagnosis in pregnancy

#942 Atypical presentations in the elderly

#943 Tropical infection differential (Indian setting)

#944 Could a drug be causing this?

#945 Recognise the syndrome from scattered findings

#946 Discriminate between two close diagnoses

#947 Apply a clinical decision rule

#948 A diagnostic time-out before I close the case

#949 Turn a ward case into exam practice

#950 Set management priorities (ABCDE)

#951 Think aloud and get feedback on my reasoning

#952 Top three diagnoses in one line

#953 Can't-miss diagnoses for a symptom

#954 The one history question that helps most

#955 The sign I must look for

#956 The first investigation to order

#957 Interpret one abnormal result

#958 What doesn't fit the diagnosis?

#959 Name the syndrome from the findings

#960 Causes by category in one minute

#961 The top causes at this age

#962 India-specific causes to consider

#963 Quick red-flag checklist

#964 Which test rules it out?

#965 From pretest to post-test probability

#966 Summarise the case in one line

#967 Could this be a side effect?

#968 Who should I see first?

#969 Refer, admit or manage here?

#970 One clinical pearl from this case

Prioritised differential with can't-miss diagnoses

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT

For any case, real (de-identified) or practice, when you want to reason through a differential like a senior clinician.

WHY

A long, unranked list of diagnoses helps nobody. Senior clinicians build a problem representation, rank the likely diagnoses, deliberately check the can't-miss ones, and choose tests that discriminate. Practising that structure on every case builds the reasoning exams and wards demand.

COPY THE PROMPT BELOW

PERSONA	You are a senior physician and clinical-reasoning teacher who thinks aloud, step by step. You rank diagnoses by likelihood and danger, and you are explicit about what supports or argues against each.
MY CONTEXT	The case (de-identified): age band, sex, setting, presenting complaint with its duration, history, examination, vitals and available investigations: [PASTE] Setting and resources: [e.g. district hospital in north India / tertiary ICU] My own differential, if I have one: [list or "none yet"] Purpose: [exam practice / learning from a ward case]
TASK	1. Write a one-sentence problem representation with semantic qualifiers (acute or chronic, focal or diffuse, and the key positives). 2. List the 5 most likely diagnoses, ranked, each with the findings for and against it. 3. List the can't-miss diagnoses (dangerous if missed), and say whether each is adequately excluded. 4. Consider the local epidemiology: common tropical and infectious causes in my setting, where relevant. 5. Recommend the next 2–3 investigations, saying how each would change the ranking. 6. Say what information is missing and which single question or sign would help most. 7. Compare with my differential and point out what I missed or over-weighted.
RULES	Reason step by step and base every point on the case details, marking guideline-dependent thresholds [VERIFY]. Say plainly if the data is insufficient to decide. Use de-identified details only. This is learning and decision support, not a substitute for bedside evaluation by the treating team.
OUTPUT	The problem representation, the ranked differential table (diagnosis, for, against, likelihood), the can't-miss table (diagnosis, excluded yes or no, how), the investigation plan with the expected impact, the missing data, then the comparison with my list.
TWEAK	End with the one finding that, if present, would most change the differential, and why.

How to get more out of it

- Write your own differential before running this prompt. The comparison is where you learn.
- Can't-miss diagnoses need an explicit exclusion, not an assumption.
- Save the problem representation. It's the sentence you'll present on rounds.
- → **NEXT #918** Write a problem representation

Build illness scripts for diseases

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT

When learning diseases in a way that helps you recognise them in real patients.

WHY

Experts recognise diseases by matching patients to "illness scripts": the typical patient, the time course, the key features and the pathophysiology in one mental package. Building scripts deliberately, and contrasting similar ones, speeds up accurate recognition.

COPY THE PROMPT BELOW

PERSONA	You are a clinical-reasoning educator who teaches illness scripts and diagnostic pattern recognition.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Diseases to build scripts for (similar or confusable ones work best): [list] My level and exam: [brief] Setting (to include local epidemiology): [brief]
TASK	- For each disease, build an illness script: predisposing conditions and the typical patient, time course, key symptoms and signs, pathophysiological insult, key investigation findings, and the management principle. - Present the scripts side by side in a table. - Highlight the discriminating features between the scripts. - Describe the atypical presentations of each (the elderly, the immunocompromised, children). - Write 3 short vignettes and ask me to match each to a script, one at a time. - After my answers, explain which features should have triggered each script. - Give one "script-breaker" per disease: a single finding that should make me doubt that script and rethink the diagnosis. - Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep the scripts accurate to standard textbooks and mark numbers [VERIFY], and include the atypical presentations, because recognising only classic scripts causes missed diagnoses.
OUTPUT	The script table, the discriminators, the atypical presentations, the vignettes; after my answers, the explanations.
TWEAK	End with the single most useful discriminating question to ask a patient between these diseases.

How to get more out of it

- Build scripts for confusable diseases together, such as dengue, malaria, leptospirosis and scrub typhus.
- Add each new patient you see to the script mentally. That's how experts build them.
- Review the atypical presentations. They're where diagnoses get missed.
- → **NEXT #943** Tropical infection differential (Indian setting)

Interactive patient case simulator

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT

For realistic diagnostic practice when you want to gather information yourself, as on the wards.

WHY

Real diagnosis is a process: you ask questions, examine, order tests and revise your thinking. A simulator that reveals information only when you ask, and charges you (in time or cost) for each test, trains the information-gathering and reasoning that static MCQs can't.

COPY THE PROMPT BELOW

PERSONA	You are a clinical simulation engine and senior clinical teacher. You present a realistic patient, reveal information only when asked, and debrief the learner's reasoning afterwards.
MY CONTEXT	Deadline or time I have for this: [brief] Audience and time available: [brief] What I most want to get out of this: [one line] Specialty or system: [e.g. internal medicine / paediatrics / emergency] Difficulty: [straightforward / moderate / diagnostic puzzle] Setting: [e.g. Indian district hospital emergency / tertiary OPD] My level: [brief]
TASK	1. Present only the chief complaint, age band, sex, setting and vitals. 2. Wait for me to ask history questions, examine or order tests; answer each realistically, revealing only what I specifically ask for. 3. Track my time and the tests ordered; note tests that aren't available in the setting. 4. Ask me for my differential after the history, and again after the examination. 5. When I commit to a final diagnosis and plan, reveal the true diagnosis. 6. Debrief: my key good decisions, the missed questions or signs, unnecessary tests, biases shown, and the ideal pathway.
RULES	Keep the case internally consistent and realistic for the setting, and don't reveal the diagnosis or unrequested information early, because the learning is in the searching. Mark management specifics in the debrief [VERIFY]. This is simulation for learning only.
OUTPUT	An interactive case, one exchange at a time; then a structured debrief table (my decision, ideal decision, comment).
TWEAK	End the debrief with the "illness script" of the diagnosis and the one clue that should have triggered it.

How to get more out of it

- › Say your differential aloud before each test. It reveals your reasoning.
- › Order tests only when you can say how the result will change your plan.
- › Run a new case every day for a month. Your reasoning speed will change noticeably.
- › → NEXT #951 Think aloud and get feedback on my reasoning

Approach to any presenting symptom

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT

For "approach to..." questions in theory, vivas and real practice.

WHY

"Approach to" questions test whether you can think systematically about a symptom, not recite a list of causes. A structured approach (define and characterise the symptom, categorise the causes, recognise red flags, use targeted history, examination and tests, and an algorithm) works for any symptom and any exam.

COPY THE PROMPT BELOW

PERSONA	You are a senior clinician and teacher famous for clear, systematic "approach to a symptom" teaching.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] Symptom or sign: [e.g. haemoptysis / syncope / pancytopenia / hepatomegaly / chronic cough] Setting and resources: [brief] My level and exam: [brief]
TASK	1. Define the symptom precisely and distinguish it from its mimics (e.g. haemoptysis versus haematemesis). 2. Classify the causes in a clinically useful framework (anatomical, pathophysiological or by acuity), including causes common in India. 3. List the red flags and can't-miss causes. 4. Give the targeted history questions and what each answer points to. 5. Give the focused examination and the key signs. 6. Give a stepwise investigation plan as an algorithm (first-line, second-line and specialised). 7. Give initial management principles and when to refer, marked [VERIFY]. 8. Summarise in a one-page flowchart description. 9. Give 3 classic exam-style mistakes students make when answering this approach, and how to avoid each. 10. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Keep the causes and algorithms consistent with standard textbooks and guidelines, marking thresholds [VERIFY], and emphasise the red flags, because approach questions test both breadth and safety.
OUTPUT	Nine headed sections, ending with the flowchart description and the common mistakes.
TWEAK	End with a 60-second spoken version of the approach for a viva.

How to get more out of it

- The same framework works for every symptom. Learn it once and apply it everywhere.
- Examiners reward "first I would confirm it's truly X and not Y". Start there.
- Draw the flowchart in theory answers. It often earns the most marks.
- → **NEXT #924** Red flags for a presenting symptom

Debrief my reasoning for cognitive bias

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · INTERN / PG RESIDENT

USE IT

After a case where your initial diagnosis was wrong, or after a simulator case.

WHY

Most diagnostic errors come from how we think, not what we know: anchoring, premature closure, availability, confirmation bias. Reviewing your own reasoning for these patterns, without blame, is how clinicians become safer diagnosticians.

COPY THE PROMPT BELOW

PERSONA	You are a diagnostic-reasoning expert who teaches cognitive bias and de-biasing strategies using real cases, in a supportive, non-judgemental way.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The case and how my thinking evolved (initial impression, tests, the eventual diagnosis), de-identified: [PASTE] What I felt at each stage (certainty, pressure, fatigue): [brief]
TASK	1. Map my reasoning timeline: each impression and the evidence I used. 2. Identify the likely biases at each point (anchoring, premature closure, availability, confirmation, framing, overconfidence, the diagnosis-momentum effect), with evidence from my account. 3. Identify the system and situational factors (workload, handover, fatigue, interruptions). 4. Suggest specific de-biasing strategies for next time (a diagnostic time-out, "what else could this be?", considering the opposite, checklists). 5. Distinguish the reasoning errors from reasonable decisions that simply had a bad outcome. 6. Write 3 personal learning points. 7. Suggest one short, safe way to share this learning with my team, such as at a morbidity meeting. 8. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Stay supportive and specific, without blame, and base the bias identification on my described thinking, because reflective practice only works when it feels safe.
OUTPUT	The reasoning timeline, the bias table (moment, bias, evidence), the system factors, the strategies, the distinction note, then the learning points.
TWEAK	End with one question I should ask myself on every future case with a similar presentation.

How to get more out of it

- › A good decision can have a bad outcome. Judge the reasoning, not the result.
- › "What else could this be?" before closing a diagnosis is the cheapest safety tool in medicine.
- › Keep a private reflection log. Patterns in your own thinking become visible over months.
- › → **NEXT #948** A diagnostic time-out before I close the case

Choose and interpret tests like an expert (Bayesian reasoning)

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT

When deciding which test to order and what the result really means.

WHY

A test result doesn't give a diagnosis; it changes a probability. Understanding pretest probability, likelihood ratios and thresholds for testing and treating prevents over-testing, false reassurance and misdiagnosis, and it answers a whole class of exam questions.

COPY THE PROMPT BELOW

PERSONA	You are a clinical epidemiologist and physician who teaches practical Bayesian reasoning at the bedside, using numbers only where they help.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The clinical scenario (de-identified): [PASTE] The diagnosis in question: [diagnosis] Tests I'm considering: [list] Known sensitivity, specificity or likelihood ratios, with their source: [LIST or "suggest with [VERIFY]"]
TASK	1. Estimate the pretest probability from the clinical features, or from a validated clinical decision rule if one exists, with the reasoning. 2. For each test, state its likelihood ratios (from my sources or marked [VERIFY]) and calculate the post-test probability for positive and negative results. 3. Explain the test and treatment thresholds for this diagnosis in plain language. 4. Recommend which test (if any) would change management, and why. 5. Explain the common pitfalls (spectrum bias, testing low-probability patients, false reassurance from an insensitive test). 6. Show how to explain the uncertainty to a patient in simple words.
RULES	Show all the calculations, mark every test-performance figure [VERIFY FROM SOURCE] unless I supplied it, and keep the conclusions for learning, because test characteristics vary between studies and populations.
OUTPUT	The pretest estimate, the calculation table (test, LR+, LR-, post-test positive, post-test negative), the threshold explanation, the recommendation, the pitfalls, then the patient explanation.
TWEAK	End with the one test in this scenario that would be most likely to mislead, and why.

How to get more out of it

- › Ask "will this result change what I do?" before ordering any test.
- › A negative result from a test with a moderate LR- doesn't rule out a high-probability diagnosis.
- › Learn the LRs of 10 common tests. They're used every day.
- › → **NEXT #923** Use likelihood ratios at the bedside

Reason through a management decision

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · INTERN / PG RESIDENT

USE IT

For management vivas and when learning how seniors make treatment decisions.

WHY

Management isn't reciting a guideline; it's weighing benefits, harms, patient factors, resources and preferences. Practising explicit management reasoning, with the trade-offs laid out, prepares you for "what would you do and why?" questions and for real decisions.

COPY THE PROMPT BELOW

PERSONA	You are a senior clinician who teaches management reasoning by making trade-offs explicit and patient-centred.
MY CONTEXT	Deadline or time I have for this: [brief] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Audience and time available: [brief] What I most want to get out of this: [one line] The case with the diagnosis established (de-identified): [PASTE] Patient factors (age, comorbidities, frailty, preferences, finances, social support): [brief] Setting and resources: [brief] The decision to make: [e.g. anticoagulate or not? admit or discharge? surgery or conservative?]
TASK	1. Frame the decision and the options clearly. 2. For each option, list the benefits, harms and uncertainties, with evidence or guideline support marked [VERIFY]. 3. Apply the patient factors and resources to each option. 4. Recommend the most reasonable option for learning purposes, with the reasoning. 5. Say what would change the recommendation (new information, patient preference). 6. Show how to discuss the options with the patient in shared decision-making. 7. List the monitoring and follow-up plan that goes with the recommended option. 8. Finish with a short list of anything you were unsure about, so I can verify it before use.
RULES	Present the options fairly, mark guideline specifics for verification, and state that the real decision belongs to the treating team with the patient, because this is a learning exercise.
OUTPUT	The decision framing, an options table (option, benefits, harms, uncertainties, fit for this patient), the recommendation, the change triggers, then the shared-decision script.
TWEAK	End with the "senior's answer": how an experienced clinician would summarise this decision in 2 sentences on rounds.

How to get more out of it

- › Always name the patient factor that tipped the decision. That's what examiners want to hear.
- › Include "doing nothing, or watchful waiting" as an option. It's often reasonable.
- › Practise saying the risks in absolute terms. Patients understand them better.
- › → **NEXT #950** Set management priorities (ABCDE)

918 Write a problem representation

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

USE IT After gathering the history and examination, before listing the differentials.

PERSONA	You are a clinical-reasoning tutor.
MY CONTEXT	The case details (de-identified): [PASTE]
TASK	1. Write a one-sentence problem representation: the patient context, the time course, and the key clinical syndrome using semantic qualifiers. 2. Explain the qualifiers used and why they matter for the differential. 3. Show how changing one qualifier (e.g. acute to chronic) changes the differential. 4. Critique my own representation, if I provide one.
RULES	Use only the case facts, and don't embed a diagnosis in the representation, because the representation should frame the reasoning rather than conclude it.
OUTPUT	The representation, the qualifier explanation, the "what if" example, then the critique.

PRO TIP "A 60-year-old smoker with 3 weeks of progressive, painless jaundice and weight loss" nearly diagnoses itself.

TRY IT WITH A 25-year-old woman with 2 days of fever, headache, vomiting and neck stiffness.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

919 Practise semantic qualifiers

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT

USE IT To learn the vocabulary expert clinicians use to frame cases.

PERSONA	You are a clinical-reasoning teacher.
MY CONTEXT	My level: [brief] Practice cases or findings (paste, or ask me to generate them): [PASTE / GENERATE]
TASK	1. List the common semantic-qualifier pairs (acute or chronic, unilateral or bilateral, painful or painless, intermittent or constant, proximal or distal, focal or diffuse, and so on). 2. For each practice case, convert the story into qualifiers. 3. Show how the qualifiers narrow the differential. 4. Give me 3 cases to convert myself, and check my answers.
RULES	Keep the case interpretations accurate, because qualifiers must reflect the data.
OUTPUT	The qualifier list, the worked conversions, then the practice cases; feedback after my attempt.

PRO TIP Pairs such as "painless" versus "painful" jaundice split a differential in half.

TRY IT WITH Cases of jaundice, joint pain and weakness.

→ **NEXT #918** Write a problem representation

920 Generate causes systematically (VINDICATE)

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT

USE IT When asked to list the causes of anything and your mind goes blank.

PERSONA	You are a clinical tutor who teaches systematic cause generation.
MY CONTEXT	The finding or condition: [e.g. hepatomegaly, peripheral neuropathy]
TASK	1. Apply a systematic framework (e.g. VINDICATE: vascular, infective, neoplastic, drugs and toxins, inflammatory or immune, congenital, autoimmune, trauma, endocrine and metabolic) to list the causes. 2. Mark the common causes, the can't-miss causes and the India-relevant causes. 3. Reorganise the list by likelihood for a typical adult in India.
RULES	Keep the causes accurate and relevant, and avoid padding with vanishingly rare causes, because a systematic list is only useful if it's prioritised.
OUTPUT	The framework table, the marked causes, then the prioritised list.

PRO TIP Use the framework to generate, then re-rank by likelihood. Exams and patients need the ranking.

TRY IT WITH Finding: splenomegaly.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

921 Localise the lesion (neurology)

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For neurology cases, where localisation comes before the differential.

PERSONA	You are a neurologist who teaches anatomical localisation step by step.
MY CONTEXT	The neurological findings (de-identified): [PASTE]
TASK	1. Determine the level: brain, brainstem, spinal cord, root, plexus, nerve, neuromuscular junction or muscle, with the reasoning from each finding. 2. Determine the side and whether the lesion is focal, multifocal or diffuse. 3. Name the most likely localisation. 4. Generate the differential for that localisation, using the time course. 5. Suggest the confirming investigation.
RULES	Reason step by step from the findings, and state any finding that doesn't fit, because localisation errors derail the whole differential.
OUTPUT	The level analysis, the side and pattern, the localisation, the differential, then the investigation.

PRO TIP Upper versus lower motor neuron signs are the first fork. Get that right first.

TRY IT WITH Weakness of both legs with brisk reflexes, a sensory level at T10 and urinary retention.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

922 Estimate a pretest probability

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT Before ordering tests for a suspected diagnosis.

PERSONA	You are a clinical epidemiologist.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The scenario (de-identified): [PASTE] The suspected diagnosis: [diagnosis]
TASK	1. Identify the features that raise and lower the probability. 2. Use a validated clinical decision rule, if one exists for this diagnosis, and show the calculation, marked [VERIFY RULE CRITERIA]. 3. Give a qualitative pretest category (low, intermediate or high) with the reasoning. 4. State what that category means for testing.
RULES	Present the estimates as approximate and mark decision-rule criteria for verification, because exact numbers vary between populations.
OUTPUT	The feature list, the rule calculation, the category, then the testing implication.

PRO TIP Low-probability patients with a positive test are where false positives live.

TRY IT WITH Suspected pulmonary embolism in a post-operative patient with tachycardia.

→ **NEXT #916** Choose and interpret tests like an expert (Bayesian reasoning)

923 Use likelihood ratios at the bedside

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT To learn how findings and tests shift probability.

PERSONA	You are an evidence-based medicine tutor.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The findings or tests I want to learn about: [list] Clinical context: [brief]
TASK	1. Explain LR+ and LR- in plain language, with the rough effect of LRs of 2, 5 and 10 (and 0.5, 0.2 and 0.1) on probability. 2. Give the LRs for my findings, marked [VERIFY FROM SOURCE]. 3. Work through one example from pretest to post-test probability. 4. Give 3 practice problems.
RULES	Mark every LR value for verification, because published LRs vary between studies.
OUTPUT	The explanation, the LR table, the worked example, then the practice problems.

PRO TIP A rough rule: an LR of 10 raises the probability by about 45 percentage points, and an LR of 0.1 lowers it by about the same.

TRY IT WITH Findings in suspected acute appendicitis.

→ **NEXT #916** Choose and interpret tests like an expert (Bayesian reasoning)

924 Red flags for a presenting symptom

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

USE IT For any common symptom, to know when it is dangerous.

PERSONA	You are an emergency physician who teaches red-flag recognition.
MY CONTEXT	Symptom: [e.g. headache, back pain, chest pain, fever in a child] Setting: [OPD / emergency / primary care]
TASK	1. List the red-flag features, and the serious diagnosis each suggests. 2. Give the question or examination step that detects each. 3. Give the immediate action or referral for each, marked [VERIFY LOCAL PROTOCOL]. 4. Give a memory aid if a standard one exists.
RULES	Keep the red flags consistent with standard teaching and guidelines, and mark actions for verification, because red-flag lists guide urgent decisions.
OUTPUT	A red-flag table (feature, suggests, how to detect, action), then the memory aid.

PRO TIP Ask the red-flag questions in every patient with this symptom, even when you think it's benign.

TRY IT WITH Symptom: headache in adults.

→ **NEXT #925** Can't-miss diagnoses for a presentation

925 Can't-miss diagnoses for a presentation

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT For any presentation, as a safety check before settling on a diagnosis.

PERSONA	You are a senior emergency physician focused on diagnostic safety.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The presentation (de-identified): [PASTE]
TASK	1. List the can't-miss diagnoses for this presentation. 2. For each, give the clinical features that suggest it and the fastest reliable way to exclude it. 3. Mark which ones the current information has already excluded. 4. Give the order in which to exclude them.
RULES	Be explicit about what counts as adequate exclusion, and mark tests and thresholds [VERIFY], because an assumed exclusion is not an exclusion.
OUTPUT	A can't-miss table (diagnosis, suggestive features, how to exclude, excluded yes or no), then the order.

PRO TIP For chest pain, the classic can't-miss list includes ACS, aortic dissection, pulmonary embolism, tension pneumothorax, tamponade and oesophageal rupture.

TRY IT WITH A 35-year-old with sudden severe headache during exercise.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

926 The next best investigation

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For "what is the next best step?" questions and real decisions.

PERSONA	You are a clinical teacher who explains test selection.
MY CONTEXT	The scenario and results so far (de-identified): [PASTE]
TASK	1. State the leading diagnoses at this point. 2. Recommend the next best investigation, and explain why it beats the alternatives (discriminating power, availability, safety, cost). 3. Explain the difference between the "best initial test", the "most accurate test" and the "next best step" for this scenario. 4. Give the expected results for each leading diagnosis.
RULES	Reason from the scenario and standard guidelines, marked [VERIFY] , because "next best step" answers depend on the exact stage of the case.
OUTPUT	The leading diagnoses, the recommendation with reasoning, the test-type distinction, then the expected results.

PRO TIP Exams separate the "best initial test" from the "gold standard". Read the question carefully.

TRY IT WITH Suspected acute cholecystitis with an equivocal ultrasound.

→ **NEXT #916** Choose and interpret tests like an expert (Bayesian reasoning)

927 Interpret a lab pattern in clinical context

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT When lab results come back and you need to make sense of them.

PERSONA	You are a clinical pathologist and physician.
MY CONTEXT	Lab results with dates (de-identified): [PASTE] The clinical context: [brief]
TASK	1. Identify the abnormal values and the pattern (e.g. hepatocellular versus cholestatic, pre-renal versus renal). 2. Interpret the pattern in the clinical context. 3. Give the differential suggested by the pattern. 4. Suggest the next tests to clarify it. 5. Flag any value that is critical and needs urgent action.
RULES	Use reference ranges marked [VERIFY WITH LAB] , and reason step by step, because patterns only make sense in context.
OUTPUT	The abnormal values, the pattern, the interpretation, the differential, the next tests, then the critical flags.

PRO TIP Trends tell more than single values. Always compare with the previous result.

TRY IT WITH Raised ALP and GGT, mildly raised ALT, and a raised direct bilirubin in a 55-year-old with pruritus.

→ **NEXT #926** The next best investigation

928 Reason through an acid-base problem

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For ABG interpretation in exams and on the wards.

PERSONA	You are a critical-care physician who teaches acid-base analysis systematically.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] ABG and electrolyte values (de-identified): [PASTE] The clinical context: [brief]
TASK	1. Check the internal consistency of the values. 2. Identify the primary disorder. 3. Check compensation using the standard formulas (marked [VERIFY]). 4. Calculate the anion gap (and albumin correction) and the delta ratio where relevant. 5. Identify mixed disorders. 6. Give the differential for each disorder in this clinical context.
RULES	Show every calculation, and use standard formulas marked for verification, because acid-base errors usually come from skipped steps.
OUTPUT	Step-by-step working, then the final interpretation and differential.

PRO TIP Always calculate the anion gap, even when the pH looks simple. Hidden mixed disorders show up there.

TRY IT WITH pH 7.25, PaCO₂ 24, HCO₃ 10, Na 138, Cl 100, albumin 2.5, in a diabetic patient with vomiting.

→ **NEXT #927** Interpret a lab pattern in clinical context

929 Interpret an ECG in clinical context

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT When an ECG finding must be interpreted alongside the patient's presentation.

PERSONA	You are a cardiologist who teaches contextual ECG interpretation.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] ECG findings (describe, or upload a de-identified tracing): [DESCRIBE / UPLOAD] The clinical context: [brief]
TASK	1. Interpret the ECG systematically. 2. Relate the findings to the clinical context. 3. Give the differential for the key abnormality. 4. State the urgency and the immediate next steps, marked [VERIFY].
RULES	Describe what is seen before interpreting, and mark uncertain readings [CONFIRM WITH A SENIOR OR CARDIOLOGIST], because ECG misreads can be dangerous.
OUTPUT	The systematic interpretation, the clinical correlation, the differential, then the urgency and next steps.

PRO TIP The same ST elevation means different things in a young athlete and a diabetic with chest pain. Context is everything.

TRY IT WITH Widespread ST elevation with PR depression in a young man with pleuritic chest pain.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

930 Interpret imaging in clinical context

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT When an imaging report or image must be integrated into the clinical picture.

PERSONA	You are a radiologist who teaches clinicians to integrate imaging findings.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The imaging findings or report (de-identified): [PASTE] The clinical context: [brief]
TASK	1. Summarise the key findings in plain language. 2. Relate them to the clinical picture: consistent, inconsistent or incidental. 3. Give the differential for the imaging findings in this context. 4. Suggest further imaging or correlation if needed.
RULES	Stick to the reported findings and don't over-interpret, marking uncertainty [DISCUSS WITH RADIOLOGIST], because clinicians often over-read reports.
OUTPUT	The findings summary, the correlation, the differential, then the next steps.

PRO TIP Always read the "impression" and the "findings" sections of a report. They sometimes disagree.

TRY IT WITH CT chest showing a cavitating lesion in the right upper lobe in a diabetic with fever.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

931 Approach to fever, including PUO

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For fever, one of the commonest presentations in Indian practice.

PERSONA	You are an infectious-diseases physician experienced in Indian tropical medicine.
MY CONTEXT	The fever case (de-identified): duration, pattern, associated features, exposures, travel, season and examination: [PASTE] Setting: [brief]
TASK	1. Classify the fever: acute undifferentiated, fever with localising features, or prolonged fever or PUO. 2. Give the differential weighted by local epidemiology and season (e.g. dengue, malaria, scrub typhus, typhoid, leptospirosis, TB). 3. Give the discriminating history and examination features. 4. Give a stepwise investigation plan appropriate to the setting. 5. Give the red flags and the empirical-treatment considerations, marked [VERIFY].
RULES	Use local epidemiology and mark treatment specifics for verification, because tropical fevers overlap and management differs.
OUTPUT	The classification, the weighted differential, the discriminators, the investigation plan, then the red flags.

PRO TIP An eschar search belongs in every fever examination in India. Scrub typhus is missed without it.

TRY IT WITH A 30-year-old farmer with 6 days of fever, myalgia, headache and conjunctival suffusion in the monsoon.

→ **NEXT #943** Tropical infection differential (Indian setting)

932 Approach to chest pain

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT For chest pain in emergency, OPD and exams.

PERSONA	You are an emergency physician and cardiologist.
MY CONTEXT	The chest pain case (de-identified): [PASTE]
TASK	1. Exclude the life-threatening causes first (ACS, aortic dissection, PE, tension pneumothorax, tamponade, oesophageal rupture). 2. Characterise the pain (site, onset, character, radiation, associated features, timing, exacerbating and relieving factors, severity). 3. Give the ranked differential with the reasoning. 4. Give the immediate investigations (ECG within 10 minutes, troponin, chest X-ray, and others), marked [VERIFY]. 5. Give a risk-stratification tool if applicable, marked [VERIFY].
RULES	Put life-threatening causes first in the reasoning, because chest pain is where missed diagnoses kill.
OUTPUT	The life-threat check, the characterisation, the differential, the investigations, then the risk stratification.

PRO TIP A normal first ECG and troponin don't exclude ACS. Serial testing matters.

TRY IT WITH A 50-year-old diabetic with 2 hours of epigastric discomfort and sweating.

→ **NEXT #925** Can't-miss diagnoses for a presentation

933 Approach to breathlessness

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For acute and chronic breathlessness.

PERSONA	You are a pulmonologist and internist.
MY CONTEXT	The breathlessness case (de-identified): acute or chronic, associated features, examination and vitals: [PASTE]
TASK	1. Separate the acute and chronic approaches. 2. Categorise the causes: airway, parenchyma, pleura, vasculature, cardiac, chest wall and neuromuscular, and metabolic or haematological. 3. Give the discriminating clinical features. 4. Give the investigation plan (SpO₂ and ABG, chest X-ray, ECG, echo, PFT, D-dimer or CTPA as appropriate, marked [VERIFY]). 5. Give the red flags needing immediate action.
RULES	Put hypoxaemia and life-threatening causes first, because respiratory decline can be rapid.
OUTPUT	The acute and chronic frameworks, the cause categories, the discriminators, the investigations, then the red flags.

PRO TIP Is it the lungs, the heart, or neither? That first split guides everything else.

TRY IT WITH A 65-year-old smoker with 2 days of worsening breathlessness and wheeze.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

934 Approach to altered sensorium

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT For patients with confusion, delirium or coma.

PERSONA	You are a neuro-critical-care physician.
MY CONTEXT	The case (de-identified): onset, GCS, vitals, focal signs, history and medications: [PASTE]
TASK	1. Give the immediate stabilisation priorities (ABCDE, glucose, and so on), marked [VERIFY LOCAL PROTOCOL]. 2. Categorise the causes: structural versus metabolic or toxic versus infective, using focal signs and history. 3. Give the ranked differential, including India-relevant causes (e.g. cerebral malaria, snakebite, organophosphate poisoning, Japanese encephalitis). 4. Give the investigation plan. 5. Give the reversible causes to treat empirically while awaiting results, marked [VERIFY].
RULES	Put immediate stabilisation and reversible causes first, because altered sensorium can be rapidly fatal.
OUTPUT	The stabilisation steps, the categorisation, the differential, the investigations, then the reversible causes.

PRO TIP Check the glucose first, always. Hypoglycaemia is the fastest reversible cause.

TRY IT WITH A 40-year-old farmer found drowsy with pinpoint pupils and excessive secretions.

→ **NEXT #925** Can't-miss diagnoses for a presentation

935 Approach to acute abdominal pain

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT For acute abdominal pain in emergency and surgical practice.

PERSONA	You are a general surgeon and emergency physician.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The case (de-identified): site, onset, character, associated features, vitals and examination: [PASTE]
TASK	1. Identify the surgical emergencies to exclude (perforation, obstruction, ischaemia, ruptured AAA, ectopic pregnancy, peritonitis). 2. Give the differential by pain location and character. 3. Give the discriminating findings. 4. Give the investigation plan (bloods, pregnancy test where relevant, imaging), marked [VERIFY]. 5. Say when to call the surgeon.
RULES	Include extra-abdominal causes (MI, DKA, pneumonia) and gynaecological causes, because they are commonly missed.
OUTPUT	The emergency check, the location-based differential, the discriminators, the investigations, then the escalation triggers.

PRO TIP Pain out of proportion to the examination findings suggests mesenteric ischaemia. Don't be falsely reassured.

TRY IT WITH A 22-year-old woman with right iliac fossa pain and a missed period.

→ **NEXT #925** Can't-miss diagnoses for a presentation

936 Approach to anaemia

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For anaemia on any blood report.

PERSONA	You are a haematologist who teaches a stepwise approach.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] CBC and indices, the peripheral smear if available, and the clinical context (de-identified): [PASTE]
TASK	1. Classify the anaemia by MCV and reticulocyte count. 2. Give the differential for each category, weighted for India (iron deficiency, nutritional deficiency, haemoglobinopathies, chronic disease, haemolysis). 3. Give the targeted investigations. 4. Interpret my patient's values through this framework.
RULES	Mark reference ranges [VERIFY WITH LAB], and reason step by step, because anaemia work-ups go wrong when steps are skipped.
OUTPUT	The classification, the differential table, the investigations, then my patient's interpretation.

PRO TIP The reticulocyte count separates "not making enough" from "losing or destroying too many". Check it early.

TRY IT WITH Hb 7.8, MCV 64, RDW high, in a 19-year-old woman with menorrhagia.

→ **NEXT #927** Interpret a lab pattern in clinical context

937 Approach to jaundice

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For jaundice in exams and practice.

PERSONA	You are a gastroenterologist and hepatologist.
MY CONTEXT	The case (de-identified): the bilirubin fractions, liver enzymes, clinical features and history: [PASTE]
TASK	1. Classify it as pre-hepatic, hepatic or post-hepatic using the bilirubin fractions and enzyme pattern. 2. Give the differential for each category, including India-relevant causes (viral hepatitis, including hepatitis E in pregnancy; malaria; leptospirosis; drug-induced, including antitubercular therapy). 3. Give the discriminating history and examination features. 4. Give the investigation plan. 5. Give the red flags (liver failure, cholangitis), marked [VERIFY].
RULES	Reason from the lab pattern and the history, and mark thresholds for verification, because jaundice has overlapping causes.
OUTPUT	The classification, the differential, the discriminators, the investigations, then the red flags.

PRO TIP Painless progressive jaundice with weight loss in an older patient points to malignancy until proven otherwise.

TRY IT WITH A 45-year-old on antitubercular treatment for 6 weeks with new jaundice and vomiting.

→ **NEXT #944** Could a drug be causing this?

938 Approach to acute kidney injury

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For a rising creatinine or reduced urine output.

PERSONA	You are a nephrologist who teaches a structured AKI approach.
MY CONTEXT	The case (de-identified): creatinine trend, urine output, urinalysis, medications, fluid status and context: [PASTE]
TASK	1. Confirm and stage the AKI using standard criteria, marked [VERIFY CRITERIA]. 2. Categorise it as pre-renal, intrinsic renal or post-renal, with the evidence. 3. Give the differential in this context (sepsis, nephrotoxins, contrast, obstruction; and India-relevant causes such as snakebite, leptospirosis and malaria). 4. Give the investigations (urinalysis, urine indices, ultrasound). 5. Give the immediate management principles and the indications for dialysis, marked [VERIFY].
RULES	Reason from the data and mark criteria and thresholds for verification, because AKI staging and indications must be precise.
OUTPUT	The staging, the categorisation, the differential, the investigations, then the management principles.

PRO TIP Always review the medication chart in AKI. NSAIDs, ACE inhibitors and aminoglycosides are frequent culprits.

TRY IT WITH A creatinine rising from 1.0 to 2.6 over 48 hours in a septic patient on vancomycin.

→ **NEXT #927** Interpret a lab pattern in clinical context

939 Approach to hyponatraemia

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For low sodium, a common and often mismanaged problem.

PERSONA	You are an endocrinologist and internist.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Serum sodium, osmolality, urine osmolality, urine sodium, volume status, medications and symptoms (de-identified): [PASTE]
TASK	1. Exclude pseudo-hyponatraemia and hyperglycaemia. 2. Assess the severity and symptoms, and the urgency. 3. Categorise using osmolality, volume status and the urine studies. 4. Give the differential (SIADH, diuretics, hypovolaemia, heart failure, cirrhosis, hypothyroidism, adrenal insufficiency). 5. Give the correction principles and the safe correction limits, marked [VERIFY].
RULES	Mark the correction-rate limits for verification, because over-rapid correction causes osmotic demyelination.
OUTPUT	The exclusion step, the severity, the categorisation, the differential, then the correction principles.

PRO TIP Always assess volume status clinically. It splits the differential into three.

TRY IT WITH Na 121 in an elderly woman on a thiazide diuretic, confused, clinically euvolaemic.

→ **NEXT #928** Reason through an acid-base problem

940 Age-specific differential in children

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT (PAEDIATRICS)

USE IT For paediatric cases, where causes vary sharply with age.

PERSONA	You are a paediatrician who teaches age-based differential thinking.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The presentation and the child's age (de-identified): [PASTE]
TASK	1. Give the differential by age group (neonate, infant, preschool, school age, adolescent) for this presentation. 2. Highlight the most likely causes at this child's age. 3. Give the paediatric-specific red flags. 4. Give the investigations appropriate for age.
RULES	Keep the age-specific causes accurate and mark doses and thresholds [VERIFY], because paediatric reasoning errors are common when adult frameworks are applied.
OUTPUT	An age-group differential table, the likely causes for this child, the red flags, then the investigations.

PRO TIP A neonate isn't a small child, and a child isn't a small adult. Age changes everything.

TRY IT WITH A 2-year-old with fever and a seizure.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

941 Differential diagnosis in pregnancy

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT When a pregnant woman presents with a symptom that may be pregnancy-specific.

PERSONA	You are an obstetric physician.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The presentation, gestation and history (de-identified): [PASTE]
TASK	1. Separate the pregnancy-specific causes (pre-eclampsia, HELLP, acute fatty liver of pregnancy, and others) from those that are not. 2. Give the discriminating features and the tests. 3. Give the investigations safe in pregnancy. 4. Give the red flags needing urgent obstetric input, marked [VERIFY].
RULES	Consider the effects on both mother and fetus, and mark drug and imaging safety specifics for verification, because pregnancy changes both the differential and the safe management.
OUTPUT	The two-category differential, the discriminators, the safe investigations, then the red flags.

PRO TIP Headache with raised BP in the third trimester is pre-eclampsia until proven otherwise.

TRY IT WITH A 32-week pregnant woman with epigastric pain, raised liver enzymes and low platelets.

→ **NEXT #925** Can't-miss diagnoses for a presentation

942 Atypical presentations in the elderly

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For older patients, where serious disease presents atypically.

PERSONA	You are a geriatrician.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The presentation of an older patient (de-identified): [PASTE]
TASK	1. List the serious conditions that present atypically in the elderly (e.g. infection as confusion or falls, MI without chest pain). 2. Give the differential for this presentation. 3. Include medication-related causes (polypharmacy). 4. Include frailty, function and delirium assessment. 5. Give the investigation priorities.
RULES	Reason with atypical presentation in mind, and include a medication review, because "just old age" is a dangerous conclusion.
OUTPUT	The atypical-presentation list, the differential, the medication review, the assessments, then the investigations.

PRO TIP New confusion in an older person is delirium until proven otherwise. Look for the trigger.

TRY IT WITH An 82-year-old who stopped eating and became confused over 2 days.

→ **NEXT #944** Could a drug be causing this?

943 Tropical infection differential (Indian setting)

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT For acute febrile illness with multisystem features in Indian practice.

PERSONA	You are a tropical-medicine physician experienced in India.
MY CONTEXT	The case (de-identified): fever duration, season, occupation or exposures, rash or eschar, organ involvement and lab results: [PASTE]
TASK	1. Compare dengue, malaria, scrub typhus, leptospirosis, enteric fever and other relevant local infections for this case. 2. Build a discriminating-feature table (e.g. thrombocytopenia, eschar, conjunctival suffusion, transaminases, renal involvement). 3. Give the tests for each (e.g. NSI or IgM, rapid tests and smear, IgM ELISA), with timing considerations. 4. Give the empirical-treatment considerations while tests are pending, marked [VERIFY CURRENT GUIDELINES].
RULES	Base the differential on local epidemiology, and mark the test performance and treatment specifics for verification, because overlapping tropical fevers need careful discrimination.
OUTPUT	The comparison, the discriminating table, the test guide, then the empirical-treatment considerations.

PRO TIP Co-infections happen. A positive malaria test doesn't exclude dengue or scrub typhus.

TRY IT WITH Fever for 7 days, thrombocytopenia, raised transaminases and AKI in a farmer in September.

→ **NEXT #931** Approach to fever, including PUO

944 Could a drug be causing this?

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT For any new symptom or lab abnormality in a patient on medications.

PERSONA	You are a clinical pharmacologist.
MY CONTEXT	The patient's medication list (with start dates) and the new problem (de-identified): [PASTE]
TASK	1. Identify which drugs could cause this problem, and how. 2. Assess the timing and plausibility for each. 3. Suggest a causality assessment approach (e.g. the WHO-UMC criteria). 4. Suggest the next steps (stop, substitute or monitor), marked [DISCUSS WITH THE TREATING TEAM]. 5. Remind me to report suspected adverse drug reactions to the pharmacovigilance programme.
RULES	Base the associations on known adverse-effect profiles and mark uncertain ones [VERIFY], and don't advise stopping drugs in a real patient without the treating team, because drug changes carry risks.
OUTPUT	A drug-problem table, the plausibility assessment, the causality approach, then the next steps.

PRO TIP "Is this a drug side effect?" should be asked for every new problem. It's often the answer.

TRY IT WITH New hyperkalaemia in a patient started on spironolactone and an ACE inhibitor.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

945 Recognise the syndrome from scattered findings

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT When findings seem unrelated but may form a syndrome.

PERSONA	You are a diagnostician who excels at pattern recognition.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The findings (de-identified): [PASTE]
TASK	1. Group the findings into clusters. 2. Suggest the unifying diagnoses that explain most or all of them (Occam's razor), and the alternative of several diagnoses (Hickam's dictum). 3. Give the confirming tests for the leading unifying diagnosis. 4. Say which findings don't fit.
RULES	Consider both unifying and multiple diagnoses, and reason explicitly, because forcing one diagnosis can mislead.
OUTPUT	The clusters, the unifying diagnoses, the confirming tests, then the misfits. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP "What single disease explains all of this?" is worth asking, and so is "does it have to be one disease?".

TRY IT WITH A young woman with a rash, joint pain, proteinuria, a pleural effusion and low platelets.

→ **NEXT #912** Build illness scripts for diseases

946 Discriminate between two close diagnoses

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT When your differential comes down to two possibilities.

PERSONA	You are a clinical teacher who specialises in fine discrimination.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The two diagnoses and the case details (de-identified): [PASTE]
TASK	1. Compare the two diagnoses on history, examination, labs and imaging. 2. Identify the single most discriminating feature and test. 3. Apply the comparison to my case. 4. Say what result would favour each.
RULES	Base the discrimination on standard knowledge and mark test-performance claims [VERIFY], because close differentials hinge on precise features.
OUTPUT	The comparison table, the key discriminator, the case application, then the result interpretation.

PRO TIP Exams love pairs: Crohn's versus UC, SIADH versus cerebral salt wasting, dengue versus leptospirosis.

TRY IT WITH Tuberculous versus malignant pleural effusion in a 60-year-old ex-smoker.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

947 Apply a clinical decision rule

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT When a validated rule (Wells, CURB-65, CHA2DS2-VASc, Glasgow-Blatchford, qSOFA) applies.

PERSONA	You are an evidence-based physician who teaches the correct use of decision rules.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The rule: [name] The case details (de-identified): [PASTE]
TASK	1. List the rule's criteria, marked [VERIFY CRITERIA FROM ORIGINAL SOURCE]. 2. Score my case step by step. 3. Interpret the score and the recommended action. 4. State the rule's limitations and the populations in which it wasn't validated.
RULES	Mark the criteria and cut-offs for verification, and remind me that rules support rather than replace judgement, because misremembered criteria give wrong scores.
OUTPUT	The criteria list, the scoring, the interpretation, then the limitations.

PRO TIP Know when not to use a rule, for example scores validated in adults applied to children, or in pregnancy.

TRY IT WITH CURB-65 for a 70-year-old with pneumonia, confusion, a urea of 9 and a BP of 95/60.

→ **NEXT #922** Estimate a pretest probability

948 A diagnostic time-out before I close the case

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · INTERN / PG RESIDENT

USE IT Before finalising a diagnosis, especially in complex or high-risk cases.

PERSONA	You are a patient-safety physician who teaches reflective diagnostic checks.
MY CONTEXT	My working diagnosis and the case summary (de-identified): [PASTE]
TASK	1. Run a structured time-out: What else could this be? What doesn't fit? Did I consider the can't-miss diagnoses? Am I anchoring on the first impression? Would I be comfortable if a senior reviewed this? 2. Answer each question for my case. 3. Identify any action needed before closure (a test, a senior review, observation).
RULES	Answer honestly from the case details and highlight any unresolved finding, because the time-out only works if it can change the diagnosis.
OUTPUT	The time-out questions with answers, then the actions needed.

PRO TIP Taking 60 seconds for a time-out on every admission diagnosis prevents a surprising number of errors.

TRY IT WITH Working diagnosis: gastroenteritis in a 50-year-old diabetic with vomiting and abdominal pain.

→ **NEXT #915** Debrief my reasoning for cognitive bias

949 Turn a ward case into exam practice

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / PG RESIDENT

USE IT After seeing an interesting patient, to convert it into exam preparation.

PERSONA	You are a clinical teacher who turns ward cases into exam practice.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The case (de-identified): [PASTE] My exam: [brief]
TASK	1. Write the case as a long-case presentation summary. 2. List the 10 viva questions examiners would ask about this case, with short answers. 3. Write 3 MCQs based on the case. 4. Identify the theory questions this case could answer.
RULES	Keep the case fully de-identified and the answers accurate, marking specifics [VERIFY], because real cases become memorable exam material.
OUTPUT	The summary, the viva Q&A, the MCQs, then the theory links.

PRO TIP Patients you have seen are remembered far better than textbook cases. Convert every interesting one.

TRY IT WITH A patient with infective endocarditis and a splinter haemorrhage, seen on rounds.

→ **NEXT #862** Long case: presentation and examiner questions

950 Set management priorities (ABCDE)

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

USE IT For sick patients, and for "what will you do first?" questions.

PERSONA	You are an emergency physician who teaches prioritisation.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The sick patient scenario (de-identified): [PASTE]
TASK	1. Apply the ABCDE approach with specific actions at each step. 2. Identify the immediate life threats and their treatment. 3. Order the investigations by urgency. 4. List when to call for senior help. 5. Plan the reassessment after each intervention.
RULES	Mark treatment specifics and doses [VERIFY LOCAL PROTOCOL], because emergency management must follow approved protocols.
OUTPUT	The ABCDE actions, the life threats, the prioritised investigations, the escalation triggers, then the reassessment plan.

PRO TIP Fix problems as you find them in ABCDE order. Don't wait to finish the assessment.

TRY IT WITH A patient with BP 80/50, HR 130, RR 30, SpO2 88% and cold peripheries after 3 days of fever.

→ **NEXT #911** Prioritised differential with can't-miss diagnoses

951 Think aloud and get feedback on my reasoning

STANDARD

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN / PG RESIDENT

USE IT To practise articulating clinical reasoning, as on rounds and in vivas.

PERSONA	You are a clinical-reasoning coach who evaluates think-aloud reasoning.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] The case (de-identified) and my think-aloud reasoning (typed as I would say it): [PASTE]
TASK	1. Map my reasoning steps. 2. Identify the strengths (e.g. good problem representation, considering danger). 3. Identify the gaps (missed data, a premature conclusion, unjustified tests). 4. Rewrite my reasoning as an expert would say it. 5. Give 2 habits to practise.
RULES	Quote my words for every comment and stay constructive, because reasoning feedback must be specific to change habits.
OUTPUT	The reasoning map, the strengths, the gaps, the expert version, then the habits.

PRO TIP Saying your reasoning aloud on rounds, even when unsure, is how seniors can teach you.

TRY IT WITH My reasoning on a patient with fever and a new heart murmur.

→ **NEXT #913** Interactive patient case simulator

952 Top three diagnoses in one line

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

Act as a senior physician teaching on rounds. For this de-identified presentation, [PASTE KEY FEATURES], give the top 3 differentials ranked, each with the single strongest supporting feature, in one line each, and flag any can't-miss diagnosis that isn't among them.

TRY IT WITH A 60-year-old smoker with haemoptysis, weight loss and clubbing.

953 Can't-miss diagnoses for a symptom

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

Act as an emergency physician. List the can't-miss diagnoses for [SYMPTOM] in [AGE GROUP], each with one feature that suggests it and the fastest test that excludes it. Mark tests and thresholds [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH Sudden-onset breathlessness in young adults.

954 The one history question that helps most

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

Act as a clinical-reasoning tutor. For this presentation, [PASTE FEATURES], with the differential [LIST], tell me the single most discriminating history question to ask next and how each possible answer would shift the differential. Flag anything you are unsure of so I can check it.

TRY IT WITH Chronic cough with a differential of asthma, GERD, post-nasal drip and TB.

955 The sign I must look for

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

Act as a clinical examiner. For a patient with [PRESENTATION] and a differential of [LIST], tell me the one or two examination signs that would best discriminate between them, how to elicit each correctly, and what a positive finding means.

TRY IT WITH Breathlessness with a differential of heart failure, COPD and pulmonary fibrosis.

956 The first investigation to order

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

Act as a senior resident. For this presentation, [PASTE FEATURES], in a [SETTING], name the first investigation to order, why it comes first (speed, availability, yield), and what each result would mean. Mark thresholds [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH Suspected meningitis in a district-hospital emergency department.

957 Interpret one abnormal result

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS STUDENT / INTERN

Act as a clinical pathologist. Interpret this result, [RESULT WITH UNITS], in the context of [CLINICAL CONTEXT]: what it suggests, the top 3 causes in this context, and the next test. Mark reference ranges [VERIFY WITH LAB]. Flag anything you are unsure of so I can check it.

TRY IT WITH Serum potassium of 6.4 in a patient with CKD and no ECG changes.

958 What doesn't fit the diagnosis?

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / PG RESIDENT

Act as a sceptical senior clinician. For the working diagnosis [DIAGNOSIS] and these findings, [PASTE FINDINGS], list the findings that don't fit it, what each might suggest instead, and whether they change the plan. Flag anything you are unsure of so I can check it.

TRY IT WITH Diagnosis: viral fever. Findings include a new murmur and splinter haemorrhages.

959 Name the syndrome from the findings

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / PG RESIDENT

Act as a diagnostician. Which syndrome or unifying diagnosis best explains these findings, [PASTE FINDINGS]? Give the most likely answer, one alternative, and the single confirmatory test, with a one-line reason for each. Flag anything you are unsure of so I can check it.

TRY IT WITH Hypertension, hypokalaemia and metabolic alkalosis in a 35-year-old.

960 Causes by category in one minute

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT

Act as a clinical tutor. Using the VINDICATE categories, list the causes of [FINDING] in a quick grouped list, marking the 3 most common in India with an asterisk and the can't-miss ones with an exclamation mark. Flag anything you are unsure of so I can check it.

TRY IT WITH Finding: generalised lymphadenopathy.

961 The top causes at this age

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / PG RESIDENT

Act as a clinician. For [PRESENTATION] in a [AGE] patient, give the 5 most likely causes for this age group in India, ranked, with one line each on why age changes the likelihood. Flag anything you are unsure of so I can check it.

TRY IT WITH Seizures in a 70-year-old.

962 India-specific causes to consider

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / PG RESIDENT

Act as an internist experienced in Indian practice. For [PRESENTATION], list the causes more common in India than in Western textbooks (infections, toxins, nutritional causes, bites and envenomation) that I must consider, with one clue for each. Flag anything you are unsure of so I can check it.

TRY IT WITH Acute kidney injury in a rural patient.

963 Quick red-flag checklist

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / INTERN

Act as an emergency physician. Give me a quick red-flag checklist for [SYMPTOM], each flag in under 8 words with the serious cause it suggests, formatted as a pocket list I can use in the OPD. Mark actions [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH Symptom: low back pain.

964 Which test rules it out?

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / PG RESIDENT

Act as an evidence-based physician. For excluding [DIAGNOSIS] in a [LOW / INTERMEDIATE] probability patient, name the test with the best negative predictive value in that setting, its limitation, and when a negative result isn't enough. Mark test performance [VERIFY].

TRY IT WITH Excluding pulmonary embolism with D-dimer.

965 From pretest to post-test probability

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / PG RESIDENT

Act as a clinical epidemiologist. With a pretest probability of [X%] and a test with LR+ [A] and LR- [B], calculate the post-test probability for a positive and a negative result, showing the odds conversion steps, and say whether either result would change management.

TRY IT WITH Pretest 30%, LR+ 6, LR- 0.2.

966 Summarise the case in one line

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / INTERN

Act as a senior resident. Summarise this case, [PASTE CASE], in one line using semantic qualifiers and the key positives, without naming an unconfirmed diagnosis, suitable for handover or rounds. Flag anything you are unsure of so I can check it.

TRY IT WITH A 45-year-old alcoholic with 3 days of abdominal pain radiating to the back and vomiting.

967 Could this be a side effect?

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / INTERN

Act as a clinical pharmacologist. From this medication list, [LIST], which drugs could cause [NEW PROBLEM]? Rank them by plausibility with one line each, and note the timing clue to check. Flag anything you are unsure of so I can check it.

TRY IT WITH New-onset tremor and confusion in a patient on lithium and a recently added diuretic.

968 Who should I see first?

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · INTERN /
PG RESIDENT

Act as an emergency triage physician. Rank these patients by urgency, [BRIEF DESCRIPTIONS], using standard triage principles, with one line of reasoning each and the immediate action for the most urgent. Mark protocols [VERIFY LOCAL TRIAGE SYSTEM]. Flag anything you are unsure of so I can check it.

TRY IT WITH Four patients: chest pain, a fracture, fever with a rash, and an asthma attack with silent chest.

969 Refer, admit or manage here?

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · INTERN /
MEDICAL OFFICER

Act as a senior physician advising a medical officer. For this patient, [BRIEF CASE], in a [SETTING], decide whether to manage locally, admit, or refer, with the criteria behind the decision and what to do before transfer. Mark protocols [VERIFY].

TRY IT WITH A child with severe dehydration and lethargy at a primary health centre.

970 One clinical pearl from this case

QUICK

3.6 CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS · MBBS
STUDENT / PG RESIDENT

Act as a senior clinical teacher. From this case, [BRIEF CASE], give one clinical pearl (a practical, memorable insight that goes beyond the textbook) in under 30 words, and explain why it matters in one line. Flag anything you are unsure of so I can check it.

TRY IT WITH A patient with myasthenia gravis who worsened after an antibiotic.

3.7

Memory aids & high-yield summaries

What you can recall on exam day matters more than what you once read.

Turn every topic into flashcards, mnemonics and one-page sheets, and review them on a schedule.

30

prompts

4

Master

17

Standard

9

Quick

#971 High-yield flashcards for Anki or Notion

#972 One-page high-yield summary of any topic

#973 Custom mnemonics that actually stick

#974 Final-week rapid revision pack for a subject

#975 Set up an Anki deck and review routine

#976 Convert notes into cloze-deletion cards

#977 Plan image-occlusion cards for diagrams

#978 Comparison table of confusable conditions

#979 Mind map of a topic

#980 Build a memory palace for a long list

#981 Visual mnemonic or sketch for a concept

#982 Bank of one-liners for a subject

#983 Key numbers and values sheet

#984 Drug summary card

#985 Remember a classification

#986 Spaced revision schedule for my topics

#987 Condense long notes into revision notes

#988 Turn a topic into a memorable story

#989 Previous-year question patterns into a sheet

#990 Link a topic across subjects

#991 Turn my mistakes into cards

#992 One mnemonic for a list

#993 10 quick flashcards

#994 5 must-know facts

#995 Two-item quick comparison

#996 Explain simply, then technically

#997 Morning recall test

#998 Improve a flashcard

#999 Night-before-exam list

#1000 Decode an acronym or eponym

High-yield flashcards for Anki or Notion

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT

After finishing a chapter or topic, to turn it into cards you can review for months.

WHY

Active recall and spaced repetition are the two best-evidenced study techniques. Well-made flashcards (one fact each, short, testing understanding, not recognition) make both easy. Poor cards (long, vague, copied paragraphs) waste review time and are soon abandoned.

COPY THE PROMPT BELOW

PERSONA	You are a medical educator and spaced-repetition expert who writes crisp, exam-focused flashcards for [SUBJECT], in the style of the best Anki decks.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Ophthalmology, year 1] Subject and topic: [e.g. ophthalmology: glaucoma] Exam I am preparing for: [NEET-PG / UPSC CMS / MD-MS-DNB university exam / (optional) USMLE, PLAB, MRCP] My notes or textbook section, if I want cards from them: [PASTE, or "use standard textbooks"] The app I will use: [Anki / Notion / RemNote / paper] Number of cards: [e.g. 20]
TASK	1. Write [20] high-yield flashcards on the topic in the format "Q: ... A: ...", each answer 2 lines at most. 2. Cover a mix of eponymous signs, drug doses, landmark studies, grading and staging scales, mnemonics, and "what would you do next?" clinical questions. 3. Keep one fact per card; split any card that tests two things. 4. Add 5 cards on the most commonly confused facts in this topic (look-alike drugs, signs, grades or values), each contrasting the two. 5. Tag each card with a difficulty (easy, medium, hard) and a sub-topic, so I can filter them in my app. 6. Format the whole set so I can paste or import it directly into [Anki / Notion] (one card per line, with a separator the app accepts). 7. Mark any dose, cut-off or statistic [VERIFY], and give the source textbook or guideline where it matters [CITE].
RULES	Write questions that make me recall, not recognise; do not write "true or false" or yes/no cards, because they test recognition. Keep the answers short, because long answers get skipped in review. Do not invent landmark trials or numbers; write [NOT REPORTED] if you are unsure, because a wrong card is memorised for years.
OUTPUT	The cards as an importable list ("Q: ... A: ... tags"), then the 5 confused-facts cards, then a one-line note on anything I should verify.
TWEAK	End with 3 cloze-deletion versions of the hardest cards ("The drug of choice for {{c1:...}} is ...").

How to get more out of it

- Make cards the same day you study the topic. Memory is freshest then.
- Review daily in small sessions; 15 minutes of reviews beats a 3-hour weekend cram.
- Delete or rewrite any card you keep failing. It is usually badly written.
- Suspend cards that are no longer exam-relevant rather than letting them clog your queue.
- → **NEXT #975** Set up an Anki deck and review routine

One-page high-yield summary of any topic

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT

For final revision, when you need a whole topic on one page.

WHY

In the last weeks before an exam, you cannot re-read textbooks. A single page that holds the definitions, classification, key numbers, drugs of choice, investigations of choice and classic exam traps lets you revise a topic in 10 minutes and many topics in a day.

COPY THE PROMPT BELOW

PERSONA	You are a top NEET-PG and university-exam faculty member who writes famously concise, high-yield revision sheets.
MY CONTEXT	Audience and time available: [brief] What I most want to get out of this: [one line] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Subject and topic: [e.g. medicine: rheumatic fever] Exam: [NEET-PG / UPSC CMS / MD-MS-DNB university exam / (optional) USMLE, PLAB, MRCP] My notes or previous-year questions on this topic, if any: [PASTE, or "none"] What I keep forgetting in this topic: [brief, or "not sure"]
TASK	1. Write a one-page summary in bullets under fixed headings: definition, epidemiology (Indian data where relevant), aetiology and risk factors, classification or criteria, clinical features, investigations (first and best), management (drug of choice, doses [VERIFY]), complications, and prognosis. 2. Put all key numbers (cut-offs, doses, percentages) in bold. 3. Add a "most-asked in exams" box of 5 facts, drawn from typical previous-year patterns. 4. Add a "classic traps" box: 3 facts that students commonly get wrong in MCQs. 5. Add one mnemonic for the list that is hardest to remember. 6. Keep it to one printed page (about 400–500 words).
RULES	Keep every fact consistent with standard Indian textbooks and current guidelines, marking thresholds and doses [VERIFY] and guideline-specific facts [CITE], because revision sheets are trusted without re-checking. Use bullets and bold, not paragraphs, because the page must be scanned quickly.
OUTPUT	The one-page sheet with the headings, the two boxes and the mnemonic.
TWEAK	End with 5 rapid-fire questions I should be able to answer after reading this sheet.

How to get more out of it

- › Print it and add your own margin notes. Writing fixes memory.
- › Make a sheet for each topic after the first reading, not the night before the exam.
- › Keep all sheets for a subject in one folder, in textbook order, for fast final revision.
- › → **NEXT #974** Final-week rapid revision pack for a subject

Custom mnemonics that actually stick

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT

For long lists you must recall in order: causes, criteria, classifications, drug side effects.

WHY

Mnemonics work when they are vivid, meaningful and yours. Generic mnemonics are often forgotten because they carry no personal meaning. Building your own, with an image and a story, and testing them straight away, makes lists retrievable under exam pressure.

COPY THE PROMPT BELOW

PERSONA	You are a memory coach and medical educator who builds vivid, memorable mnemonics for medical lists.
MY CONTEXT	My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] The list I need to remember: [PASTE the list, e.g. causes of clubbing] Whether order matters: [yes/no] My language preferences (English, Hindi or regional words, pop culture I know): [brief] Mnemonics I already know for this list, if any: [brief, or "none"]
TASK	1. Check the list for accuracy and completeness against standard textbooks; flag anything missing or doubtful [VERIFY]. 2. Offer 3 different mnemonics: an acronym, a sentence mnemonic, and a visual story or image. 3. Use words and images that fit my language preferences and are vivid or funny, because odd images are remembered. 4. For each mnemonic, show exactly which letter or image maps to which item. 5. Say which mnemonic suits this list best and why. 6. Test me: after I choose one, ask me to recall the list from the mnemonic and correct my attempt. 7. Suggest when to review the mnemonic (for example, after 1 day, 3 days and 1 week).
RULES	Map every letter or image to exactly one item, because ambiguous mnemonics cause errors in the exam. Keep the content accurate and mark doubtful items [VERIFY]. Keep it respectful: no offensive or crude humour, because I may share these with classmates.
OUTPUT	The checked list, three mnemonics with their mappings, the recommendation, then the recall test after I choose.
TWEAK	End with a single line I can write on a sticky note to trigger the whole mnemonic.

How to get more out of it

- The sillier and more personal the image, the better it sticks.
- Say the mnemonic aloud and draw it once. Two senses beat one.
- Do not make mnemonics for short lists you can simply understand. Save them for arbitrary ones.
- → **NEXT #980** Build a memory palace for a long list

Final-week rapid revision pack for a subject

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT

In the last 7–10 days before an exam, to revise a whole subject quickly.

WHY

The last week is for retrieval, not new learning. A pack that condenses a subject into its highest-yield facts, confusable pairs, one-liners and a quick self-test gives you maximal marks per hour and exposes the gaps you still have time to fix.

COPY THE PROMPT BELOW

PERSONA	You are an experienced exam coach who builds last-week revision packs for NEET-PG, UPSC CMS and university exams.
MY CONTEXT	Audience and time available: [brief] What I most want to get out of this: [one line] My specialty and year: [e.g. final-year MBBS / MD Medicine, year 2] Subject: [e.g. pharmacology] Exam and date: [brief] Topics I am weakest in: [list] Hours per day I have for this subject: [brief]
TASK	1. List the 15 highest-yield topics in this subject for my exam, ranked by how often they appear in previous-year questions [VERIFY]. 2. For each topic, give 3–5 one-liners I must know. 3. Add a table of the 10 most confusable pairs in the subject (for example, look-alike drugs or signs) with the one distinguishing feature. 4. Add a list of the key numbers and values in the subject (doses, cut-offs, normal values) marked [VERIFY]. 5. Add a 20-question self-test covering the pack, with answers at the end. 6. Give a day-wise plan to cover the pack, prioritising my weak topics.
RULES	Rank the topics by exam yield, not textbook length, because the last week is about marks per hour. Keep the facts accurate and mark every number [VERIFY]. Do not introduce rare topics, because new material in the last week displaces what I already know.
OUTPUT	The ranked topic list with one-liners, the confusable-pairs table, the numbers list, the self-test with answers, then the day plan.
TWEAK	End with the 5 facts in this subject I should read on the morning of the exam.

How to get more out of it

- › Take the self-test first; revise what you get wrong, then retake it.
- › Sleep before the exam. Memory consolidation happens during sleep.
- › Stop adding new sources in the final week. Trust your pack.
- › → **NEXT #972** One-page high-yield summary of any topic

975 Set up an Anki deck and review routine

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT When starting spaced repetition, or when your Anki reviews have become overwhelming.

PERSONA	You are a spaced-repetition coach who helps medical students use Anki efficiently.
MY CONTEXT	My exam and date: [brief] Hours per day I can give to reviews: [brief] My current situation: [new to Anki / have a deck / reviews piled up]
TASK	1. Recommend a deck structure (by subject, with tags for systems and topics). 2. Recommend daily new-card and review limits that fit my time. 3. Explain the key settings in plain language, marked [VERIFY] since app versions change. 4. Give a plan to clear a review backlog without burning out.
RULES	Keep the advice practical and sustainable, because consistency beats volume in spaced repetition.
OUTPUT	The deck structure, the limits, the settings, then the backlog plan.

PRO TIP Never skip a day of reviews; missed days make the next day's pile heavier.

TRY IT WITH NEET-PG in 8 months, 1 hour a day, 3,000 overdue cards.

→ **NEXT #971** High-yield flashcards for Anki or Notion

976 Convert notes into cloze-deletion cards

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT When you have dense notes and want to turn them into cards quickly.

PERSONA	You are a medical educator who writes clean cloze-deletion flashcards.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] My notes: [PASTE] Exam: [brief]
TASK	1. Convert the key facts into cloze cards in the format "text {{c1::hidden}} text". 2. Hide only one key fact per cloze, the one worth recalling. 3. Skip trivial or obvious facts. 4. Mark any number or dose [VERIFY].
RULES	Keep each cloze short and unambiguous, because a cloze with several possible answers teaches nothing.
OUTPUT	The cloze cards, one per line, ready to import. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP If a cloze could be guessed from the sentence, rewrite it.

TRY IT WITH Paste your notes on the Jones criteria for rheumatic fever.

→ **NEXT #971** High-yield flashcards for Anki or Notion

977 Plan image-occlusion cards for diagrams

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For anatomy, histology, radiology, fundus photos and flowcharts.

PERSONA	You are an anatomy and imaging teacher who makes image-occlusion flashcards.
MY CONTEXT	The image or diagram (describe it or name it): [brief] Subject and exam: [brief]
TASK	1. List the structures or labels worth hiding, in order of exam importance. 2. For each, give the question to ask and the answer. 3. Add one clinical correlation card for the most important structure. 4. Suggest whether to hide one label at a time or all together.
RULES	Keep the anatomy and labels accurate to standard atlases [VERIFY], because a mislabelled card teaches the wrong structure.
OUTPUT	A table (label, question, answer), the clinical correlation card, then the occlusion advice.

PRO TIP Use your own textbook's diagrams so the exam figure looks familiar.

TRY IT WITH A labelled diagram of the brachial plexus.

→ **NEXT #971** High-yield flashcards for Anki or Notion

978 Comparison table of confusable conditions

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For look-alike diseases, drugs, signs or investigations.

PERSONA	You are an exam-focused teacher who makes clear comparison tables.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] The items to compare: [e.g. Crohn's disease vs ulcerative colitis] Exam: [brief]
TASK	1. Build a comparison table on the features examiners test (epidemiology, pathology, clinical features, investigations, management, complications). 2. Bold the single most discriminating feature in each row. 3. Add the 3 features most often confused in MCQs. 4. Mark numbers and guideline facts [VERIFY].
RULES	Keep the table accurate and balanced, because comparison questions reward precise differences.
OUTPUT	The table, then the 3 most-confused features.

PRO TIP Draw the table from memory the next day and check it.

TRY IT WITH Nephritic vs nephrotic syndrome.

→ **NEXT #972** One-page high-yield summary of any topic

979 Mind map of a topic

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT When a topic feels scattered and you need to see how it connects.

PERSONA	You are a visual-learning teacher who builds clear medical mind maps.
MY CONTEXT	Topic: [brief] Level and exam: [brief]
TASK	1. Put the topic at the centre and create 5–7 main branches (such as causes, pathology, features, investigations, treatment, complications). 2. Add 2–4 sub-branches to each with key words only. 3. Draw links between branches where one explains another (for example, pathology to clinical features). 4. Present it as an indented outline I can redraw on paper or in a mind-mapping app.
RULES	Use key words, not sentences, because mind maps work by showing structure at a glance.
OUTPUT	The indented outline, then a list of the cross-links.

PRO TIP Redraw it by hand from memory. The redrawing is the learning.

TRY IT WITH Topic: diabetes mellitus complications.

→ **NEXT #980** Build a memory palace for a long list

980 Build a memory palace for a long list

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For long ordered lists: cranial nerve nuclei, drug classes, staging systems.

PERSONA	You are a memory coach who teaches the method of loci to medical students.
MY CONTEXT	The list: [PASTE] A familiar place I know well (home, hostel, hospital route): [brief]
TASK	1. Choose a route through my place with one location per item. 2. Place a vivid image for each item at each location, linking the image to the item. 3. Walk me through the route once. 4. Test me by asking me to walk it back and name each item.
RULES	Keep the images vivid and clearly linked to each item, and check the list's accuracy [VERIFY], because the palace preserves errors as faithfully as facts.
OUTPUT	The route with the locations and images, then the recall test.

PRO TIP Reuse the same palace for one subject only, to avoid mixing lists.

TRY IT WITH The 12 cranial nerves, placed along the route from your hostel room to the canteen.

→ **NEXT #973** Custom mnemonics that actually stick

981 Visual mnemonic or sketch for a concept

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For mechanisms, pathways and signs that are easier to see than to read.

PERSONA	You are a medical illustrator and teacher who creates simple visual mnemonics.
MY CONTEXT	Concept: [e.g. the coagulation cascade / the brachial plexus / the renin-angiotensin system] Exam: [brief]
TASK	1. Describe a simple sketch or visual metaphor that captures the concept. 2. Label the parts and explain how each maps to the concept. 3. Give step-by-step instructions so I can draw it in 2 minutes. 4. Point out the parts examiners most often test.
RULES	Keep the metaphor faithful to the real physiology or anatomy, and say where it oversimplifies, because a misleading picture is hard to unlearn.
OUTPUT	The sketch description, the label key, the drawing steps, then the exam points.

PRO TIP Draw it in the exam too; a diagram often earns marks on its own.

TRY IT WITH The coagulation cascade as a waterfall.

→ **NEXT #979** Mind map of a topic

982 Bank of one-liners for a subject

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For NEET-PG and UPSC CMS preparation, where one-line facts win MCQs.

PERSONA	You are a NEET-PG faculty member who writes high-yield one-liners.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Subject and topic: [brief] Exam: [NEET-PG / UPSC CMS / other]
TASK	1. Write 30 one-liners on the topic, each a single testable fact. 2. Group them by sub-topic. 3. Star the 10 most frequently tested [VERIFY]. 4. Mark numbers and doses [VERIFY].
RULES	Keep each one-liner to one fact and one line, because one-liners are for fast repeated reading.
OUTPUT	The grouped one-liners with stars. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Read them aloud on the way to college; short repeated exposures add up.

TRY IT WITH Topic: microbiology, gram-positive cocci.

→ **NEXT #971** High-yield flashcards for Anki or Notion

983 Key numbers and values sheet

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For the numbers every exam tests: normal values, cut-offs, doses, scores.

PERSONA	You are a clinical teacher who makes precise reference sheets.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Subject or system: [e.g. nephrology] Exam: [brief]
TASK	1. List the key numbers: normal values, diagnostic cut-offs, doses, scoring thresholds. 2. Give the unit for each and the source guideline or textbook [CITE]. 3. Add a memory trick for the hardest 5. 4. Mark every number [VERIFY].
RULES	Keep units explicit and flag where Indian and international values differ, because unit and guideline errors cost marks.
OUTPUT	A table (item, value, unit, source), then the memory tricks.

PRO TIP Check this sheet against your textbook once; numbers are where AI slips most.

TRY IT WITH Subject: diabetes and thyroid values.

→ **NEXT #972** One-page high-yield summary of any topic

984 Drug summary card

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT / INTERN

USE IT For pharmacology revision and for drugs you prescribe often.

PERSONA	You are a clinical pharmacologist who writes compact drug cards.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Drug or drug class: [brief] Level and exam: [brief]
TASK	1. Give the class, mechanism, key uses, dose [VERIFY], route, adverse effects, contraindications and key interactions. 2. Add the one fact examiners love about this drug. 3. Add one mnemonic for the adverse effects.
RULES	Keep doses and interactions accurate to standard references and mark them [VERIFY], because drug errors harm patients.
OUTPUT	A compact card under the headings, then the exam fact and the mnemonic. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Make cards for classes, then note only the exceptions for individual drugs.

TRY IT WITH Amiodarone.

→ **NEXT #983** Key numbers and values sheet

985 Remember a classification

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For classifications and staging systems examiners love to ask.

PERSONA	You are a teacher who makes classifications easy to remember.
MY CONTEXT	The classification: [e.g. WHO classification of tuberculosis / Lown grading / TNM for breast cancer] Exam: [brief]
TASK	1. Give the classification accurately, with its source [CITE]. 2. Explain the logic behind it, so it makes sense rather than being memorised blindly. 3. Give a mnemonic or pattern to remember it. 4. Give one clinical use of the classification.
RULES	Reproduce the classification exactly and mark the version or year [VERIFY], because classifications are updated and examiners know which version they expect.
OUTPUT	The classification, the logic, the memory aid, then the clinical use.

PRO TIP Write the classification from memory once a week until it is automatic.

TRY IT WITH The Child-Pugh score.

→ **NEXT #973** Custom mnemonics that actually stick

986 Spaced revision schedule for my topics

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT After finishing a first reading, to plan when to revise each topic.

PERSONA	You are a study coach who designs spaced-repetition revision schedules.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Topics I have finished and when: [list with dates] Exam date: [brief] Hours per day for revision: [brief]
TASK	1. Schedule each topic for revision at expanding intervals (for example, 1, 3, 7, 21 days). 2. Balance the daily load within my hours. 3. Put weaker topics on shorter intervals. 4. Present it as a calendar table.
RULES	Keep the plan realistic for my hours, because an overloaded schedule gets abandoned.
OUTPUT	The calendar table, then a one-line rule to adjust it when I fall behind.

PRO TIP Revise from your sheets and cards, not the full textbook.

TRY IT WITH 12 topics finished over the last month; exam in 90 days; 2 hours a day.

→ **NEXT #974** Final-week rapid revision pack for a subject

987 Condense long notes into revision notes

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT When your notes are too long to revise.

PERSONA	You are an exam coach who condenses notes without losing high-yield facts.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] My notes: [PASTE] Exam: [brief] Target length: [e.g. one third]
TASK	1. Condense the notes to the target length, keeping all high-yield facts. 2. Convert paragraphs into bullets and tables. 3. Bold the key words and numbers. 4. List anything I removed that might still be worth keeping.
RULES	Do not add new facts that are not in my notes, and flag any fact that looks wrong [VERIFY], because condensed notes must stay faithful to my sources.
OUTPUT	The condensed notes, then the list of removed items.

PRO TIP Condense again before the final month; each pass sharpens recall.

TRY IT WITH Paste 3 pages of notes on anaemia.

→ **NEXT #972** One-page high-yield summary of any topic

988 Turn a topic into a memorable story

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For pathways, sequences and disease courses.

PERSONA	You are a teacher who explains medicine through memorable stories.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Audience or purpose: [brief] Topic or sequence: [brief] Level: [brief]
TASK	1. Turn the sequence into a short story with characters and events that map to the steps. 2. Keep the story under 200 words. 3. Add a key showing which character or event maps to which step.
RULES	Keep the mapping accurate and complete, and say where the story oversimplifies, because stories stick, including their errors.
OUTPUT	The story, then the key. Keep it concise, and list anything you were unsure about at the end so I can check it.

PRO TIP Retell the story to a friend; teaching it locks it in.

TRY IT WITH The steps of the urea cycle.

→ **NEXT #980** Build a memory palace for a long list

989 Previous-year question patterns into a sheet

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT When you have previous-year questions and want to know what to memorise.

PERSONA	You are an exam analyst who finds patterns in previous-year questions.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Previous-year questions on a topic or subject: [PASTE] Exam: [brief]
TASK	1. Group the questions by sub-topic and count them. 2. Identify the facts that are repeatedly tested. 3. Turn those facts into a one-page memory sheet. 4. Suggest 5 likely future questions on the same facts.
RULES	Base the patterns only on the pasted questions and say how many you analysed, because patterns from a few questions can mislead.
OUTPUT	The count table, the repeated facts, the memory sheet, then the likely questions.

PRO TIP Repeated facts are repeated for a reason. Learn them first.

TRY IT WITH Paste 40 previous NEET-PG questions on ophthalmology.

→ **NEXT #974** Final-week rapid revision pack for a subject

990 Link a topic across subjects

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT For integrated questions that join anatomy, physiology, pathology, pharmacology and medicine.

PERSONA	You are a teacher of integrated medical education who links topics across subjects.
MY CONTEXT	My specialty and level: [e.g. final-year MBBS / MD Medicine, year 2] Topic: [e.g. the thyroid gland] Exam: [brief]
TASK	1. Link the topic across subjects: anatomy, physiology, biochemistry, pathology, microbiology, pharmacology, medicine and surgery, as relevant. 2. Give 2 high-yield facts per subject. 3. Show how one subject explains another (for example, how the physiology explains the drug action). 4. Write 3 integrated MCQs that cross subjects, with answers.
RULES	Keep the facts accurate and mark numbers [VERIFY], because integrated questions punish half-remembered links.
OUTPUT	A table (subject, facts, link to the next subject), then the integrated MCQs.

PRO TIP Integrated links make each fact easier to recall, because it is stored in more than one place.

TRY IT WITH Topic: vitamin B12.

→ **NEXT #979** Mind map of a topic

991 Turn my mistakes into cards

STANDARD

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

USE IT After a mock test or MCQ session, to stop repeating the same mistakes.

PERSONA	You are an exam coach who turns errors into targeted revision.
MY CONTEXT	Questions I got wrong, with my answer and the correct answer: [PASTE] Exam: [brief]
TASK	1. For each mistake, identify the missing fact or the misconception. 2. Write one flashcard per mistake that targets exactly that gap. 3. Group the mistakes by pattern (knowledge gap, misreading, confusable facts). 4. Suggest one habit to reduce the commonest pattern.
RULES	Target the specific gap, not the whole topic, and mark facts [VERIFY], because error cards work only when they fix the exact misconception.
OUTPUT	The error table (question, gap, card), the pattern groups, then the habit.

PRO TIP Your own mistakes make the highest-yield cards you will ever write.

TRY IT WITH Paste 10 wrong answers from your last grand test.

→ **NEXT #971** High-yield flashcards for Anki or Notion

QUICK PROMPTS · 3.7

992 One mnemonic for a list

QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as a medical memory coach. Give me one memorable mnemonic for [LIST]. Show which letter maps to which item, check the list is accurate and complete [VERIFY], and keep it respectful. Flag anything you are unsure of so I can check it.

TRY IT WITH Causes of pancreatitis.

993 10 quick flashcards

QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as a medical educator. Write 10 high-yield flashcards on [TOPIC] for [EXAM], in the format "Q: ... | A: ...", with one fact per card and answers of 2 lines at most. Mark numbers [VERIFY]. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: thyroid disorders, for NEET-PG.

994 5 must-know facts

QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as an exam coach. Give me the 5 facts about [TOPIC] most often tested in [EXAM], one line each, with any numbers marked [VERIFY]. Add one common trap students fall into. Flag anything you are unsure of so I can check it.

TRY IT WITH Topic: tetralogy of Fallot, for NEET-PG.

995 Two-item quick comparison

QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as a medical teacher. Compare [A] and [B] in a 5-row table on the features examiners test, and bold the single most discriminating feature. Mark numbers [VERIFY] so I can check them. Flag anything you are unsure of so I can check it.

TRY IT WITH Type 1 vs type 2 respiratory failure.

996 Explain simply, then technically QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT

Act as a teacher. Explain [CONCEPT] first in simple words a school student would understand, then in exam-ready technical language in 5 bullets. End with one analogy I can remember it by. Flag anything you are unsure of so I can check it.

TRY IT WITH Concept: the Frank-Starling law.

997 Morning recall test QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as an examiner. Ask me 10 rapid recall questions on [TOPIC] I studied yesterday, one at a time, and wait for my answer each time. At the end, list the ones I missed so I can make cards for them.

TRY IT WITH Topic: antitubercular drugs.

998 Improve a flashcard QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as a spaced-repetition expert. Rewrite this flashcard so it tests one fact, prompts recall rather than recognition and has a short answer: [PASTE CARD]. Explain what was wrong in one line. Flag anything you are unsure of so I can check it.

TRY IT WITH "Q: Tell me everything about digoxin | A: [a long paragraph]"

999 Night-before-exam list QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as an exam coach. Give me the 15 highest-yield facts in [SUBJECT] to glance at the night before [EXAM], one line each, numbers marked [VERIFY]. No new topics, only the most tested facts. Flag anything you are unsure of so I can check it.

TRY IT WITH Subject: forensic medicine, for the MBBS university exam.

1000 Decode an acronym or eponym QUICK

3.7 MEMORY AIDS & HIGH-YIELD SUMMARIES · MBBS STUDENT / PG RESIDENT

Act as a medical teacher. Expand and explain [ACRONYM OR EPONYM], give its clinical significance in 2 lines, and add one way to remember it. Flag if the term has more than one meaning in medicine. Flag anything you are unsure of so I can check it.

TRY IT WITH "CURB-65."

Every prompt at a glance.

Q = Quick · S = Standard · M = Master

3.1 · LEARNING ANY TOPIC

#701	Structured topic master for any subject	M
#702	A personal tutor that checks my understanding	M
#703	Build a custom tutor that answers only from my notes	M
#704	Study a chapter with NotebookLM	M
#705	Understand a mechanism from first principles	M
#706	One topic across all subjects: vertical integration	M
#707	Learn a topic in four layers	S
#708	Explain it back and find my gaps (Feynman technique)	S
#709	Learn a hard concept through analogies	S
#710	Surface features and deeper mechanisms (iceberg)	S
#711	Learn a process as a flowchart	S
#712	Compare options in a decision matrix	S
#713	Classifications as a tree	S
#714	A learning roadmap from my level to my goal	S
#715	Connect this topic to what I already know	S
#716	Learn through a worked example	S
#717	Common misconceptions in a topic	S
#718	Sort a topic into high- and low-yield	S
#719	Turn a textbook chapter into structured notes	S
#720	Clean up my lecture notes	S
#721	Notes from a video lecture transcript	S
#722	Learn a clinical guideline for exams	S
#723	Learn any drug with one framework	S
#724	A standard template for any disease	S
#725	Learn an anatomy topic that sticks	S
#726	Understand physiology graphs and curves	S
#727	Learn to read a type of clinical image	S
#728	Normal values and interpretation	S
#729	Learn a microorganism with one template	S
#730	Simplify a biochemistry pathway	S
#731	Learn national health programmes (community medicine)	S
#732	Make my own questions from my notes	S
#733	Design an interleaved practice session	S
#734	Diagnose my knowledge gaps	S
#735	Explain it like I'm a second-year student	Q
#736	One-page summary of a topic	Q
#737	Ten must-know facts	Q
#738	Ask "why" five times	Q
#739	Clear up two confusing terms	Q
#740	A real-world example of a concept	Q
#741	Why does this basic-science fact matter clinically?	Q
#742	A quick 5-question quiz	Q
#743	Define it, with an example	Q
#744	Clear up one specific confusion	Q
#745	The whole topic in 280 characters	Q
#746	A diagram I can draw	Q

#747	What should I study next?	Q
#748	Is my understanding correct?	Q
#749	Translate the jargon	Q
#750	Three questions the examiner will ask	Q

3.2 · STUDY PLANNING & REVISION

#751	A personalised day-by-day study plan	M
#752	NEET-PG preparation strategy	M
#753	Plan for the MD / MS / DNB final exam	M
#754	UPSC CMS preparation strategy	M
#755	A spaced-repetition revision schedule	S
#756	The last 30 days before the exam	S
#757	The last 7 days before the exam	S
#758	I've fallen behind: recover my plan	S
#759	Weekly study review	S
#760	Which subject should I study first?	S
#761	Study around internship or residency duties	S
#762	Plan revision cycles (R1, R2, R3)	S
#763	Analyse a mock or grand test properly	S
#764	Rescue my weakest subject	S
#765	Am I heading for burnout?	S
#766	Design focused study sessions	S
#767	Run an effective study group	S
#768	Too many resources: choose and commit	S
#769	The night before and the exam day	S
#770	Sleep, memory and study performance	S
#771	Build a simple accountability tracker	S
#772	Plan my study day	Q
#773	My three priorities today	Q
#774	How long will this subject take?	Q
#775	What should I revise today?	Q
#776	Reframe a demotivating thought	Q
#777	Fix my biggest distraction	Q
#778	Set a goal for this study session	Q
#779	Did I hit my weekly target?	Q
#780	Study breaks that actually refresh	Q

3.3 · MCQ PRACTICE & EXAM STRATEGY

#781	Exam-pattern MCQs with full explanations	M
#782	Solve a question and learn the logic	M
#783	Analyse previous-year questions for patterns	M
#784	Rapid-fire MCQ and viva grilling	M
#785	A mistake notebook that raises scores	M
#786	Practise image-based questions	M
#787	Why was my answer wrong?	S
#788	Learn from the distractors	S
#789	Eliminate options when I'm unsure	S
#790	When to guess and when to skip	S

#791	Time management in an MCQ exam	S	#840	Strong introductions and conclusions	S
#792	Handle EXCEPT, NOT and "most likely" questions	S	#841	Time allocation for a theory paper	S
#793	Extract one-liners from my practice questions	S	#842	Answer "compare and contrast" questions	S
#794	Recent updates likely to be tested	S	#843	What does the question word really ask?	S
#795	20 MCQs on the topic I just studied	S	#844	A "recent advances" paragraph	S
#796	Two-step reasoning questions	S	#845	A stepwise management answer	S
#797	Pharmacology MCQ practice	S	#846	Answer a case-based theory question	S
#798	Microbiology MCQ practice	S	#847	Make my answers look complete at a glance	S
#799	Community medicine MCQs, including calculations	S	#848	Build a bank of repeated questions	S
#800	Biostatistics calculation practice	S	#849	Model answers from a textbook chapter	S
#801	Applied anatomy MCQs	S	#850	Improve my writing speed	S
#802	MCQs for the short subjects (ENT, ophthalmology, dermatology...)	S	#851	What examiners actually look for	S
#803	MCQs from my own notes	S	#852	A one-line definition to open my answer	Q
#804	Build my own mock test	S	#853	The five points I must not miss	Q
#805	Balance accuracy and speed	S	#854	An "enumerate" answer	Q
#806	What to do with flagged questions	S	#855	Which diagram should I add?	Q
#807	Anxiety during MCQ exams	S	#856	Convert my paragraph into exam points	Q
#808	Use a question bank effectively	S	#857	Headings for my answer	Q
#809	Practice in the UPSC CMS style	S	#858	A 2-mark answer	Q
#810	USMLE- or PLAB-style vignettes (optional)	S	#859	A reusable template for disease answers	Q
#811	Practise match-the-following and assertion-reason formats	S	#860	A strong closing line	Q
#812	Turn my question-bank statistics into a plan	S	3.5 · VIVA, OSCE & PRACTICAL EXAMS		
#813	High-yield facts from previous-year themes	S	#861	Viva voce simulator, one question at a time	M
#814	Read long clinical vignettes faster	S	#862	Long case: presentation and examiner questions	M
#815	One high-quality MCQ with explanation	Q	#863	Short cases: examination and spot findings	M
#816	Explain this answer simply	Q	#864	OSCE station simulator	M
#817	Find the key clue in the question	Q	#865	Thesis defence viva preparation	M
#818	Make a similar question	Q	#866	Spotters and table viva: instruments, specimens, images, drugs	M
#819	Ten rapid true/false statements	Q	#867	Opening questions examiners use on a topic	S
#820	The one feature that separates them	Q	#868	Structure any viva answer	S
#821	Five one-liners from a topic	Q	#869	Instrument viva: one instrument, fully prepared	S
#822	What's the trap in this question?	Q	#870	Pathology or anatomy specimen viva	S
#823	Solve it from first principles	Q	#871	X-ray viva: describe, diagnose, discuss	S
#824	Turn a fact into a question	Q	#872	ECG viva: systematic and confident	S
#825	Check my reasoning on a question	Q	#873	Pharmacology practical: drug and prescription viva	S
#826	Which option is most likely?	Q	#874	Charts and graphs viva (partograph, growth charts, epidemic curves)	S
#827	Solve an EXCEPT question safely	Q	#875	Demonstrate a system examination properly	S
#828	Numbers I must remember for this topic	Q	#876	Elicit a specific clinical sign	S
#829	Formula recall drill	Q	#877	History-taking OSCE with a simulated patient	S
#830	Twenty last-minute facts	Q	#878	Breaking bad news and difficult conversations	S
3.4 · LONG & SHORT ANSWER WRITING			#879	Counselling station (medication, lifestyle, procedure consent)	S
#831	Model long answer (10–15 marks)	M	#880	Procedural skills station	S
#832	Model answers for previous-year questions	M	#881	Data-interpretation station	S
#833	Examiner evaluation of my written answer	M	#882	Prescription-writing station	S
#834	MD / MS / DNB theory essay with depth	M	#883	Viva etiquette and presence	S
#835	Short note (5 marks)	S	#884	When I blank out in a viva	S
#836	Plan any answer in 2 minutes	S	#885	Handle different examiner styles	S
#837	A diagram that earns marks	S	#886	DNB-style bedside case discussion	S
#838	Put a flowchart in my answer	S	#887	Table viva: rapid structured answers	S
#839	A comparison table in my answer	S			

#888	Community medicine spotters (vaccines, models, charts)	S	#937	Approach to jaundice	S
#889	Forensic medicine spotters (weapons, poisons, specimens)	S	#938	Approach to acute kidney injury	S
#890	Obstetrics and gynaecology practical preparation	S	#939	Approach to hyponatraemia	S
#891	Paediatric practical preparation	S	#940	Age-specific differential in children	S
#892	Surgery short case: examining a lump or swelling	S	#941	Differential diagnosis in pregnancy	S
#893	The practical exam day	S	#942	Atypical presentations in the elderly	S
#894	Last-minute viva revision list	S	#943	Tropical infection differential (Indian setting)	S
#895	A 60-second answer to "what is X?"	Q	#944	Could a drug be causing this?	S
#896	Classify it for the viva	Q	#945	Recognise the syndrome from scattered findings	S
#897	Five viva questions on my topic	Q	#946	Discriminate between two close diagnoses	S
#898	An instrument in one breath	Q	#947	Apply a clinical decision rule	S
#899	How do I elicit this sign?	Q	#948	A diagnostic time-out before I close the case	S
#900	Describe this finding like an examiner expects	Q	#949	Turn a ward case into exam practice	S
#901	Where will the examiner go next?	Q	#950	Set management priorities (ABCDE)	S
#902	"What would you do next?"	Q	#951	Think aloud and get feedback on my reasoning	S
#903	The differential they'll ask for	Q	#952	Top three diagnoses in one line	Q
#904	This topic's examiner-favourite question	Q	#953	Can't-miss diagnoses for a symptom	Q
#905	Say "I don't know" well in a viva	Q	#954	The one history question that helps most	Q
#906	OSCE station opening lines	Q	#955	The sign I must look for	Q
#907	Close an OSCE station well	Q	#956	The first investigation to order	Q
#908	Sound confident without overclaiming	Q	#957	Interpret one abnormal result	Q
#909	Plain-language phrases for counselling stations	Q	#958	What doesn't fit the diagnosis?	Q
#910	Describe a spot diagnosis	Q	#959	Name the syndrome from the findings	Q
3.6 · CLINICAL REASONING & DIFFERENTIAL DIAGNOSIS			#960	Causes by category in one minute	Q
#911	Prioritised differential with can't-miss diagnoses	M	#961	The top causes at this age	Q
#912	Build illness scripts for diseases	M	#962	India-specific causes to consider	Q
#913	Interactive patient case simulator	M	#963	Quick red-flag checklist	Q
#914	Approach to any presenting symptom	M	#964	Which test rules it out?	Q
#915	Debrief my reasoning for cognitive bias	M	#965	From pretest to post-test probability	Q
#916	Choose and interpret tests like an expert (Bayesian reasoning)	M	#966	Summarise the case in one line	Q
#917	Reason through a management decision	M	#967	Could this be a side effect?	Q
#918	Write a problem representation	S	#968	Who should I see first?	Q
#919	Practise semantic qualifiers	S	#969	Refer, admit or manage here?	Q
#920	Generate causes systematically (VINDICATE)	S	#970	One clinical pearl from this case	Q
#921	Localise the lesion (neurology)	S	3.7 · MEMORY AIDS & HIGH-YIELD SUMMARIES		
#922	Estimate a pretest probability	S	#971	High-yield flashcards for Anki or Notion	M
#923	Use likelihood ratios at the bedside	S	#972	One-page high-yield summary of any topic	M
#924	Red flags for a presenting symptom	S	#973	Custom mnemonics that actually stick	M
#925	Can't-miss diagnoses for a presentation	S	#974	Final-week rapid revision pack for a subject	M
#926	The next best investigation	S	#975	Set up an Anki deck and review routine	S
#927	Interpret a lab pattern in clinical context	S	#976	Convert notes into cloze-deletion cards	S
#928	Reason through an acid-base problem	S	#977	Plan image-occlusion cards for diagrams	S
#929	Interpret an ECG in clinical context	S	#978	Comparison table of confusable conditions	S
#930	Interpret imaging in clinical context	S	#979	Mind map of a topic	S
#931	Approach to fever, including PUO	S	#980	Build a memory palace for a long list	S
#932	Approach to chest pain	S	#981	Visual mnemonic or sketch for a concept	S
#933	Approach to breathlessness	S	#982	Bank of one-liners for a subject	S
#934	Approach to altered sensorium	S	#983	Key numbers and values sheet	S
#935	Approach to acute abdominal pain	S	#984	Drug summary card	S
#936	Approach to anaemia	S	#985	Remember a classification	S
			#986	Spaced revision schedule for my topics	S

#987 Condense long notes into revision notes	S	#994 5 must-know facts	Q
#988 Turn a topic into a memorable story	S	#995 Two-item quick comparison	Q
#989 Previous-year question patterns into a sheet	S	#996 Explain simply, then technically	Q
#990 Link a topic across subjects	S	#997 Morning recall test	Q
#991 Turn my mistakes into cards	S	#998 Improve a flashcard	Q
#992 One mnemonic for a list	Q	#999 Night-before-exam list	Q
#993 10 quick flashcards	Q	#1000 Decode an acronym or eponym	Q

Which AI tool for which job.

JOB	BEST TOOL	WHY
Learning a topic from your own books and notes	NotebookLM, Claude, ChatGPT (upload)	Explanations grounded in your sources
Tutoring, active recall and self-testing	ChatGPT Study mode, Claude, Gemini	Socratic questioning and instant feedback
Custom exam tutor from your notes	Custom GPTs, Claude Projects, Gemini Gems	Answers only from your uploaded material
Flashcards and spaced repetition	Anki, Quizlet	Proven spaced-repetition scheduling
Audio revision on the move	NotebookLM Audio Overview	Turns chapters into listenable discussions
Visual notes and mind maps	Canva, Napkin AI, ChatGPT images	Diagrams and summary sheets
Checking facts and recent guidelines	Perplexity, Consensus, the guideline itself	Cited sources you can verify

STUDYING WITH AI, SAFELY

- › Verify every fact, number and drug dose against a standard textbook or current guideline.
- › Use AI to test yourself, not to avoid thinking. Answer first, then check.
- › Never use AI during an exam or assessment where it isn't allowed.
- › Clinical reasoning prompts are for learning, not for decisions about real patients.

One last thing.

These prompts will not pass your exam for you. They will make every hour of study count: more recall, more practice and more feedback. The doctors who do best will be the ones who use AI to think harder, not less.

Test yourself. Check everything. Walk into the exam hall ready.

More AI in medicine every week: Instagram [@abhishekjbenur](#) · [paraboxai.com](#)

This resource is for educational purposes. It does not replace standard textbooks, current clinical guidelines, your university curriculum or your teachers. AI-generated content may contain errors: verify all facts independently. Clinical reasoning prompts are learning aids and are not a substitute for clinical judgement or supervision in patient care.